Answer Key & Rationales · Form 2

Quick key. **P** = pretest (unscored; do not count). Bank 2026.10.22-0834.

#	Key	Area	Sub-area	P
1	**B**	II	II.B	
2	**A**	III	III.B	
3	**A**	I	I.B	
4	**D**	I	I.B	
5	**C**	I	I.C	
6	**B**	III	III.B	
7	**A**	III	III.B	
8	**A**	I	I.C	
9	**B**	II	II.B	
10	**B**	II	II.A	
11	**C**	III	III.B	**P**
12	**C**	I	I.B	
13	**B**	III	III.B	
14	**B**	III	III.B	
15	**A**	III	III.A	
16	**C**	II	II.A	
17	**B**	II	II.A	
18	**B**	I	I.B	**P**
19	**C**	I	I.B	
20	**C**	III	III.B	
21	**A**	II	II.A	
22	**C**	III	III.B	
23	**C**	II	II.B	
24	**C**	I	I.A	
25	**A**	II	II.B	
26	**D**	III	III.B	
27	**C**	II	II.B	
28	**D**	III	III.B	
29	**A**	II	II.A	
30	**C**	III	III.D	
31	**B**	III	III.C	**P**
32	**C**	II	II.A	
33	**B**	I	I.B	
34	**B**	II	II.A	
35	**C**	I	I.C	
36	**C**	II	II.A	
37	**A**	III	III.A	
38	**A**	I	I.C	
39	**B**	II	II.C	
40	**D**	II	II.C	
41	**B**	I	I.C	**P**
42	**B**	II	II.A	
43	**A**	I	I.A	
44	**A**	I	I.C	
45	**B**	II	II.B	
46	**A**	III	III.D	
47	**D**	III	III.A	
48	**B**	I	I.A	
49	**A**	I	I.A	
50	**A**	I	I.A	
51	**C**	II	II.B	**P**
52	**C**	III	III.C	
53	**D**	I	I.B	
54	**B**	II	II.C	
55	**B**	I	I.B	
56	**A**	I	I.A	
57	**A**	III	III.C	
58	**D**	I	I.B	
59	**C**	I	I.B	
60	**B**	III	III.B	**P**
61	**B**	III	III.B	
62	**B**	III	III.D	
63	**B**	III	III.B	
64	**A**	II	II.A	
65	**B**	III	III.D	
66	**B**	II	II.B	
67	**A**	I	I.C	
68	**C**	I	I.B	
69	**B**	I	I.B	
70	**B**	II	II.B	
71	**B**	I	I.A	**P**
72	**A**	III	III.B	
73	**A**	II	II.B	**P**
74	**C**	III	III.A	
75	**C**	II	II.A	
76	**C**	I	I.B	
77	**A**	III	III.A	
78	**A**	I	I.A	
79	**A**	I	I.B	
80	**C**	III	III.B	
81	**C**	II	II.C	
82	**C**	II	II.A	
83	**D**	III	III.A	
84	**B**	III	III.B	**P**
85	**A**	III	III.B	
86	**B**	II	II.B	**P**
87	**B**	III	III.B	
88	**A**	I	I.A	
89	**D**	III	III.B	
90	**C**	II	II.A	
91	**A**	I	I.C	
92	**C**	I	I.C	
93	**C**	II	II.A	
94	**A**	I	I.A	
95	**A**	I	I.B	
96	**C**	II	II.A	
97	**B**	II	II.B	
98	**D**	III	III.C	
99	**A**	III	III.B	
100	**B**	I	I.B	
101	**A**	III	III.B	
102	**A**	II	II.B	
103	**C**	II	II.B	
104	**B**	I	I.A	
105	**C**	I	I.B	
106	**B**	II	II.B	
107	**C**	I	I.A	
108	**A**	III	III.B	
109	**C**	II	II.A	
110	**C**	II	II.A	**P**
111	**A**	III	III.B	
112	**A**	II	II.A	
113	**C**	I	I.C	
114	**C**	II	II.A	
115	**A**	II	II.C	
116	**B**	I	I.B	**P**
117	**A**	III	III.C	
118	**D**	I	I.B	
119	**C**	I	I.A	
120	**A**	I	I.C	
121	**B**	III	III.C	
122	**B**	I	I.C	

Scoring worksheet

Count correct answers on **scored items only**. Item numbers for each sub-area are listed; pretest items are left out.

Area / sub-area	Scored item numbers	Items	Correct
I. Values and Ethics		**40**	**____ / 40**
I.A Ethical Principles and Responsibilities	24, 43, 48, 49, 50, 56, 78, 88, 94, 104, 107, 119	12	____
I.B Ethical Service Delivery	3, 4, 12, 19, 33, 53, 55, 58, 59, 68, 69, 76, 79, 95, 100, 105, 118	17	____
I.C Diversity and Social Justice	5, 8, 35, 38, 44, 67, 91, 92, 113, 120, 122	11	____
II. Assessment and Planning		**35**	**____ / 35**
II.A Assessment Concepts	10, 16, 17, 21, 29, 32, 34, 36, 42, 64, 75, 82, 90, 93, 96, 109, 112, 114	18	____
II.B Assessment Methods and Techniques	1, 9, 23, 25, 27, 45, 66, 70, 97, 102, 103, 106	12	____
II.C Assessment Practices	39, 40, 54, 81, 115	5	____
III. Intervention and Practice		**35**	**____ / 35**
III.A Practice Concepts	15, 37, 47, 74, 77, 83	6	____
III.B Intervention Methods and Techniques	2, 6, 7, 13, 14, 20, 22, 26, 28, 61, 63, 72, 80, 85, 87, 89, 99, 101, 108, 111	20	____
III.C Practice Evaluation and Research	52, 57, 98, 117, 121	5	____
III.D Supervision and Administration	30, 46, 62, 65	4	____

Pretest items (skip when scoring): 11, 18, 31, 41, 51, 60, 71, 73, 84, 86, 110, 116

Total correct (scored items)	_____ / 110
Practice pass line	77 / 110 (70%)
Correct needed to reach the line	77 − your total = _____

The real exam sets its passing score separately for each form (it is reported as a scaled score). 77 of 110 is a deliberately cautious practice target that builds a margin; it is not a prediction of your real result.

Score conversion (raw out of 110 → percent)

The boxed row is the practice pass line: **77 / 110 = 70%**. Any score at or above it meets the line.

Raw	%	Raw	%	Raw	%	Raw	%	Raw	%	Raw	%
110	100%	91	83%	72	65%	53	48%	34	31%	15	14%
109	99%	90	82%	71	65%	52	47%	33	30%	14	13%
108	98%	89	81%	70	64%	51	46%	32	29%	13	12%
107	97%	88	80%	69	63%	50	45%	31	28%	12	11%
106	96%	87	79%	68	62%	49	45%	30	27%	11	10%
105	95%	86	78%	67	61%	48	44%	29	26%	10	9%
104	95%	85	77%	66	60%	47	43%	28	25%	9	8%
103	94%	84	76%	65	59%	46	42%	27	25%	8	7%
102	93%	83	75%	64	58%	45	41%	26	24%	7	6%
101	92%	82	75%	63	57%	44	40%	25	23%	6	5%
100	91%	81	74%	62	56%	43	39%	24	22%	5	5%
99	90%	80	73%	61	55%	42	38%	23	21%	4	4%
98	89%	79	72%	60	55%	41	37%	22	20%	3	3%
97	88%	78	71%	59	54%	40	36%	21	19%	2	2%
96	87%	**77**	**70%**	58	53%	39	35%	20	18%	1	1%
95	86%	76	69%	57	52%	38	35%	19	17%	0	0%
94	85%	75	68%	56	51%	37	34%	18	16%		
93	85%	74	67%	55	50%	36	33%	17	15%		
92	84%	73	66%	54	49%	35	32%	16	15%		

Per-area practice guide: aim for at least 70% in each area as well as overall (Area I 28 of 40; Areas II and III 25 of 35 each).

Rationales for every item

Section 1

1. ASM-0007 (rev 1) · Best answer: B

A 24-year-old client is brought to a community mental health agency by a sibling. For the past 3 months, the client has heard voices commenting on their actions and has spoken in a disorganized way. There is no history of mood episodes, substance use or medical illness. What is the **MOST** likely diagnosis?

A. Brief psychotic disorder
B. Schizophreniform disorder
C. Schizophrenia
D. Schizoaffective disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Brief psychotic disorder lasts at least 1 day and less than 1 month, with an eventual full return to prior functioning. Three months of symptoms is too long for this diagnosis.

B **Correct.** Schizophreniform disorder has the same core symptom picture as schizophrenia, such as hallucinations and disorganized speech, but the episode lasts at least 1 month and less than 6 months (DSM-5-TR). Three months of symptoms with no mood episodes, substances or medical cause fits it exactly. It beats C because schizophrenia requires continuous signs of the disturbance for at least 6 months. If the symptoms persist past 6 months, the diagnosis would be revised.

C **Incorrect** (trap: Near-miss diagnosis). The symptoms fit schizophrenia, but the duration does not. Schizophrenia requires at least 6 months of continuous signs, counting any prodromal or residual periods.

D **Incorrect** (trap: Near-miss diagnosis). Schizoaffective disorder requires a major mood episode alongside the psychotic symptoms. The stem rules out mood episodes, so this diagnosis does not fit.

Takeaway: Psychosis under 1 month is brief psychotic disorder; 1 to under 6 months is schizophreniform; 6 months or more is schizophrenia.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizophreniform Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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2. INT-1038 (rev 1) · Best answer: A

In a role-play, which technique has the client portray the other person in a conflict while the social worker portrays the client?

A. Role reversal
B. Doubling
C. Mirroring
D. Empty chair

Why each option is right or wrong

A **Correct.** In role reversal, the client takes the part of the other person, such as a parent or partner, while someone else plays the client. Speaking from the other person's position helps the client understand that person's view and hear their own words from the outside (Blatner, 1996). It beats C because in the mirror technique the client steps out of the scene to watch someone portray them, but never takes on the other person's role.

B **Incorrect** (trap: Wrong model or concept). In doubling, someone stands beside the client and voices feelings or thoughts the client may be holding back. The client keeps their own role throughout.

C **Incorrect** (trap: Wrong model or concept). In mirroring, the client watches from outside the scene while another person portrays them, to gain perspective. The client does not play the other person, which is the defining feature here.

D **Incorrect** (trap: Wrong model or concept). In the empty-chair technique, the client speaks to an imagined person in an empty seat. No one else plays the client, and the other person is imagined rather than portrayed by the social worker.

Takeaway: Role reversal: the client plays the other person while someone plays the client, building empathy and perspective on the conflict.

Sources: Blatner, A. (1996). Acting-In: Practical Applications of Psychodramatic Methods (3rd ed.). Springer Publishing, role reversal, doubling, and mirror techniques

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.28 Using role-play in sessions · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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3. ETH-1667 (rev 2) · Best answer: A

A firefighter with PTSD after a fatal blaze has had 4 sessions of prolonged exposure therapy. The client says, 'I can't keep retelling that night. Can we just talk instead?' The client still has nightmares and wants help with them. What should the social worker do **FIRST**?

A. Ask what has made these sessions feel so hard

B. Describe other effective treatments and let the client choose

C. Shift to the supportive talk sessions the client asked for

Why each option is right or wrong

A **Correct.** Before changing a treatment at a client's request, the social worker learns what is driving the request. Retelling can feel overwhelming midway through exposure, and the client's answer shows whether an adjustment or a different trauma-focused therapy would fit; understanding the client's own view comes before planning (Miller & Rollnick, 2023). The client may still decline exposure (NASW Code 1.02). It beats B because describing alternatives is the right next step, but offering them before hearing the client's reasons skips what makes the choice informed.

B **Incorrect** (trap: Right action, wrong time). Clients may refuse a treatment and should hear reasonable alternatives (NASW Code 1.03(a)), and guidelines recommend several trauma-focused therapies, such as cognitive processing therapy and EMDR, that do not rely on repeated detailed retelling (VA/DoD, 2023). This is the right next step once the

social worker knows what makes retelling so hard, so the options match the client's concern.

C **Incorrect** (trap: Good but not best). Moving to supportive talk honors the request, and it may be what the client chooses. Doing it at once, without learning what made retelling unbearable or describing trauma-focused options, leaves the nightmares without an effective treatment.

Takeaway: When a client asks to stop a treatment midway, first learn what drives the request, then present effective options so the choice is informed.

Sources: NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.03(a); VA/DoD (2023). Clinical Practice Guideline for the Management of Posttraumatic Stress Disorder and Acute Stress Disorder. U.S. Department of Veterans Affairs and Department of Defense, recommended individual trauma-focused psychotherapies (CPT, EMDR, PE); Miller & Rollnick (2023). Motivational Interviewing (4th ed.). Guilford Press, engaging: understanding the client's perspective before planning
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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4. ETH-1561 (rev 1) · Best answer: D

A city has begun clearing homeless encampments every month under a new ordinance. During each sweep, clients of a street outreach team lose medications, IDs and benefit papers. The team has replaced these items after every sweep, and the losses keep recurring. What is the **BEST** action for the team at this point?

A. Keep replacing lost items for clients after each sweep
B. Ask police to tell the team before each sweep happens
C. Encourage clients to move their camps outside city limits
D. Present documented harms to city leaders with proposed changes

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Replacing medications and documents is essential direct help, and the team has done it. Repeating it every month treats the effects while the ordinance keeps producing the same losses.

B **Incorrect** (trap: Good but not best). Advance notice would reduce losses and could be one of the changes the team proposes. As an informal request alone, it depends on goodwill and leaves the ordinance's harms unrecorded and unaddressed.

C **Incorrect** (trap: Overrides the client's choice). Steering clients to move shifts the burden onto them and may cut them off from services and support. Where clients live is their choice; the problem the team sees lies in the policy.

D **Correct.** When a policy keeps harming clients, social workers pair direct help with advocacy for policy change that expands choice for vulnerable people (NASW Code 6.04(a), 6.04(b)). The team has met immediate needs, and the harm recurs with each sweep. Bringing documented effects to city leaders, with proposals such as advance notice and safe storage, addresses the cause. It beats B because advance notice would help, but a private arrangement leaves the ordinance unchanged and can be withdrawn at any time; the proposal to leaders can include notice.

Takeaway: Once immediate needs are met and a policy keeps causing the same harm, document its effects and press decision-makers for change.

Sources: NASW Code of Ethics (2021), 6.04(a); NASW Code of Ethics (2021), 6.04(b)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.8 How local, state, federal policy affects service delivery · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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5. ETH-1635 (rev 2) · Best answer: C

A community counseling agency's intake form asks every new client for immigration status. No funder or program rule requires it, and staff say it is just for agency records. Since local enforcement actions made the news, several clients have left the question blank or ended intake early. What is the **BEST** recommendation for the social worker on the quality committee?

A. Keep the question but label it optional
B. Add a note on the form assuring clients of privacy
C. Take the question off the form

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Marking a question optional reduces pressure and works for some demographic items. For immigration status, the question itself can frighten clients in mixed-status families, and the agency would still hold sensitive answers it has no use for.

B **Incorrect** (trap: Misreads law or ethics). Reassurance helps, but confidentiality has legal limits, and a promise cannot fully protect records the agency should not keep. Collecting sensitive status data with no service purpose remains the core problem.

C **Correct.** The Code bars soliciting private information from clients except for compelling professional reasons (NASW Code 1.07(a)). Immigration status is sensitive, no program requires it, and asking everyone is now driving clients away. Removing the question protects privacy and access; status can be asked later, with an explanation, only if a specific benefit requires it. It beats A because an optional question still signals that the agency collects status, which is what deters fearful clients. The Code also names immigration status among the dimensions of oppression social workers must understand (NASW Code 1.05(d)).

Takeaway: Ask about immigration status only when a specific service or eligibility rule requires it; unneeded questions deter fearful clients.

Sources: NASW Code of Ethics (2021), 1.07(a); NASW Code of Ethics (2021), 1.05(d)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.9 Globalization, immigration, refugee status effects on clients and services · **Qualifier** BEST · **Cognitive** application · **Difficulty** 3 of 3

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6. INT-1133 (rev 2) · Best answer: B

Which DBT skills help a client feel less easily upset by caring for sleep, food, exercise, and illness?

A. Opposite action
B. PLEASE skills
C. TIP skills
D. Accumulating positive emotions

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Opposite action is an emotion regulation skill, but it changes an emotion that does not fit the facts by acting against its urge. It does not target sleep, eating, or illness.

B **Correct.** PLEASE is the DBT emotion regulation set for reducing vulnerability to emotion mind: treat physical illness, balance eating, avoid mood-altering substances, balance sleep, and get exercise (Linehan, 2015). A body that is tired, hungry, ill, or intoxicated reacts to triggers faster and more intensely. It beats D because accumulating positive emotions, the neighboring ABC skill, lowers vulnerability by building pleasant experiences rather than by caring for the body; and TIP skills (temperature, intense exercise, paced breathing, paired muscle relaxation) are distress tolerance tools for bringing down acute arousal in a crisis, not routine habits that lower day-to-day vulnerability.

C **Incorrect** (trap: Wrong model or concept). TIP skills include intense exercise, which makes them tempting, but they are a distress tolerance set for quickly lowering extreme arousal in a crisis (temperature, intense exercise, paced breathing, paired muscle relaxation), not a daily self-care routine.

D **Incorrect** (trap: Wrong model or concept). Accumulating positive emotions sits in the same emotion regulation set for reducing vulnerability (the ABC of ABC PLEASE), so it is a close neighbor. It works by scheduling pleasant events and building a life worth living, not by caring for sleep, eating, exercise, and illness.

Takeaway: DBT's PLEASE skills reduce emotional vulnerability by caring for the body: illness, eating, substances, sleep, and exercise.

Sources: Linehan, M. M. (2015). DBT Skills Training Manual (2nd ed.). Guilford Press, Emotion regulation: reducing vulnerability (PLEASE skills)
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.6 Teaching emotion-regulation skills · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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7. INT-1028 (rev 1) · Best answer: A

In a hospital ICU waiting room, a patient's adult child tells the social worker they have not slept in 2 days. They are pacing, their hands are shaking, and they say, 'I can't think straight.' The medical team will meet with the family in half an hour to discuss treatment choices. What should the social worker do **FIRST**?

A. Guide a few minutes of slow, paced breathing
B. Help write down questions for the medical team
C. Explore their fears about the parent's prognosis

Why each option is right or wrong

A **Correct.** When acute stress is high enough to block clear thinking, a brief calming technique comes before problem solving. Slow, even breathing is quick and portable, and Psychological First Aid uses it to help distressed people steady themselves (Brymer et al., 2006). The pacing, shaking, and 'I can't think straight' show arousal that will undercut the family meeting. It beats B because preparing questions is the right next step and will go better once the person is calmer; started now, it asks a flooded mind to plan. Thirty minutes leaves time for both.

B **Incorrect** (trap: Right action, wrong time). Writing down questions is useful problem-focused coping before a stressful meeting, and it should follow. Right now the person says they can't think straight, so planning will work better after a few minutes of calming.

C **Incorrect** (trap: Good but not best). Exploring fears has value later, but opening the hardest topic now may raise arousal further with the meeting 30 minutes away. Brief calming and practical preparation fit this moment better.

Takeaway: When acute stress floods thinking, use a brief calming technique first, then move to problem-focused steps such as preparing questions.

Sources: Brymer, M., et al. (2006). Psychological First Aid: Field Operations Guide (2nd ed.). National Child Traumatic Stress Network and National Center for PTSD, Stabilization and Information on Coping: breathing technique

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.9 Managing stress: client techniques · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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8. ETH-1639 (rev 1) · Best answer: A

A social worker at a residential substance use program is writing an intake summary for a 24-year-old client. The client describes being made to sell sex by an older partner starting at age 16. An old court record lists 'juvenile prostitution arrest, age 16.' What is the **MOST** appropriate way to document this history?

A. Describe it as sexual exploitation that began at age 16

B. Copy the court record's wording for consistency

C. Leave the history out to protect the client from future stigma

Why each option is right or wrong

A **Correct.** Under federal law, a minor induced into commercial sex is a trafficking victim, whatever an old charge said (22 U.S.C. 7102(11)(A)). The Code calls for accurate, respectful language in all communication about clients (NASW Code 1.12). Describing the history as exploitation that began at 16 is accurate and nonstigmatizing, and it flags trauma that matters for treatment. It beats C because the history bears on substance use and trauma care; leaving it out gives up accuracy to avoid a stigma that better wording already prevents.

B **Incorrect** (trap: Misreads law or ethics). Matching outside records can seem objective, but the court label treats a 16-year-old trafficking victim as an offender. Copying it carries a stigmatizing frame, at odds with federal law's view of minors as victims, into the treatment record.

C **Incorrect** (trap: Good but not best). Protecting clients from stigma is a valid goal, and records should hold only relevant details. This history bears directly on trauma and substance use treatment, so it belongs in the record in respectful terms.

Takeaway: Document the exploitation of a minor as victimization in accurate, respectful terms, not with stigmatizing labels carried over from old records.

Sources: Trafficking Victims Protection Act of 2000, 22 U.S.C. 7102(11)(A); NASW Code of Ethics (2021), 1.12

Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.7 Exploitation's cultural impact on marginalized communities (trafficking, MMIW) · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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9. ASM-1037 (rev 2) · Best answer: B

A school social worker meets with a 17-year-old referred for 'seeing things.' The student describes vivid dreamlike images while drifting off to sleep, falls asleep in class almost daily despite 9 hours of sleep at night, and has had knees buckle suddenly while laughing with friends. These problems began 6 months ago, and grades are dropping. What is the social worker's **BEST** recommendation?

A. Refer the student for a psychiatric evaluation of possible psychosis
B. Refer the student for evaluation by a sleep medicine specialist
C. Screen the student for depression as a cause of sleep problems

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Images that come only while falling asleep are common and are not, by themselves, a sign of psychosis. With sleep attacks and laughter-triggered weakness, a psychosis workup would send care in the wrong direction.

B **Correct.** Daytime sleep attacks despite enough sleep, dreamlike images when falling asleep (hypnagogic hallucinations) and sudden muscle weakness triggered by laughter (cataplexy) together point to narcolepsy, a sleep-wake disorder that often begins in adolescence (DSM-5-TR). A sleep medicine specialist can confirm it, usually with overnight and daytime sleep testing, so this referral fits. It beats C because depression can cause sleepiness but does not explain cataplexy, the most specific clue here.

C **Incorrect** (trap: Good but not best). Depression can cause daytime sleepiness and is worth screening for in a teen with falling grades. It does not explain sudden knee buckling with laughter or dream images at sleep onset, which point to a sleep disorder.

Takeaway: Sleep attacks plus cataplexy or vivid sleep-onset images suggest narcolepsy; refer to sleep medicine rather than treating them as psychosis.

Sources: DSM-5-TR (APA, 2022), Sleep-Wake Disorders: Narcolepsy
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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10. ASM-0107 (rev 1) · Best answer: B

A 48-year-old client says work and home life feel empty. The client recently quit mentoring new employees and dropped out of a community youth program, saying, 'Nothing I do will matter to anyone after me.' The client spends most free time alone watching TV. According to Erikson, which psychosocial crisis is the client **MOST** likely facing?

A. Intimacy versus isolation
B. Generativity versus stagnation
C. Integrity versus despair
D. Identity versus role confusion

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Intimacy versus isolation is the young-adult task of forming close, committed relationships. The client spends time alone, but the core concern is a lack of lasting contribution to others, the hallmark of midlife.

B **Correct.** Erikson's midlife crisis, generativity versus stagnation, centers on contributing to the next generation through parenting, mentoring, work or community (Erikson, 1963). At 48, this client has withdrawn from mentoring and youth work and feels nothing will outlast them, which signals stagnation. It beats C because integrity versus despair is the late-life task of looking back over a whole life, usually after 65, while this client is in midlife and focused on contributing going forward. Work might aim at rebuilding meaningful contribution.

C **Incorrect** (trap: Wrong model or concept). Integrity versus despair is the late-life review of whether one's life had meaning, typically after 65. It is the nearest neighbor here, but this client is 48 and focused on contributing to future generations.

D **Incorrect** (trap: Wrong model or concept). Identity versus role confusion is the adolescent task of forming a stable sense of self. A 48-year-old questioning their impact on others is facing a later, midlife crisis.

Takeaway: Generativity versus stagnation is Erikson's midlife task of contributing to the next generation; integrity versus despair is the late-life review.

Sources: Erikson, E. H. (1963). Childhood and Society (2nd ed.). W. W. Norton, eight ages of man: generativity vs. stagnation

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.16 Typical vs. atypical development across the lifespan · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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11. INT-1126 (rev 1) · Best answer: C

PRETEST — unscored on this form; do not count it

A young adult in weekly video sessions describes finishing a first art exhibit, then pauses and looks eagerly at the social worker for a reaction. In earlier sessions, the client seemed deflated when accomplishments drew little response. The social worker uses a self psychology approach. Which response is **MOST** consistent with this approach?

A. Point out that the client seems to need approval to feel worthwhile

B. Keep a neutral stance so the wish for praise can be analyzed

C. Respond warmly to the pride the client is showing

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Naming the wish for approval as a problem treats the need as pathology to confront. That stance fits Kernberg's interpretive approach to narcissism (Kernberg, 1975), not Kohut's, and it is likely to land as criticism with a client who already deflates easily.

B **Incorrect** (trap: Wrong model or concept). Withholding a response so the wish can be analyzed reflects classical neutrality and abstinence. Self psychology instead lets this kind of transference develop, and a flat response here would repeat the deflation the client has shown before.

C **Correct.** Kohut's self psychology treats the need to have one's accomplishments received with delight, a mirroring selfobject need, as a normal developmental need rather than a defect (Kohut, 1971). The client is looking to the worker for that response and has deflated when it was missing. Warm, attuned interest in the client's pride meets the need and supports a more stable sense of self over time. It beats A because

labeling the need for approval as a problem is a confrontive stance that this model avoids and that the client would likely hear as shaming.

Takeaway: In self psychology, meet a client's mirroring needs with attuned, warm responsiveness rather than confronting them or withholding a response.

Sources: Kohut, H. (1971). The Analysis of the Self. International Universities Press, mirror transference; Kernberg, O. F. (1975). Borderline Conditions and Pathological Narcissism. Jason Aronson, pathological narcissism: interpretation of grandiosity

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** MOST · **Cognitive** application · **Difficulty** 1 of 3

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12. ETH-1059 (rev 1) · Best answer: C

A former client posts an online review of a social worker's private practice. The review claims the social worker 'never returned calls' and mentions the client's divorce. Phone logs show the calls were returned the same day. What is the **BEST** way for the social worker to handle the review?

A. Reply with the call dates without naming the client

B. Call the former client and ask to have the review removed

C. Refrain from any reply that confirms a client relationship

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Leaving out the name does not protect confidentiality: any factual reply confirms the person was a client and discloses service details. Protecting the worker's reputation does not justify the disclosure (1.07(r)).

B **Incorrect** (trap: Boundary slip or collusion). Contacting a former client to request removal pressures that person and serves the worker's interests rather than the client's. It risks a further boundary problem and can escalate the conflict.

C **Correct.** Social workers must not post identifying or confidential client information online (NASW Code 1.07(r); Tech Standards 2.09). Even a reply that omits the name confirms the reviewer was a client and reveals details of services, and the client's own public post does not waive the worker's duty. C beats A because correcting the facts with call dates is the natural impulse, yet it discloses the relationship. At most, a general statement of practice policies that refers to no one may be posted.

Takeaway: Never confirm a client relationship when answering online reviews; a client's public post does not waive confidentiality.

Sources: NASW Code of Ethics (2021), 1.07(r); NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 2.09

Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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13. INT-1043 (rev 1) · Best answer: B

During a family session about a teen's recent arrest, one parent speaks in a flat, even tone. The parent cites studies on teen brain growth and says feelings 'only cloud the issue.' According to Satir, which communication stance is this parent **MOST** likely showing?

A. Blaming
B. Super-reasonable
C. Placating
D. Irrelevant

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Blaming protects the self by attacking or finding fault with others. This parent is detached and impersonal, focusing on data rather than accusing the teen or anyone else.

B **Correct.** Satir described four stress stances, each discounting something: placating discounts the self, blaming discounts others, super-reasonable (called computing in her early work) discounts both self and others and keeps only facts and rules, and irrelevant discounts self, others, and context. Congruent communication is the goal (Satir, 1972). Citing research in a flat tone and dismissing feelings is the super-reasonable stance. It beats A because blaming involves fault-finding and attack, while this parent stays detached and impersonal.

C **Incorrect** (trap: Wrong model or concept). Placating gives up the self to keep others happy, through agreeing and apologizing. This parent is not seeking approval; they set feelings aside in favor of facts.

D **Incorrect** (trap: Wrong model or concept). The irrelevant stance distracts from the topic altogether by changing the subject or joking. This parent stays on topic but strips out feelings, which fits a different stance.

Takeaway: Satir's super-reasonable stance relies on facts and logic while discounting the feelings of self and others; congruence is the healthy goal.

Sources: Satir, V. (1972). Peoplemaking. Science and Behavior Books, communication stances under stress
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.21 Family therapy models and interventions · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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14. INT-1161 (rev 1) · Best answer: B

In a hospital emergency department, a social worker meets an adult patient who was sexually assaulted by an acquaintance about 6 hours ago. The patient is tearful, wants to shower and go home, and does not want police involved. The patient asks the social worker, 'Is there any reason I should stay?' What is the **BEST** response?

A. Support the decision to leave and arrange a call tomorrow
B. Explain the care and exam options that need no police report
C. Encourage a police report to protect others from the acquaintance

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Respecting the wish to leave matters, and a follow-up call is good practice. But the patient asked whether to stay, and answering without the time-sensitive options leaves the decision uninformed.

B **Correct.** Trauma-informed care restores control to survivors through voice and choice, and real choice depends on accurate information (SAMHSA, 2014). The patient is asking for exactly that. Some care after an assault, such as preventive medication and evidence collection, is time-sensitive, and federal guidance says a medical forensic exam can be done without involving police (National Protocol, U.S. Department of Justice, 2013). Explaining these options lets the patient decide with full facts. It beats A because supporting the wish to leave without answering the question withholds what the patient needs to choose.

C **Incorrect** (trap: Overrides the client's choice). Urging a report pressures the survivor toward a step they have declined and puts others' interests ahead of the patient's control. Whether to report is the patient's decision.

Takeaway: Trauma-informed care supports survivor choice by giving full, accurate options, including time-sensitive care that does not require a police report.

Sources: SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), Six key principles: empowerment, voice and choice; U.S. Department of Justice, Office on Violence Against Women (2013). A National Protocol for Sexual Assault Medical Forensic Examinations: Adults/Adolescents (2nd ed.), patient-centered care; exams without law enforcement involvement

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.1 Trauma-informed care across trauma types (complex, vicarious) · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 1 of 3

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15. INT-1505 (rev 1) · Best answer: A

A social worker at a community mental health center canceled two sessions in a row, each with only an hour's notice. At the next session, a client who is usually talkative gives one-word answers and looks at the floor. What is the social worker's **BEST** response?

A. Name the late cancellations and take responsibility for them

B. Ask the client why they seem so quiet today

C. Move ahead with the planned agenda to restore a sense of routine

Why each option is right or wrong

A **Correct.** When the worker's own behavior has likely strained the relationship, repair starts with the worker raising it and owning it. Two last-minute cancellations are a plausible cause of the client's withdrawal; naming them openly, without excuses, models accountability and invites the client's reaction (Safran & Muran, 2000). It beats B because asking the client to explain the silence puts the burden on the client and sidesteps the strain the worker caused, and a 'why' question tends to provoke defensiveness.

B **Incorrect** (trap: Misuses the technique). Inviting the client's view is often a good opening, but this version asks 'why,' which can feel accusatory, and it ignores the likely cause the worker already knows. Open curiosity fits best after the worker owns the cancellations.

C **Incorrect** (trap: Good but not best). Routine can steady some clients, but pressing on past a visible strain leaves it unaddressed and can deepen the client's sense that the relationship is unreliable.

Takeaway: When the worker's own lapse strains the alliance, raise it and take responsibility before pressing on or asking the client to explain.

Sources: Safran, J. D., & Muran, J. C. (2000). Negotiating the Therapeutic Alliance: A Relational Treatment Guide. Guilford Press, rupture resolution: therapist acknowledging own contribution
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.2 Building and sustaining the helping relationship (empathy, rapport, acceptance) · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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16. ASM-1119 (rev 1) · Best answer: C

A 14-year-old tells a school social worker they have felt 'huge' since joining a dance team where teammates trade diet tips. At home, a parent often criticizes their own weight, and the student follows several fitness accounts online. Which framework **BEST** accounts for these sources of the student's body dissatisfaction?

A. Objectification theory
B. Social comparison theory
C. Tripartite influence model
D. Social learning theory

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Objectification theory explains how being viewed as a body to be looked at leads people to watch their own appearance. It does not frame the combined roles of a parent, peers and media named here.

B **Incorrect** (trap: Wrong model or concept). Social comparison is part of the picture, since comparing oneself with dancers or online figures can raise dissatisfaction. It describes a mechanism, not the three sources the student names.

C **Correct.** The tripartite influence model holds that three sources, parents, peers and media, shape body image, working through internalized appearance ideals and appearance comparison (van den Berg et al., 2002). The student names all three: a parent's weight criticism, teammates' diet talk and fitness accounts online. It beats B because social comparison describes one way these sources act but does not organize the three sources. Assessing each source helps target family guidance, peer context and media use.

D **Incorrect** (trap: Wrong model or concept). Social learning explains how the student might copy a parent's dieting or weight talk. It fits the family piece but does not account for peer and media pressures together.

Takeaway: The tripartite influence model frames body image as shaped by parents, peers and media, through internalization and appearance comparison.

Sources: van den Berg, P., Thompson, J. K., Obremski-Brandon, K., & Coovert, M. (2002). The Tripartite Influence model of body image and eating disturbance: A covariance structure modeling investigation testing the mediational role of appearance comparison. Journal of Psychosomatic Research, 53(5), 1007-1020, whole article
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.10 Influences on self-image and body image · **Qualifier** BEST · **Cognitive** recall · **Difficulty** 3 of 3

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17. ASM-1517 (rev 3) · Best answer: B

A post-adoption social worker meets with parents whose 14-year-old was adopted from South Korea as an infant. The teen recently asked to take Korean language classes and join a Korean American youth group. The parents, who have spent most of the session voicing their fear that the teen is rejecting them, now ask the social worker to help talk the teen out of it. What is the **BEST** response?

A. Ask the parents to say more about their fear of rejection
B. Explain that exploring heritage is a normal part of identity development
C. Refer the family to therapy for adoption attachment issues
D. Suggest that the teen postpone these activities until the parents feel ready

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Inviting the parents to talk about their fear is useful early in the conversation, and they have already done so. Asking for more now delays the developmental information they need before they act to discourage the teen.

B **Correct.** Ethnic identity development in adolescence typically includes a period of actively exploring one's heritage before a secure identity forms (Phinney, 1990). For a transracially adopted teen, wanting language classes and a Korean American peer group is a healthy developmental step, not a rejection of the adoptive family. It beats A because the parents have already described their fear; exploring it further leaves their request to discourage healthy development unanswered. Reframing the behavior opens the way for the parents to support the teen's exploration.

C **Incorrect** (trap: Hands off too soon). Family therapy for attachment problems fits signs of a disturbed parent-child bond. Nothing here suggests an attachment problem; a teen exploring heritage is a typical developmental step, so referral pathologizes it.

D **Incorrect** (trap: Overrides the client's choice). Delaying the activities to ease the parents' discomfort puts their needs ahead of the teen's healthy identity development and signals that the teen's heritage threatens the family.

Takeaway: Adolescent exploration of ethnic or cultural heritage is typical identity development, including for transracial adoptees; help families support it rather than discourage it.

Sources: Phinney, J. S. (1990). Ethnic identity in adolescents and adults: Review of research. Psychological Bulletin, 108(3), 499-514, stages of ethnic identity development: exploration
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.19 Racial, ethnic, cultural, spiritual development across the lifespan · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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18. ETH-1160 (rev 1) · Best answer: B

PRETEST — unscored on this form; do not count it

When should a social worker who uses social media professionally review their social media policy with clients?

A. When a client first sends a connection request
B. At the first interview, updating it as needed
C. Once a working alliance has formed

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Explaining the policy only after a client sends a request makes it reactive, and the client may feel rejected. The standards call for setting expectations at the start, so a request can be declined by pointing to a known policy.

B **Correct.** Social workers who use social media professionally should have a written policy, share it with clients, review it at the first interview and update it as circumstances change (Tech Standard 2.10). The Code likewise asks workers to discuss their technology policies with clients (NASW Code 1.03(e)). Early review sets expectations about connection requests, messaging, online searches and reviews before a problem arises. It beats C because waiting for an alliance to form leaves common boundary questions unaddressed when they are most likely to come up.

C **Incorrect** (trap: Right action, wrong time). Waiting for a working alliance can seem gentler, but clients often reach out online early. The policy belongs with other informed-consent topics at the first interview, then gets updated as needed.

Takeaway: Share a written social media policy at the first interview, alongside informed consent, and update it as circumstances change.

Sources: NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 2.10; NASW Code of Ethics (2021), 1.03(e)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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19. ETH-1528 (rev 1) · Best answer: C

A social worker in private practice has seen a client for depression for 4 months. The depression has resolved, but the client now spends hours on checking rituals. The social worker is not trained in exposure and response prevention, and two consultations confirmed the client needs it. The client wants to keep meeting with the social worker. What should the social worker do **NEXT**?

A. Attend a weekend workshop and then start the treatment
B. Continue supportive counseling, as the client prefers
C. Help the client transfer to a provider of that treatment

Why each option is right or wrong

A **Incorrect** (trap: Outside the social work role). A single workshop does not create competence in a new treatment. Social workers use a new method only after appropriate study, training, consultation and supervision (NASW Code 1.04(b)). Treating the client as a practice case risks harm when trained clinicians are available.

B **Incorrect** (trap: Good but not best). Respecting the client's preference matters, and the attachment deserves discussion. But continuing sessions that do not treat the main problem leaves the client without effective care. Supportive work fits only if the client declines the referral after a full discussion.

C **Correct.** Social workers refer when another professional's specialized expertise is needed to serve the client fully, and they make the transfer orderly (NASW Code 1.16(a), (b)). The depression has resolved, the remaining need is exposure and response prevention, and two consultations confirmed it is beyond the social worker's

training. A planned transfer includes discussing the reasons, sharing records with consent and holding a closing session. It beats B because honoring the wish to stay would leave the main problem untreated; the client still chooses among qualified providers.

Takeaway: When a client needs a treatment beyond the worker's training, refer and arrange an orderly transfer rather than continuing ineffective care.

Sources: NASW Code of Ethics (2021), 1.16(a)-(b); NASW Code of Ethics (2021), 1.04(b)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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20. INT-1113 (rev 3) · Best answer: C

A client in weekly outpatient therapy has begun calling the agency 4 to 6 times a day to talk through daily conflicts. The calls started 2 weeks ago, after months with no contact between sessions. They are not about emergencies, and front desk staff are struggling to keep up. What should the social worker do **FIRST**?

A. Return each call promptly to protect the working relationship
B. Set a scheduled check-in time and crisis plan with the client
C. Ask the client what has changed for them recently

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). Returning every call feels supportive but teaches the client that unlimited contact is available and can build dependence. It also sets up a pattern the worker cannot sustain, which risks a more painful rupture later (Reamer, 2020).

B **Incorrect** (trap: Right action, wrong time). Clear, predictable limits on contact, a set check-in plus crisis resources, protect both the client and the work (Linehan, 1993), and they will likely be needed. Setting them before learning why the calls suddenly began skips assessment and could miss a new stressor or worsening symptoms.

C **Correct.** Assessment comes before intervention. A sudden change, frequent calls after months without contact between sessions, signals that something has shifted, such as a new stressor, rising symptoms or a problem the client has not yet named (Hepworth et al.). Exploring the calls with the client shows what they provide and whether risk has changed, and lets the two design limits that still meet the need. It beats B because a check-in schedule is the likely next step, but setting it before understanding the calls could miss a worsening problem and feel like rejection.

Takeaway: When a client's contact pattern suddenly changes, explore what prompted it before setting limits; then agree on a predictable check-in plan.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Assessment: exploring and understanding problems and strengths; Linehan, M. M. (1993). Cognitive-Behavioral Treatment of Borderline Personality Disorder. Guilford Press, observing limits procedures; Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, managing boundaries; dependence and availability
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.4 Setting limits and boundaries with clients · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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21. ASM-1509 (rev 2) · Best answer: A

In a residential treatment program, a roommate earns the early weekend pass that a resident had been working toward. Over the next week, the resident praises the roommate loudly in every group, buys the roommate snacks and calls the roommate 'the kindest person here.' Staff notice the praise sounds forced, and the resident tenses whenever the pass comes up. Which defense mechanism is the resident **MOST** likely using?

A. Reaction formation
B. Idealization
C. Altruism

Why each option is right or wrong

A **Correct.** Reaction formation keeps an unacceptable feeling out of awareness by displaying exaggerated feelings and behavior of the reverse kind (Vaillant, 1992). The resident lost a hoped-for pass to the roommate, then shifted to loud, forced praise and gifts and tenses when the pass comes up, signs of defended envy rather than genuine warmth. It beats B because idealization assigns exaggerated positive qualities to someone admired or needed, without the hidden resentment and overdone, forced quality that drive this display.

B **Incorrect** (trap: Wrong model or concept). Idealization attributes exaggerated perfection to another person, often someone the client depends on. The praise fits partly, but its timing after the lost pass, the tension when the pass comes up and its forced, overdone quality point to a defense against hidden resentment.

C **Incorrect** (trap: Wrong model or concept). Altruism is a mature defense that meets one's own needs through genuine, constructive service to others. The gifts look generous, but they follow a painful loss to the roommate and feel forced, which marks a defensive reversal rather than mature helping.

Takeaway: Reaction formation shows exaggerated, often forced behavior that reverses a hidden unacceptable feeling; altruism and idealization lack that hidden reversal.

Sources: Vaillant, G. E. (1992). Ego Mechanisms of Defense: A Guide for Clinicians and Researchers. American Psychiatric Press, reaction formation; altruism; idealization
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.11 Defense mechanisms and how they shape behavior and relationships · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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22. INT-1151 (rev 1) · Best answer: C

Ten days after a fatal crane accident, a social worker on an employer's crisis response team meets again with an employee who saw it happen. The employee sleeps about 2 hours a night, eats little, and has not been able to drive near the site. The employee says the breathing exercises, walks, and talks with their partner that they planned together last week have not helped. What should the social worker do **NEXT**?

A. Review the coping plan again and meet next week
B. Ask the employee to go over the accident in more detail
C. Link the employee to a mental health provider for assessment
D. Encourage a gradual return to the site alongside a coworker

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Reviewing coping and following up fit earlier in crisis work, and that step was done last week. Repeating a plan the employee says has not worked delays the evaluation that persistent impairment calls for.

B **Incorrect** (trap: Misuses the technique). A detailed retelling resembles single-session debriefing, which has not been shown to prevent PTSD and is not part of crisis stabilization. The employee needs further evaluation, not more recounting.

C **Correct.** Crisis intervention ends by checking whether functioning is returning and referring when it is not; the last step of Everly's SAFER-R model is recovery or referral (Everly, 1996). Ten days on, the employee is barely sleeping or eating, is avoiding the site, and the coping plan built last week has not helped. Those facts call for evaluation by a mental health professional who can assess for an acute stress reaction and offer treatment. It beats A because repeating a plan that has already failed delays care while impairment continues.

D **Incorrect** (trap: Right action, wrong time). Gradual return to avoided places can be part of later trauma treatment planned by a qualified clinician. Pushing it now, while the employee is barely sleeping or eating, comes before the evaluation that should happen first.

Takeaway: When crisis intervention has not restored functioning, the final step is referral for mental health evaluation, not repeating stabilization.

Sources: Everly, G. S., Jr. (1996). A rapid crisis intervention technique for law enforcement: The SAFER(R) model. In J. T. Reese & R. Solomon (Eds.), Organizational Issues in Law Enforcement. Federal Bureau of Investigation, SAFER-R final step: recovery or referral

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.5 Crisis intervention and crisis treatment approaches · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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23. ASM-1665 (rev 2) · Best answer: C

A school-based health center social worker meets alone with a 15-year-old for a first visit about headaches and missed classes. The visit opened with a review of what stays private and when information must be shared, which the teen restated accurately. The social worker needs to ask about substance use and sexual activity during this visit. What should the social worker do **NEXT**?

A. Ask about substance use and sexual activity right away

B. Wait to cover those topics until the teen raises them

C. Begin with home life and daily activities, then risk topics

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). These questions belong in the visit, but asking them first, before any rapport on easier topics, often yields guarded answers. Starting with home and school makes later honest answers more likely.

B **Incorrect** (trap: Ignores risk). Teens rarely bring up substance use or sexual activity on their own, and waiting can leave real risks unassessed. The social worker raises these topics routinely, after easing in with less sensitive areas.

C **Correct.** Psychosocial interviews with adolescents usually move from important but less threatening topics, such as home, school and activities, toward more personal ones

such as drugs and sexuality (Klein, Goldenring & Adelman, 2014). With confidentiality already explained, starting with everyday areas builds comfort and may reveal stresses tied to the headaches and missed classes. The more sensitive questions then follow in the same visit. It beats A because opening with the most personal topics before any rapport tends to produce guarded answers.

Takeaway: With adolescents, explain confidentiality, then move from less threatening topics such as home and school toward drugs and sexuality.

Sources: Klein, D. A., Goldenring, J. M., & Adelman, W. P. (2014). HEEADSSS 3.0: The psychosocial interview for adolescents updated for a new century fueled by media. Contemporary Pediatrics, 31(1), 16-28, interview sequence and confidentiality

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.7 Eliciting sensitive or confidential information (substance use, sexual abuse) · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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24. ETH-1096 (rev 1) · Best answer: C

During a hospital case review after a patient's suicide attempt, two staff members say a social work colleague ignored the patient's call the day before. The social worker covered that unit and had read the record, which shows the colleague returned the call within an hour and documented a safety check. What is the **MOST** appropriate action?

A. Wait to raise the documented facts until the review asks

B. Tell the colleague privately about the comments

C. Give the review leader the documented call details

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Waiting may seem neutral, but the review could reach conclusions based on a false account. The duty to defend a wrongly accused colleague calls for offering the facts, not waiting to be asked.

B **Incorrect** (trap: Good but not best). Letting the colleague know is supportive, but it leaves the review relying on inaccurate information, and the colleague may not be present to respond.

C **Correct.** Social workers should defend and assist colleagues who are unjustly charged with unethical conduct (NASW Code 2.10(e)). The social worker has direct knowledge, from the record, that the accusation is false, and the review is forming conclusions now. Giving the documented facts to the person leading the review corrects the account through the proper channel. C beats B because warning the colleague helps them personally but leaves the false account standing in the review.

Takeaway: When a colleague is wrongly accused and you have direct knowledge, bring the facts to the proper review process.

Sources: NASW Code of Ethics (2021), 2.10(e)

Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.2 Legal-ethical duties to clients, colleagues, profession, and society · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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25. ASM-1081 (rev 1) · Best answer: A

At a home visit 2 weeks after delivery, a parent has barely slept for 4 days and speaks rapidly about 'signs' that the baby is evil. When asked directly about harm, the parent calmly says the baby 'needs to be sent back to God' and mentions the bathtub. The parent's partner is in the next room. What should the social worker do **NEXT**?

A. Get the parent to a psychiatric emergency service now
B. Book a perinatal psychiatry appointment for this week
C. Explore the parent's spiritual beliefs about the baby
D. Ask the partner to stay with the baby until follow-up

Why each option is right or wrong

A **Correct.** Postpartum psychosis is a psychiatric emergency linked to suicide and infanticide, and hospitalization is generally indicated (ACOG, 2023). Severe sleep loss, rapid speech, delusions about the infant and a calmly stated method show the direct assessment is done and the danger is high. The parent needs emergency evaluation today, with the partner staying with the baby in the meantime. It beats D because supervision alone leaves an acute, treatable psychosis untreated and the infant still exposed to a parent who may act on the delusion.

B **Incorrect** (trap: Ignores risk). Specialized perinatal care fits postpartum depression or anxiety without immediate danger. Psychosis with a stated way of harming the infant cannot wait days for an appointment.

C **Incorrect** (trap: Right action, wrong time). Cultural and religious meaning deserves respect. These beliefs, though, arose with days of sleeplessness and rapid speech after delivery and include a method of harm, which marks psychosis needing emergency care rather than further exploration.

D **Incorrect** (trap: Ignores risk). Keeping the baby with the partner is a useful short-term protective step. As the only response, it leaves the parent's psychosis untreated and relies on round-the-clock supervision that cannot be guaranteed.

Takeaway: Postpartum psychosis with thoughts of harming the infant is a psychiatric emergency: arrange same-day emergency evaluation.

Sources: American College of Obstetricians and Gynecologists (2023). Clinical Practice Guideline No. 5: Treatment and Management of Mental Health Conditions During Pregnancy and Postpartum, Postpartum psychosis
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.1 Assessing risk of harm to self or others · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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26. INT-1529 (rev 1) · Best answer: D

At a reentry program, a social worker and client sign an agreement to work on the two problems the client ranked highest, over 8 sessions. Each session opens with a review of progress on those problems. Each session closes with the client choosing an action step, rehearsing it, and planning for obstacles. Which approach is the social worker **MOST** likely using?

A. Solution-focused brief therapy
B. Problem-solving therapy
C. Behavioral activation
D. Task-centered practice

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). SFBT is also brief, but it builds on exceptions, scaling and the miracle question to construct a preferred future. It does not contract around ranked problems or rehearse obstacle plans for between-session steps.

B **Incorrect** (trap: Wrong model or concept). Problem-solving therapy is structured too, but its core is training a general skill set: problem orientation, defining problems, and generating and weighing options. Ranked target problems, a fixed agreement and rehearsed steps point to the task-centered model.

C **Incorrect** (trap: Wrong model or concept). Behavioral activation schedules rewarding and valued activities to lift depressed mood. The stem describes no mood target or activity monitoring; it describes client-ranked problems worked through contracted steps.

D **Correct.** Task-centered practice is brief and time-limited, works on a few target problems the client selects and ranks, uses an explicit contract, and moves forward through tasks planned and rehearsed in session, with attention to obstacles (Reid & Epstein, 1972). Every feature in the stem matches. It beats B because problem-solving therapy also follows structured steps but centers on teaching a general problem-solving method, not on contracting around ranked target problems with between-session tasks (D'Zurilla & Nezu, 2007).

Takeaway: Task-centered practice: brief, contracted work on a few client-ranked target problems through rehearsed between-session tasks.

Sources: Reid, W. J., & Epstein, L. (1972). Task-Centered Casework. Columbia University Press, target problems, contract and task implementation; D'Zurilla, T. J., & Nezu, A. M. (2007). Problem-Solving Therapy: A Positive Approach to Clinical Intervention (3rd ed.). Springer, rational problem-solving skills

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.16 Structured problem-solving approaches · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

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27. ASM-1022 (rev 1) · Best answer: C

A social worker at an assisted living facility meets with a 78-year-old resident who has no psychiatric history. Over 6 weeks the resident has stopped eating, sleeps poorly and says, 'I am worthless.' The resident now insists their organs are rotting as punishment for old sins. Staff report that this belief appeared only after the low mood began. What is the **MOST** likely diagnosis?

A. Delusional disorder, somatic type

B. Schizoaffective disorder, depressive type

C. Major depressive disorder with psychotic features

D. Major neurocognitive disorder due to Alzheimer's disease

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Delusional disorder involves fixed beliefs without major mood episodes dominating the picture. The rotting-organs belief arose within a depressive episode with worthlessness and sleep and appetite loss, which points to a mood disorder.

B **Incorrect** (trap: Near-miss diagnosis). Schizoaffective disorder requires at least 2 weeks of delusions or hallucinations with no mood episode. This belief appeared only after the low mood began, so the psychosis is tied to the depression.

C **Correct.** Major depressive disorder with psychotic features applies when delusions or hallucinations occur only during a major depressive episode (DSM-5-TR). Six weeks of poor appetite and sleep and feelings of worthlessness, followed by a belief that the body is rotting as punishment, fit a mood-congruent delusion. Refusing food makes this presentation serious and in need of prompt treatment. It beats B because schizoaffective disorder requires at least 2 weeks of psychosis without a mood episode, and here the belief appeared only after the low mood began.

D **Incorrect** (trap: Near-miss diagnosis). Alzheimer's disease develops gradually with memory and thinking decline. A 6-week change dominated by low mood, worthlessness and a guilt-themed belief points to depression with psychotic features.

Takeaway: Delusions occurring only during a major depressive episode, often with guilt or decay themes, indicate major depressive disorder with psychotic features.

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Major Depressive Disorder; DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizoaffective Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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28. INT-1078 (rev 1) · Best answer: D

A crisis that arises when a normal, expected life change, such as becoming a parent or retiring, overwhelms a person's usual coping is which type?

A. Situational crisis
B. Existential crisis
C. Environmental crisis
D. Developmental crisis

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Situational crises follow sudden, uncommon events outside a person's control, such as an assault, a crash or a layoff. Becoming a parent or retiring is an expected life change, so the crisis comes from the transition, not an unexpected event.

B **Incorrect** (trap: Wrong model or concept). Existential crises center on questions of purpose, meaning, freedom or responsibility, such as feeling that one's life has had no impact. A crisis set off by a new role at a normal life stage is the hallmark of a developmental crisis instead.

C **Incorrect** (trap: Wrong model or concept). Environmental (ecosystemic) crises are set off by natural or human-caused disasters that affect many people, such as floods or wildfires. An expected personal life change is not a community-wide event.

D **Correct.** A developmental crisis arises from a normal, expected life transition, such as the birth of a child, starting college, a midlife change or retirement, when the new demands outstrip usual coping (James & Gilliland, 2017). The defining feature is that the trigger is a normal life stage that most people pass through, and the

disorganization comes from the demands of the new role. It beats A because a situational crisis follows a sudden, unusual event, such as an accident or a job loss; an expected life change, however overwhelming, is developmental.

Takeaway: Developmental crises stem from expected life transitions; situational ones from sudden, unusual events; existential from meaning; ecosystemic from disasters.

Sources: James, R. K., & Gilliland, B. E. (2017). Crisis Intervention Strategies (8th ed.). Cengage, types of crisis: developmental, situational, existential, ecosystemic
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.5 Crisis intervention and crisis treatment approaches · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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29. ASM-1638 (rev 1) · Best answer: A

Which theory of aging holds that as people see their remaining time as limited, they narrow their social circle to the relationships that matter most to them?

A. Socioemotional selectivity theory
B. Disengagement theory
C. Activity theory

Why each option is right or wrong

A **Correct.** Carstensen's socioemotional selectivity theory proposes that when people perceive time as limited, as often happens in later life, they give priority to emotionally meaningful goals and let peripheral ties fade while keeping the closest ones. A smaller circle can therefore reflect choice rather than loss. It beats B because disengagement theory describes a mutual withdrawal of older adults and society from each other, not a chosen focus on valued relationships (Carstensen, Isaacowitz & Charles, 1999).

B **Incorrect** (trap: Wrong model or concept). Disengagement theory held that older adults and society naturally withdraw from each other. It describes a general retreat, not a deliberate narrowing toward the closest relationships based on how much time a person feels is left.

C **Incorrect** (trap: Wrong model or concept). Activity theory holds that older adults stay satisfied by keeping up activities and replacing lost roles with new ones. It predicts broad engagement, not a shrinking circle of close ties.

Takeaway: Socioemotional selectivity: perceived limited time leads people to focus on close relationships, so a smaller circle in later life can be adaptive.

Sources: Carstensen, L. L., Isaacowitz, D. M., & Charles, S. T. (1999). Taking time seriously: A theory of socioemotional selectivity. American Psychologist, 54(3), 165-181, whole article
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.18 Aging's impact on individuals and family systems · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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30. INT-1688 (rev 1) · Best answer: C

A clinical director supervises an associate who publicly questioned the director's plan at a team meeting. Since then, the director has drafted lower ratings for the associate's annual evaluation than the case reviews support. On reflection, the director recognizes resentment about the meeting as the source. The evaluation is due in 5 days. What should the director do **NEXT**?

A. Keep monitoring the reaction over the coming month
B. Tell the associate about the resentment to clear the air
C. Seek consultation on the reaction before finishing the evaluation
D. Transfer the evaluation and supervision to another manager

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Self-monitoring was the right start, and the director has done it: the reaction is identified and already shaping draft ratings. With the evaluation due in 5 days, further watching leaves biased ratings in place.

B **Incorrect** (trap: Boundary slip or collusion). Disclosing resentment shifts the supervisor's emotional work onto a supervisee who holds less power and whose evaluation is pending. A conversation about the meeting itself can come later, once the director's reaction is managed.

C **Correct.** A supervisor's countertransference, an emotional reaction to a supervisee, can distort evaluation, and supervisors manage it by getting consultation rather than acting on it or handing it to the supervisee (Bernard & Goodyear, 2019). The director has identified resentment and seen it lower draft ratings, with the evaluation due in 5 days. Consultation offers an outside check so final ratings match the case reviews, as fair evaluation requires (NASW Code 3.03). It beats D because transfer is premature: the reaction is recognized and workable, and abrupt reassignment would disrupt the associate's supervision.

D **Incorrect** (trap: Hands off too soon). Transfer is warranted if a supervisor cannot manage a reaction even with consultation. Here the reaction is recognized and the ratings can be corrected; reassignment would disrupt the associate's supervision before the less drastic step is tried.

Takeaway: When a supervisor's own reaction is biasing a supervisee's evaluation, seek consultation before finalizing it, rather than disclosing the reaction or transferring first.

Sources: Bernard, J. M., & Goodyear, R. K. (2019). Fundamentals of Clinical Supervision (6th ed.). Pearson, Supervisory relationship: transference and countertransference; NASW Code of Ethics (2021), 3.03
Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.3 Transference and countertransference in supervision · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

31. INT-1595 (rev 1) · Best answer: B

PRETEST — unscored on this form; do not count it

Which external review checks a whole organization against published standards and grants it a recognized status for a set period?

A. Utilization review
B. Accreditation survey
C. Peer review
D. Financial audit

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Utilization review examines whether specific services, such as hospital admissions and lengths of stay, are medically necessary and efficiently used (42 CFR 482.30). It looks at cases, not at a whole organization against published standards.

B **Correct.** Accreditation is an external review in which a body such as CARF assesses whether an organization's programs and services meet defined standards of quality through a consultative peer-review process, and grants accreditation for a set term (CARF International). The decisive features are whole-organization review, published standards and a time-limited status. This beats D because a financial audit examines fiscal statements and controls, not whether services meet practice standards.

C **Incorrect** (trap: Wrong model or concept). Peer review has professionals examine colleagues' work, often a sample of cases, to judge the quality of practice. It does not grant an organization a recognized status for a fixed period.

D **Incorrect** (trap: Wrong model or concept). A financial audit examines an organization's financial statements and internal controls. It checks fiscal accuracy rather than whether policies and services meet practice standards.

Takeaway: Accreditation is an external review against published standards that grants recognized status for a set term; utilization review checks medical necessity.

Sources: CARF International. What is accreditation? (web page), carf.org/accreditation, accessed 2026.; Medicare Conditions of Participation for Hospitals, 42 CFR 482.30 (utilization review)
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.6 Quality assurance: external program reviews and audits · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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32. ASM-1631 (rev 1) · Best answer: C

At an aging services agency, a social worker meets with a 76-year-old client with macular degeneration who has had only routine eye exams. The client can no longer read pill labels or recipes and has stopped cooking and going to a weekly card game. The client's adult children want the client to move to assisted living. The client says, 'If I could just see well enough to manage, I'd stay home.' What should the social worker do **FIRST**?

A. Help the family compare nearby assisted living options
B. Arrange home-delivered meals to replace cooking
C. Refer the client for low vision rehabilitation
D. Hold a family meeting to weigh the proposed move

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Assisted living may become right if vision services cannot make home safe enough. Planning the move now overrides the client's stated wish to stay home before less restrictive options have been tried.

B **Incorrect** (trap: Good but not best). Meal delivery addresses one lost task and may help in the short term. It does nothing for medication safety or social withdrawal, and it accepts the loss of function instead of trying to restore it.

C **Correct.** Vision loss can erode daily function at any age, and rehabilitation can restore much of it through magnifiers, lighting, talking or large-print labels, and training in cooking and medication safety. The client wants to stay home and has never had these services, so restoring function comes before any decision about moving. It beats D because a family meeting about the move is more productive once everyone knows what the client can manage with vision services. Ophthalmology guidelines encourage linking people with vision loss to rehabilitation (AAO, 2022; NASW Code 1.02).

D **Incorrect** (trap: Right action, wrong time). A family meeting will likely be needed, since the children are worried. Holding it before the vision evaluation means debating the move without knowing what the client could manage with low vision services.

Takeaway: Before considering a more restrictive setting for an adult with sensory loss, refer for rehabilitation that may restore function.

Sources: American Academy of Ophthalmology (2022). Vision Rehabilitation Preferred Practice Pattern, providing rehabilitation information to patients with vision loss; NASW Code of Ethics (2021), 1.02
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.17 Effects of physical and mental ability across the lifespan · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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33. ETH-1657 (rev 1) · Best answer: B

A hospital social worker serves on the ethics committee. A case comes to the committee in which an ICU patient's adult child, the health care agent, disagrees with the medical team. The social worker recognizes the agent as a former client from 3 years ago. The committee meets in an hour. What is the **BEST** action?

A. Tell the chair the agent is a former client and step aside
B. Step aside, saying a conflict exists without naming it
C. Take part, since services ended 3 years ago
D. Ask the agent for permission before taking part

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Disclosing and stepping aside is the usual way to handle a conflict, which makes this tempting. Naming the agent as a former client, though, reveals that the person received services, which is confidential. The chair needs to know that a conflict exists, not its nature.

B **Correct.** Two duties apply. A conflict of interest that could affect impartial judgment calls for stepping back (NASW Code 1.06(a)), and the fact that someone was a client is itself confidential (1.07(c)). Recusing while saying only that a conflict exists meets both duties: the committee's review stays fair, and the former client's privacy holds. It beats A because naming the past treatment relationship, even to a colleague, discloses

confidential information that the committee does not need in order to manage the conflict.

C **Incorrect** (trap: Boundary slip or collusion). The Code sets no expiration date on conflicts involving former clients. Knowing the agent's history from treatment could color the worker's view of the agent's motives, and the agent might later learn that the former provider weighed in.

D **Incorrect** (trap: Good but not best). Seeking permission involves the former client in the worker's committee role and still leaves the worker's impartiality in question. One party's consent cannot cure a conflict that affects the whole committee's deliberation.

Takeaway: Resolve a conflict of interest by stepping back, and disclose only that a conflict exists when its nature is confidential.

Sources: NASW Code of Ethics (2021), 1.06(a); NASW Code of Ethics (2021), 1.07(c)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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34. ASM-1187 (rev 1) · Best answer: B

At an outpatient intake, an American Indian client describes a heavy grief that began before any loss in their own life. The client's grandparents were sent to boarding schools that punished them for speaking their language, and the client's parent rarely spoke about that time. The client says the family 'carries a sadness nobody names.' Which concept **MOST** likely fits this presentation?

A. Complex trauma
B. Historical trauma
C. Acculturative stress
D. Vicarious trauma

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Complex trauma refers to prolonged, repeated interpersonal trauma, usually in the person's own childhood, such as ongoing abuse. The client describes no such personal history; the wound began with earlier generations.

B **Correct.** Historical trauma is cumulative emotional and psychological wounding across generations that comes from massive group trauma, such as forced removal of children to boarding schools (Brave Heart et al., 2011). Unresolved grief passed down without being spoken about, and felt before any personal loss, is a classic sign of it. It beats C because acculturative stress comes from the person's own adjustment to a new or dominant culture, not from grief inherited from a group's past.

C **Incorrect** (trap: Wrong model or concept). Acculturative stress is strain from a person's own contact with and adjustment to a dominant culture, such as language barriers or conflicting norms. The loss of language is part of this family's history, but the client describes inherited grief, not current adjustment strain.

D **Incorrect** (trap: Wrong model or concept). Vicarious trauma is the change in a helper's beliefs and emotions from repeated exposure to clients' trauma stories. It applies to professionals, not to a family's experience across generations.

Takeaway: Grief and trauma responses passed down from a group's collective trauma, such as boarding schools, describe historical trauma.

Sources: Brave Heart, M. Y. H., Chase, J., Elkins, J., & Altschul, D. B. (2011). Historical trauma among Indigenous peoples of the Americas: Concepts, research, and clinical considerations. Journal of Psychoactive Drugs, 43(4), 282-290, whole article

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.1 Trauma, stress, violence, crisis: signs and impact (historical, complex) · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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35. ETH-1580 (rev 2) · Best answer: C

In a telehealth session, a Black client describes months of coworkers touching the client's hair, asking whether the client 'got the job through a program,' and joking about the client's 'attitude.' The client now sleeps poorly and dreads Mondays. The client asks, 'Am I being too sensitive?' What is the **MOST** appropriate response by the social worker?

A. Help the client test whether the coworkers really meant harm

B. Encourage the client to report each incident to human resources

C. Affirm that repeated slights can cause real stress and harm

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Weighing the evidence for a thought helps with distorted beliefs, but the client is describing a repeated, real pattern. Testing the coworkers' intent implies the client misread events and can deepen the harm.

B **Incorrect** (trap: Too directive). Reporting may become a good option, and the social worker can help the client weigh it. Advising it now, before validating the experience or learning the client's goals, is premature and directive.

C **Correct.** Microaggressions are brief, everyday slights that send demeaning racial messages, whether intended or not, and their cumulative effect can harm well-being (Sue et al., 2007). The client describes a repeated pattern with clear effects on sleep and mood and asks whether the reaction is valid. Affirming the harm validates the experience and opens the way to assessment and coping. It beats A because questioning the coworkers' intent recasts an accurate perception as a thinking error, which can itself feel dismissive (NASW Code 1.05(b)).

Takeaway: When a client describes repeated microaggressions, validate the harm first; do not treat accurate perceptions of bias as distortions.

Sources: Sue, D. W., Capodilupo, C. M., Torino, G. C., Bucceri, J. M., Holder, A. M. B., Nadal, K. L., & Esquilin, M. (2007). Racial microaggressions in everyday life: Implications for clinical practice. American Psychologist, 62(4), 271-286, whole article; NASW Code of Ethics (2021), 1.05(b)

Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.6 Privilege, implicit bias, microaggressions and their client impact · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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36. ASM-1553 (rev 1) · Best answer: C

At a family resource center, a parent asks for help applying for food assistance for their 4-year-old, a U.S. citizen. Partway through the form, the parent stops and says, 'I don't have papers. If I apply, could they use it against me or deport me?' The parent wants the child to eat well but is afraid to continue. What is the **BEST** action by the social worker?

A. Assure the parent that a child's application cannot affect them
B. Encourage finishing the application since the child is eligible
C. Connect the parent with a nonprofit immigration legal service
D. Explore the parent's past experiences with government agencies

Why each option is right or wrong

A **Incorrect** (trap: Outside the social work role). The parent needs accurate information, but predicting immigration consequences is legal advice. Rules on public benefits and immigration are complex and have changed over time, so a blanket assurance could be wrong.

B **Incorrect** (trap: Overrides the client's choice). The child may well qualify, yet pushing ahead overrides the fear the parent just named. The parent decides once reliable legal information is available; the social worker can then help finish the application.

C **Correct.** Immigration status and displacement shape whether families use services, and fear of immigration consequences often keeps mixed-status families from benefits their citizen children can receive. The parent's question is a legal one whose answer depends on individual facts and changing rules. Social workers practice within their competence (NASW Code 1.04) and refer when specialized knowledge is needed (1.16). It beats A because a confident assurance the social worker cannot guarantee could mislead the parent about a real legal risk.

D **Incorrect** (trap: Good but not best). Exploring past experiences can build trust over time. The parent has already named a specific fear about deportation, and exploring history does not answer the question that is blocking the decision.

Takeaway: When immigration fears block access to services, connect families with qualified immigration legal help instead of predicting legal outcomes.

Sources: NASW Code of Ethics (2021), 1.04(a) Competence; NASW Code of Ethics (2021), 1.16(a) Referral for Services
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.20 Displacement effects: homelessness, immigration, refugee status · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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37. INT-1533 (rev 2) · Best answer: A

On a hospital medical unit, a nurse stops the social worker in the hallway and complains that the attending physician talks down to a patient's family and ignores their questions. The nurse asks the social worker to 'say something' to the physician. What is the **MOST** appropriate response by the social worker?

A. Encourage the nurse to raise the concern with the physician directly
B. Bring the nurse's complaint to the physician on the nurse's behalf
C. Report the physician's conduct to the unit's medical director

Why each option is right or wrong

A **Correct.** Social workers cooperate with colleagues of other professions when it serves clients' well-being (NASW Code 2.01(c)), and team concerns are best settled by the people directly involved. Pulling a third person into a two-person conflict forms a triangle, which eases tension briefly but leaves the conflict unresolved (Bowen, 1978). The nurse saw the problem and can describe it best, so the social worker supports the nurse in speaking with the physician and can suggest a team meeting if that fails. It beats B because carrying the complaint makes the social worker a go-between, distorts the message and weakens trust between nurse and physician.

B **Incorrect** (trap: Boundary slip or collusion). Carrying the complaint makes the social worker a go-between: the physician hears it secondhand and the nurse never addresses it. Speaking up directly is right when the social worker has seen the problem firsthand.

C **Incorrect** (trap: Skips assessment). Going over the physician's head before anyone has spoken with the physician skips the direct step and can damage team trust. Formal escalation fits if direct talk fails and the family's care is being harmed.

Takeaway: Support teammates in raising concerns directly with each other; carrying complaints for them creates triangles that keep team conflict unresolved.

Sources: NASW Code of Ethics (2021), 2.01(c); Bowen, M. (1978). Family Therapy in Clinical Practice. Jason Aronson, triangles

Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.3 Inter- and intradisciplinary teamwork (co-therapy, care conferences, IEPs) · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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38. ETH-1701 (rev 2) · Best answer: A

A client arrives at a private nonprofit counseling center with a dog that wears no vest. The client explains that the dog is trained to nudge them and lead them outside when a dissociative episode begins. The receptionist says animals cannot come in unless there is a doctor's letter and asks the social worker how to proceed. What should the social worker tell the receptionist?

A. Admit the dog without requiring any paperwork

B. Admit the dog once a provider letter is on file

C. Admit the dog but keep it in the waiting area during sessions

Why each option is right or wrong

A **Correct.** Under ADA Title III, a service animal is a dog individually trained to do work or perform tasks for a person with a disability, and psychiatric tasks count (28 CFR 36.104). Staff may ask whether the dog is needed because of a disability and what task it performs, but may not require certification or other documentation (28 CFR 36.302(c)(6)). The client has already named a trained task, interrupting dissociation, so the dog should be admitted. It beats C because a service animal may go with the client into every area open to clients, including the counseling room (28 CFR 36.302(c)(7)).

B **Incorrect** (trap: Misreads law or ethics). A provider letter is documentation, and the regulations bar requiring it for a service animal at a public accommodation. Letters can matter in other settings, such as housing requests for assistance animals, but not for entry to this center.

C **Incorrect** (trap: Misreads law or ethics). Keeping the dog out of the counseling room can seem a fair middle ground, but a service animal may accompany the client wherever clients are allowed to go (28 CFR 36.302(c)(7)). This dog's task, interrupting dissociation, may be needed most during sessions.

Takeaway: Admit a trained service dog, including one for a psychiatric disability; staff may ask about need and task but cannot demand documentation.

Sources: ADA Title III regulations, 28 CFR 36.302(c)(6); ADA Title III regulations, 28 CFR 36.104 (definition of service animal); ADA Title III regulations, 28 CFR 36.302(c)(7)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.4 Access: language, interpretation, ASL, physical, cultural, neurodiversity · **Qualifier** none · **Cognitive** application · **Difficulty** 2 of 3

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39. ASM-0116 (rev 1) · Best answer: B

At intake, a parent reports being evicted last week and sleeping in a car with 2 young children during freezing nights. The parent also describes low mood and a custody hearing in 6 weeks, and asks first about the hearing. The parent denies suicidal thoughts. What should the social worker address **FIRST**?

A. The parent's depressive symptoms and treatment options
B. Emergency shelter for the family for tonight
C. A legal aid referral for the custody hearing

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Low mood deserves assessment and care, and it belongs in the plan. With no suicidal thoughts reported, it comes after the family's immediate need for safe shelter.

B **Correct.** Treatment and service planning put immediate safety and basic needs first, then the client's other priorities (Maslow, 1943; Hepworth et al.). Young children sleeping in a car on freezing nights face a safety risk tonight, while the hearing is 6 weeks away and the parent denies suicidal thoughts. It beats C because, although starting where the client is matters and the hearing deserves attention soon, it does not outrank an urgent safety need. The social worker can explain this order and plan the hearing next.

C **Incorrect** (trap: Right action, wrong time). The hearing is the parent's stated concern and should be addressed soon, which honors starting where the client is. With 6 weeks until the hearing, it can follow securing shelter for tonight.

Takeaway: In triage, address immediate safety and basic needs first, then the client's other stated priorities, explaining the order collaboratively.

Sources: Maslow, A. H. (1943). A theory of human motivation. Psychological Review, 50(4), 370-396, whole article; Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, planning: prioritizing problems and goals
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.2 Goal setting, treatment planning, triage, and service plans · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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40. ASM-1066 (rev 1) · Best answer: D

An adult began aripiprazole 10 days ago for psychotic symptoms. At an outpatient visit, the client cannot stay seated, shifts from foot to foot and says, 'I feel like I have to keep moving inside.' The client is distressed by the feeling. Clinic staff call it anxiety about the new treatment. Which explanation is **MOST** likely?

A. Generalized anxiety disorder
B. Tardive dyskinesia
C. Worsening psychotic agitation
D. Akathisia

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Anxiety can cause restlessness, but generalized anxiety disorder requires months of excessive worry. A new urge to move that began within days of an antipsychotic points to a medication effect.

B **Incorrect** (trap: Near-miss diagnosis). Tardive dyskinesia involves involuntary movements, often of the mouth, tongue or face, that develop after months or years of treatment. This restlessness started within 10 days and is felt as an inner urge.

C **Incorrect** (trap: Near-miss diagnosis). Psychotic agitation would come with worsening hallucinations, suspiciousness or disorganization. Here the distress centers on a physical urge to move, a known side effect of the new medicine.

D **Correct.** Akathisia is a medication-induced movement disorder: an inner sense of restlessness with an urge to move, such as pacing or shifting weight, that usually begins within the first weeks of starting or raising an antipsychotic. Aripiprazole is commonly linked to it. It is often mistaken for anxiety or agitation and is distressing enough to raise suicide and nonadherence risk, so the prescriber should be told promptly. It beats A because anxiety does not explain new motor restlessness that began days after the medication (DSM-5-TR).

Takeaway: Inner restlessness and an urge to move soon after starting an antipsychotic suggest akathisia; inform the prescriber promptly.

Sources: DSM-5-TR (APA, 2022), Medication-Induced Movement Disorders and Other Adverse Effects of Medication: Medication-Induced Acute Akathisia

Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.5 Prescription, OTC, alternative medications: uses and side effects · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

41. ETH-1703 (rev 1) · Best answer: B

PRETEST — unscored on this form; do not count it

A community health center serving mainly Black, Latina and Native clients on Medicaid sets a quarterly target for long-acting contraceptive placements, such as implants and IUDs. Staff who meet it are praised at meetings. A client tells the social worker that a staff member kept pushing an implant after the client asked for pills and explained that a grandparent was sterilized without consent. The client now has the pills. What is the **BEST** action for the social worker?

A. Report the staff member's conduct to the clinic manager
B. Ask clinic leaders to drop the target for these methods
C. Train staff to counsel neutrally while keeping the target

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Raising the staff member's conduct is reasonable, and the manager should know. Stopping there treats the problem as one person's lapse, while the target that rewards placements keeps pressuring every staff member and client.

B **Correct.** Given the history of coerced sterilization and contraception aimed at women of color and low-income women, treating long-acting placements as the measure of success pressures staff to steer clients and erodes trust (Gómez, Fuentes & Allina, 2014). This client's experience shows the pressure is already shaping counseling. Social workers act against institutional practices that harm oppressed groups and hold institutions accountable (NASW Code 1.05(b), 1.05(c)). Dropping the target removes the cause and opens the way to measures of choice-centered counseling. It beats C because retraining staff while the reward for placements remains leaves in place the incentive that drove the pushing.

C **Incorrect** (trap: Good but not best). Full, neutral counseling is the right standard, and training can support it. While the target and its praise stay, staff still face pressure to steer clients, so training alone leaves the main cause untouched.

Takeaway: Targets or rewards for specific contraceptive methods invite coercion, especially where reproductive exploitation has a history; advocate for choice-centered measures instead.

Sources: Gómez, A. M., Fuentes, L., & Allina, A. (2014). Women or LARC first? Reproductive autonomy and the promotion of long-acting reversible contraceptive methods. Perspectives on Sexual and Reproductive Health, 46(3), 171-175, whole article; NASW Code of Ethics (2021), 1.05(b); NASW Code of Ethics (2021), 1.05(c)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.7 Exploitation's cultural impact on marginalized communities (trafficking, MMIW) · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

42. ASM-1560 (rev 2) · Best answer: B

A 20-year-old tells a social worker at the campus counseling center that worry about climate change has grown since a summer of wildfires near home. The student keeps up with classes, sleeps a little less after reading the news, and recently joined a campus sustainability group. The student asks, 'Is something wrong with me?' What is the **MOST** appropriate response?

A. Challenge the catastrophic thoughts with cognitive restructuring
B. Frame the concern as understandable and build meaning-focused coping
C. Refer the student for a medication evaluation for anxiety

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Cognitive restructuring fits when thoughts are distorted or exaggerated. Worry about climate risks the student has seen firsthand is realistic, so disputing it can feel invalidating and misses the coping need.

B **Correct.** Climate change is a real environmental threat, and worry about it can be a proportionate response rather than a disorder. This student's worry follows actual wildfire exposure, functioning is intact, and the student is already taking constructive action. Research suggests meaning-focused coping, such as finding hope and purpose through action, best supports both engagement and well-being (Clayton, 2020). It beats A because cognitive restructuring targets distorted thinking, and concern about documented climate risks is not a distortion.

C **Incorrect** (trap: Hands off too soon). A medication evaluation fits when distress meets criteria for an anxiety disorder and impairs functioning. This worry is focused on a real threat and the student functions well, so referral would pathologize a proportionate reaction (DSM-5-TR).

Takeaway: Proportionate worry about real climate threats is not pathology; support constructive, meaning-focused coping and watch for impairment.

Sources: Clayton, S. (2020). Climate anxiety: Psychological responses to climate change. Journal of Anxiety Disorders, 74, 102263, coping strategies; meaning-focused coping; DSM-5-TR (APA, 2022), Anxiety Disorders: Generalized Anxiety Disorder

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.22 Urbanization, globalization, environmental hazards, climate change impacts · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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43. ETH-1111 (rev 1) · Best answer: A

On Monday morning, a licensed clinical social worker in solo private practice checks the state board's website and finds that the license expired 10 days ago because required continuing education hours were never reported. Eight clients are scheduled this week. What should the social worker do?

A. Pause client sessions until the license is reinstated
B. Finish the missing hours online and keep this week's sessions
C. See current clients while the license renewal is pending

Why each option is right or wrong

A **Correct.** Once a license expires, continuing to provide licensed services misrepresents a credential the social worker does not currently hold (NASW Code 4.06(c)) and is the kind of unauthorized practice the Code asks social workers to prevent (5.01(e)). The social worker pauses sessions until the board reinstates the license, while telling clients promptly and helping any who need care in the meantime find it (1.17(e)). A beats B because finishing the hours does not renew the license; only the board's action does.

B **Incorrect** (trap: Misreads law or ethics). Completing the hours is needed for reinstatement, but the license stays expired until the board acts. Seeing clients this week would still be practice without a current license.

C **Incorrect** (trap: Misreads law or ethics). Limiting practice to current clients feels like a compromise, but license status is the same for every client. Any licensed service this week would be unauthorized.

Takeaway: An expired license means no licensed practice until reinstatement; pause services and help clients with interim care.

Sources: NASW Code of Ethics (2021), 4.06(c); NASW Code of Ethics (2021), 5.01(e); NASW Code of Ethics (2021), 1.17(e)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.8 Keeping knowledge current: training, licensing CE, supervision, consultation · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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44. ETH-1627 (rev 1) · Best answer: A

A middle school uses restorative circles after harm. A 13-year-old painted insults over a classmate's mural and has admitted it. The classmate tells the school social worker, 'I don't want to sit in a room with them.' The principal says the circle is school policy and should go ahead. What is the **MOST** appropriate response?

A. Meet with the classmate alone to learn what repair they want
B. Encourage the classmate to attend, as meeting aids healing
C. Hold the circle with a staff member speaking for the classmate

Why each option is right or wrong

A **Correct.** Restorative justice centers the needs of the person harmed and relies on voluntary participation (Zehr, 2015). The classmate has clearly declined to meet, and the choice belongs to them (NASW Code 1.02). Meeting separately keeps the work restorative: the social worker learns what the classmate needs, such as a repainted mural or a written apology, and can carry that to the student who caused harm. It beats C because a stand-in speaks for the classmate before anyone knows what the classmate wants, and it still runs the meeting the classmate refused.

B **Incorrect** (trap: Overrides the client's choice). Joint meetings can be powerful, and some harmed people later choose them. Urging the classmate to attend after a clear refusal turns a voluntary process into pressure, which can deepen the harm. The option can be revisited if the classmate raises it.

C **Incorrect** (trap: Misuses the technique). Surrogates and indirect formats are used in restorative work, but only after the person harmed has said what they need and agreed

to the format. Running the circle now satisfies the policy while sidelining the classmate's voice.

Takeaway: Restorative processes are voluntary for those harmed; when they decline to meet, use separate or indirect steps built around their needs.

Sources: Zehr, H. (2015). The Little Book of Restorative Justice (revised and updated ed.). Good Books, Principles: harms and needs; voluntary participation; NASW Code of Ethics (2021), 1.02
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.8 Restorative practice, truth and reconciliation with historically oppressed groups · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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45. ASM-1033 (rev 2) · Best answer: B

A partner brings a 57-year-old client to a hospital neurology clinic. Over the past year the client has made crude remarks to strangers, spent savings on impulse purchases, eaten sweets compulsively and shown little concern for family. The changes have come on gradually and steadily. The client remembers recent events and appointments well. Which diagnosis is **MOST** likely?

A. Bipolar I disorder, manic episode
B. Frontotemporal neurocognitive disorder
C. Alzheimer's neurocognitive disorder
D. Antisocial personality disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Mania is episodic: a distinct period of elevated or irritable mood with more energy and less need for sleep. A gradual, steady change in conduct starting at 57 suggests a brain disease, not a first mood episode.

B **Correct.** Frontotemporal neurocognitive disorder, behavioral variant, typically begins in the 50s or 60s with gradual disinhibition, apathy or loss of empathy, compulsive behavior and changes in eating, while memory is relatively spared early (DSM-5-TR). A steady year-long decline in social behavior at 57, with sweets bingeing and intact recall of recent events, fits this pattern. It beats C because Alzheimer's disease usually begins with memory and learning problems, which this client does not show.

C **Incorrect** (trap: Near-miss diagnosis). Alzheimer's disease typically starts with decline in memory and learning. Here memory for recent events is intact while behavior and social conduct have changed, the reverse of the usual Alzheimer's pattern.

D **Incorrect** (trap: Near-miss diagnosis). Antisocial personality disorder requires a pattern from adolescence, with conduct disorder before age 15. New behavior change in midlife does not fit any personality disorder.

Takeaway: New disinhibition, apathy or compulsive eating in midlife with spared memory points to frontotemporal neurocognitive disorder, not a psychiatric disorder.

Sources: DSM-5-TR (APA, 2022), Neurocognitive Disorders: Major or Mild Frontotemporal Neurocognitive Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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46. INT-1685 (rev 2) · Best answer: A

An associate social worker at a hospital realizes that a patient's psychosocial assessment, sent an hour ago by secure email, went to the wrong family's case manager at a nursing facility. The associate's supervisor is on site today, and weekly supervision is in 4 days. What should the associate do **FIRST**?

A. Inform the supervisor of the error now
B. Raise the error with the supervisor at the scheduled session
C. Ask the recipient to delete the message and confirm by reply

Why each option is right or wrong

A **Correct.** Supervision is a shared responsibility, and when a supervisee makes a mistake, the supervisor helps repair the harm and prevent a repeat (NASW & ASWB, 2013). A misdirected assessment is a privacy breach the organization must mitigate (HIPAA, 45 CFR 164.530(f)), so the supervisor needs to know now, while mitigation can still work. It beats C because contacting the recipient is one mitigation step that the agency's breach procedure should direct and record; with the supervisor on site, telling them first takes minutes and lets that step happen promptly and correctly.

B **Incorrect** (trap: Right action, wrong time). Supervision is the right place to review the error and learn from it, but 4 days is too long. Mitigation is time-sensitive, and the organization's breach procedure depends on prompt notice.

C **Incorrect** (trap: Right action, wrong time). Requesting deletion is a reasonable mitigation step, and the supervisor or privacy officer may direct it within minutes. Taken first on the associate's own, it bypasses the agency's breach procedure, leaves no record and may delay the timely notice the patient is owed under agency policy (NASW Code 1.07(n)).

Takeaway: Supervisees disclose errors to the supervisor promptly, especially time-sensitive ones, so repair follows agency procedure rather than private fixes.

Sources: NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Standard 4 (Ethical Issues): Ethical Decision-Making; HIPAA Privacy Rule, 45 CFR 164.530(f); NASW Code of Ethics (2021), 1.07(n)
Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.1 Supervisee's role: learning needs, self-assessment, prioritizing · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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47. INT-1626 (rev 2) · Best answer: D

In the tenth month of weekly psychodynamic therapy, a client whose parent died when the client was 8 misses the session after the social worker announces a 3-week leave. Coverage plans were given at the time of the announcement. At the next session, the client shrugs and says, 'It doesn't matter. You'll be gone anyway.' What is the social worker's **BEST** response?

A. Review how to reach the covering social worker during the leave
B. Interpret the reaction as grief for the parent who died
C. Reassure the client that the leave is short and the work will go on
D. Invite the client to say more about the coming break

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Coverage was already explained when the leave was announced. Repeating it treats the comment as a practical question and steps around the feeling behind the missed session and the shrug.

B **Incorrect** (trap: Right action, wrong time). The link to the parent's death may be accurate, and in long-term work an interpretation can be appropriate. Offered now, before the client has explored the present reaction, it jumps ahead of the client's awareness and is likely to be dismissed. It fits better after exploration brings the material closer.

C **Incorrect** (trap: Good but not best). Reassurance is kind and may ease some worry, but it answers the shrug with the worker's view instead of the client's. It closes down the feelings the missed session hints at, which are the material that matters before the break, and they may resurface as further absences.

D **Correct.** A client's reaction to the helper's absence is central relationship material, and psychodynamic work starts at the surface: the helper invites the client's own thoughts and feelings about the break before linking them to the past (Gabbard, 2014). The missed session and the shrug suggest the leave stirred more than the client says, and talking about it now protects the alliance before the gap. It beats B because offering the link to the parent's death before the client has looked at the present reaction is likely to feel intrusive and be brushed aside; that interpretation may fit later, once the client's own associations lead there.

Takeaway: A client's reaction to the helper's absence is relationship material; invite the present reaction before offering links to the past.

Sources: Gabbard, G. O. (2014). Psychodynamic Psychiatry in Clinical Practice (5th ed.). American Psychiatric Publishing, Individual psychotherapy: expressive-supportive continuum of interventions
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.2 Building and sustaining the helping relationship (empathy, rapport, acceptance) · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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48. ETH-1032 (rev 2) · Best answer: B

A community mental health agency has started offering text-message check-ins between sessions. An adult client in the depression program asks to begin receiving them this week. What should the social worker do **FIRST**?

A. Text the client the policy and start once the client replies OK
B. Review the texting policy and its risks, then get consent
C. Agree, and invite the client to text during any crisis

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Sharing the written policy is part of informed consent, which makes this tempting. A one-word reply by text, though, is not a discussion of privacy risks, response times and crisis limits, and it gives the client no real chance to ask questions before services start.

B **Correct.** Before technology-based services start, social workers discuss their technology policies and obtain informed consent (NASW Code 1.03(e), 1.03(f)). For texting, that means explaining privacy risks on shared phones, expected response times, what goes in the record, and that texts are not for emergencies. B beats A

because a texted policy and a reply of 'OK' skip the discussion of risks and limits that makes consent informed.

C **Incorrect** (trap: Ignores risk). Texting is poorly suited to crises because messages can go unseen for hours. Inviting crisis texts creates a false safety net; clients need a crisis plan that uses phone lines or emergency services.

Takeaway: Before starting any technology-based service, explain the policies and risks and obtain informed consent.

Sources: NASW Code of Ethics (2021), 1.03(e); NASW Code of Ethics (2021), 1.03(f)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.4 Informed consent: principles and how it is obtained · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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49. ETH-1136 (rev 2) · Best answer: A

Shelter staff tell a new resident that their bed will be given to someone else unless they sign up for the agency's counseling program today. The resident accurately explains the program's purpose, risks and alternatives, then signs the consent form. Which element of valid consent is **MOST** likely missing?

A. Voluntariness
B. Disclosure
C. Comprehension

Why each option is right or wrong

A **Correct.** Valid consent rests on disclosure, comprehension, capacity and voluntariness, and consent given under coercion or undue influence is not valid (Reamer, 2018). The resident's accurate account of the program's purpose, risks and alternatives shows that information was given, understood and weighed, but the threat of losing a bed makes the choice unfree. The Code also expects clients to know they may refuse or withdraw consent (NASW Code 1.03(a)). A beats C because the resident's grasp of the program is intact; what is missing is a free choice.

B **Incorrect** (trap: Wrong model or concept). Disclosure means the needed information was given. The resident can describe the purpose, risks and alternatives, so the information was provided.

C **Incorrect** (trap: Wrong model or concept). Comprehension is the client's understanding of what was disclosed. The resident explains the program accurately, so understanding is present; the problem is the pressure behind the signature.

Takeaway: Valid consent must be voluntary; agreement given under threat of losing housing is not free, even when the client fully understands.

Sources: Reamer, F. G. (2018). Social Work Values and Ethics (5th ed.). Columbia University Press, informed consent: coercion and undue influence; NASW Code of Ethics (2021), 1.03(a)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.4 Informed consent: principles and how it is obtained · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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50. ETH-1137 (rev 2) · Best answer: A

A 17-year-old requests counseling at a community clinic for panic attacks. The client lives independently and gives intake a copy of a court order of emancipation. The intake coordinator says a parent must still sign the treatment consent because the client is under 18. What is the **MOST** appropriate way for the social worker to proceed?

A. Have the client sign the treatment consent themselves
B. Contact a parent to sign the consent before treatment begins
C. Have the client sign, then let a parent know care has begun

Why each option is right or wrong

A **Correct.** Emancipated minors are treated as adults for consent and may agree to or refuse their own care without a parent (Davis & Fang, 2023). HIPAA's parental-representative rule applies to unemancipated minors (45 CFR 164.502(g)(3)), so this client also controls their own health information. The social worker obtains informed consent from the client in clear language (NASW Code 1.03(a)). A beats B because the court order gives the client adult decision-making authority; requiring a parent's signature ignores it and could disclose the client's treatment.

B **Incorrect** (trap: Misreads law or ethics). Parental consent is the general rule for minors, which is why intake raised it. Emancipation removes that requirement, and contacting a parent without the client's agreement would also disclose the client's treatment.

C **Incorrect** (trap: Misreads law or ethics). The client's own signature is right, which makes this tempting. Telling a parent without the client's agreement, though, discloses treatment the client controls; an emancipated minor decides who is told (45 CFR 164.502(g)(3)).

Takeaway: An emancipated minor consents to their own treatment and controls their own health information, without a parent's signature.

Sources: Davis, M., & Fang, A. (2023). Emancipated Minor. StatPearls. NCBI Bookshelf, consent to medical care without parental permission; HIPAA Privacy Rule, 45 CFR 164.502(g)(3); NASW Code of Ethics (2021), 1.03(a)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.4 Informed consent: principles and how it is obtained · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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51. ASM-1019 (rev 1) · Best answer: C

PRETEST — unscored on this form; do not count it

A 26-year-old client at a community agency describes mood swings that shift within hours, often after a partner is slow to answer texts. During these times the client feels intense rage, cuts their arms and fears being left. Between swings the client describes a constant inner emptiness. This pattern has continued since the client's late teens. What is the **MOST** likely diagnosis?

A. Bipolar II disorder
B. Cyclothymic disorder
C. Borderline personality disorder
D. Histrionic personality disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Bipolar II needs distinct hypomanic episodes of at least 4 days plus major depressive episodes. Shifts lasting hours, set off by relationship events, describe affective instability rather than mood episodes.

B **Incorrect** (trap: Near-miss diagnosis). Cyclothymic disorder involves 2 or more years of many milder highs and lows lasting days. It does not explain fear of abandonment, self-harm or chronic emptiness, and mood here changes within hours.

C **Correct.** Borderline personality disorder is a pervasive pattern, beginning by early adulthood, of unstable relationships, self-image and mood, with frantic efforts to avoid abandonment, self-harm, intense anger and chronic emptiness (DSM-5-TR). Mood shifts lasting hours and set off by a partner's slow replies fit its reactive mood instability. It beats A because bipolar II requires a hypomanic episode lasting at least 4 days; mood changes that come and go within hours in response to relationship events are not mood episodes.

D **Incorrect** (trap: Near-miss diagnosis). Histrionic personality disorder centers on attention seeking and shallow, dramatic emotion. Fear of abandonment, self-harm, rage and chronic emptiness point to borderline personality disorder.

Takeaway: Mood shifts lasting hours, triggered by relationships, with abandonment fears and emptiness suggest borderline personality disorder rather than a bipolar disorder.

Sources: DSM-5-TR (APA, 2022), Personality Disorders: Borderline Personality Disorder; DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Bipolar II Disorder

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

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52. INT-1593 (rev 2) · Best answer: C

During an external licensing review, a state reviewer found that 30% of sampled charts at a community mental health agency lacked treatment plan updates within the required period. The agency's quality coordinator, a social worker, has since fixed the cited charts and trained staff on the requirement. What should the coordinator do **NEXT**?

A. Require supervisor sign-off on each chart's plan update

B. Repeat the staff training at the next staff meeting

C. Audit a new sample of charts over the next few months

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). A sign-off step may strengthen the process and could become part of the fix. Adding it now assumes the cause without checking, and it adds work for every chart; a follow-up audit of new charts first shows whether the training worked and where any gap remains.

B **Incorrect** (trap: Skips assessment). Repeating training assumes knowledge is still the problem. Without data on current charts, the coordinator cannot tell whether the first training worked or whether a workflow barrier remains.

C **Correct.** Quality improvement does not end when cited records are fixed; a change must be monitored to confirm the improvement holds (Langley et al., 2009). A follow-up audit of newly written charts shows whether training changed practice and whether

documentation is now timely (NASW Code 3.04(b)). This beats A because a new sign-off layer adds work without showing whether practice has changed; checking new charts tests the fix before more controls are added.

Takeaway: After correcting audit findings, monitor current performance with follow-up audits to confirm the fix is sustained.

Sources: Langley, G. J., Moen, R. D., Nolan, K. M., Nolan, T. W., Norman, C. L., & Provost, L. P. (2009). The Improvement Guide (2nd ed.). Jossey-Bass, Ch. 8: Implementing a Change; NASW Code of Ethics (2021), 3.04(b)
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.6 Quality assurance: external program reviews and audits · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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53. ETH-1141 (rev 1) · Best answer: D

A hospital social worker plans discharge for an 82-year-old patient with moderate major neurocognitive disorder. The patient's grandchild holds power of attorney. The billing office says the patient's nursing home bills are 5 months unpaid, and bank statements show $40,000 moved to the grandchild's account. When asked, the patient cannot say where the money went. What should the social worker do **FIRST**?

A. Meet with the grandchild to request an accounting of the funds
B. Ask the patient more about their finances and wishes
C. Refer the family to an elder law attorney for advice
D. Report suspected financial exploitation to adult protective services

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). An accounting may be part of the eventual response, but asking the suspected exploiter for it first can alert them, lead to more transfers or put pressure on the patient. Investigation belongs to adult protective services.

B **Incorrect** (trap: Right action, wrong time). Exploring the patient's wishes is usually a good step, and it has been tried: the patient cannot explain where the money went. More questions now delay the report without adding needed facts.

C **Incorrect** (trap: Good but not best). Legal advice may help later, for example to change the power of attorney. A referral does not address the suspected exploitation now or bring in the agency responsible for investigating it.

D **Correct.** Suspected exploitation of a vulnerable adult calls for a report to adult protective services, which most states require of professionals; investigating is APS's role, not the worker's. Federal law defines exploitation to include improper use of an elder's resources for another's gain (Elder Justice Act, 42 U.S.C. 1397j(8)). This patient cannot protect their own interests, so the worker acts to protect them (NASW Code 1.14). D beats A because confronting the grandchild first could prompt more transfers or pressure on the patient before APS can act.

Takeaway: Reasonable suspicion of elder financial exploitation goes to adult protective services; do not confront the suspected exploiter first.

Sources: NASW Code of Ethics (2021), 1.14; Elder Justice Act, 42 U.S.C. 1397j(8)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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54. ASM-1079 (rev 1) · Best answer: B

Ten days after starting sertraline for depression, a client on a telehealth visit says the pills 'are not doing anything' and plans to stop them this weekend. The client mentions mild nausea that is fading. The client has not talked with the prescriber about stopping. What is the **MOST** appropriate response from the social worker?

A. Suggest asking the prescriber for a stronger dose
B. Explain the usual delay in benefit and urge a prescriber call
C. Explore the client's general beliefs about taking medication

Why each option is right or wrong

A **Incorrect** (trap: Outside the social work role). Recommending a dose change is outside the social work role, and 10 days is too soon to judge the current dose. The client can raise any concern with the prescriber, who decides on dosing.

B **Correct.** Antidepressants usually take 4 to 8 weeks to work, and NIMH advises against stopping a prescribed medicine without a health care provider's help (NIMH, 2023). The client may simply not know about the delay. Giving general medication information and linking the client to the prescriber fits the social work role and leaves the decision with the client and prescriber. It beats C because exploring beliefs can come later; the client plans to stop within days for a reason that brief education may resolve.

C **Incorrect** (trap: Good but not best). Beliefs about medication shape adherence and are worth exploring. Here the client's reason is specific and fixable: they expect results too soon. Exploration alone leaves the weekend plan to stop in place.

Takeaway: Antidepressants take weeks to work; give general information and route stopping decisions to the prescriber.

Sources: National Institute of Mental Health (2023). Mental Health Medications, Antidepressants; stopping medication
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.5 Prescription, OTC, alternative medications: uses and side effects · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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55. ETH-1692 (rev 1) · Best answer: B

A clinical supervisor at the only behavioral health agency in a rural county learns that a new supervisee will coach the supervisor's 10-year-old child's soccer team this season. Contact would be limited to weekend practices and games. The agency has no other supervisor with the supervisee's specialty. What is the supervisor's **MOST** appropriate action?

A. Arrange video supervision for the supervisee from another county
B. Discuss the overlap with the supervisee and agree on clear boundaries
C. Continue as usual, since the contact happens outside work

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). An outside supervisor would remove the overlap, but the overlap carries little risk: it is public, limited and unrelated to work. Moving supervision out of the agency fits when a dual relationship is significant, such as a close friendship or a family tie.

B **Correct.** Supervisors avoid dual relationships with supervisees that carry a risk of exploitation or harm, and they set clear, appropriate, culturally sensitive boundaries (NASW Code 3.01(b); 3.01(c)). In a small community some overlap cannot be avoided (Reamer, 2020). Coaching a team means public, limited contact with nothing that bears on evaluation, so naming it and agreeing on how to handle it, such as keeping work talk out of games, manages the risk. It beats A because remote supervision by an outsider trades a low, manageable risk for supervision from someone who does not know the agency or its community.

C **Incorrect** (trap: Good but not best). The contact is low risk, but ignoring it leaves both people guessing about how to act at games and whether work topics will come up. Naming the overlap early prevents awkward moments and protects the evaluation.

Takeaway: Unavoidable, low-risk overlaps with supervisees, common in small communities, are managed by naming them and setting boundaries, not by ending supervision.

Sources: NASW Code of Ethics (2021), 3.01(b); NASW Code of Ethics (2021), 3.01(c); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, dual relationships in small and rural communities

Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.7 Ethics of supervising and managing · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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56. ETH-1088 (rev 1) · Best answer: A

A hospice social worker visits a family at home. The parent who is dying has asked that their 9-year-old not be told about the prognosis yet. While drawing with the social worker, the child asks directly whether the parent is going to die. What is the **BEST** response?

A. Validate the worry and bring the question to the parent

B. Answer honestly that the parent is expected to die soon

C. Say the parent's doctors will help them feel better

Why each option is right or wrong

A **Correct.** Several duties meet here: honesty, the parent's authority over what a young child is told, and the child's well-being. Deceiving the child is never acceptable (NASW Code 4.04), but overriding the parent can harm the relationship the child will rely on. Acknowledging the worry and helping the parent plan an honest, age-appropriate talk reduces harm without deception (Dolgoff et al., 2012, least harm). A beats B because the parent is at home and can be helped to answer soon, so taking the decision away is not needed.

B **Incorrect** (trap: Overrides the client's choice). Children often sense the truth, and honesty matters. Giving the news against the parent's stated wish, however, undermines the parent at a fragile time when the parent can be supported to share it soon.

C **Incorrect** (trap: Misreads law or ethics). False reassurance comforts the child briefly but is deception (4.04). It can also damage the child's trust in the adults around them after the parent dies.

Takeaway: When a parent's wishes and a child's question collide, avoid both deception and overriding the parent; help the parent share the truth.

Sources: NASW Code of Ethics (2021), 4.04; Dolgoff, R., Harrington, D., & Loewenberg, F. M. (2012). Ethical Decisions for Social Work Practice (9th ed.). Brooks/Cole Cengage, Ethical Principles Screen: least harm
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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57. INT-1589 (rev 1) · Best answer: A

A middle school program sets this objective: 'By June, participating students will miss 25% fewer school days than last year.' To measure progress, staff plan to give students a survey asking how much they enjoy the program. Which indicator would **MOST** directly measure progress on the objective?

A. Official school attendance records for each participant
B. A student survey asking how often they miss school
C. The number of program sessions each student attends

Why each option is right or wrong

A **Correct.** An indicator must measure the exact change an objective names. For days missed, official attendance records are the most direct and objective source (Kettner, Moroney & Martin, 2022; Royse, Thyer & Padgett, 2016). The planned enjoyment survey measures satisfaction, not attendance. This beats B because self-reported absences are prone to memory errors and the wish to look good, while records already capture the exact information.

B **Incorrect** (trap: Misuses the technique). Asking students how often they miss school targets the right behavior, but self-report is open to recall errors and social desirability. Official records already hold the exact information needed.

C **Incorrect** (trap: Wrong model or concept). Session attendance measures participation in the program, an output. It does not show whether students are missing fewer school days, which is the change the objective names.

Takeaway: Choose an indicator that measures exactly the change an objective names, preferring direct records over opinions or participation counts.

Sources: Kettner, P. M., Moroney, R. M., & Martin, L. L. (2022). Designing and Managing Programs: An Effectiveness-Based Approach (6th ed.). SAGE, Ch. 10: Performance Measurement, Monitoring, and Program Evaluation; Royse, D., Thyer, B. A., & Padgett, D. K. (2016). Program Evaluation: An Introduction to an Evidence-Based Approach (6th ed.). Cengage, Ch. 12: Selecting the Best Evaluation Measure for Your Project
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.2 Setting program objectives and outcomes · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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58. ETH-1653 (rev 1) · Best answer: D

An ICU patient who cannot communicate after a stroke has no advance directive and no named agent. The patient's legal spouse, separated for 3 years, wants aggressive treatment. Two adult children want comfort care. The physician asks the social worker to help resolve the conflict. What should the social worker do **FIRST**?

A. Hold a family meeting to seek agreement on goals of care
B. Follow the adult children, who have been at the bedside
C. Ask the physician to decide based on the patient's interests
D. Find out whom the state's surrogate law authorizes to decide

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). A family meeting to clarify the patient's values and goals is usually the most helpful step. Holding it before anyone knows who has legal authority can produce an agreement the authorized surrogate rejects, or sideline the person entitled to decide.

B **Incorrect** (trap: Misreads law or ethics). Time at the bedside does not create legal authority. Many state laws rank a spouse ahead of adult children, so following the children could leave the legally authorized surrogate out of the decision.

C **Incorrect** (trap: Misreads law or ethics). Physicians decide alone in narrow situations, such as emergencies or when no surrogate can be found. Here relatives who may hold authority are present, so handing the choice to the physician bypasses them.

D **Correct.** Before mediating, the social worker needs to know who holds legal authority. Most states have default surrogate laws that rank relatives, usually placing a spouse first, and some set rules for disagreements (Wynn, 2014). Separation may or may not change a spouse's standing, so state law and hospital policy must be checked. Knowing the decision-maker protects the rights of a patient who cannot decide (NASW Code 1.14). It beats A because a family meeting is the right next step, and it works when everyone knows whose decision it is.

Takeaway: When an incapacitated patient left no directive, identify the legally authorized surrogate before mediating family disagreement.

Sources: Wynn, S. (2014). Decisions by surrogates: An overview of surrogate consent laws in the United States. Bifocal, 36(1). American Bar Association Commission on Law and Aging, default surrogate priority and disagreement rules; NASW Code of Ethics (2021), 1.14
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.6 Law and ethics around death and dying · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 3 of 3

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59. ETH-1560 (rev 2) · Best answer: C

After a client shouts during a video session, the social worker sends an email that evening saying services are over. The client still has active panic symptoms. The email offers no referral and no final session. Which term **MOST** likely describes the social worker's action?

A. Interruption of services
B. Boundary violation
C. Client abandonment

Why each option is right or wrong

- **A** **Incorrect** (trap: Wrong model or concept). Interruption refers to a disruption caused by circumstances such as the worker's illness, relocation, unavailability or a technology failure, which the Code asks workers to manage by ensuring continuity (NASW Code 1.15). Here the worker made a deliberate decision to end care.
- **B** **Incorrect** (trap: Wrong model or concept). A boundary violation is a harmful dual relationship or blurring of roles that exploits the client. Ending care abruptly is harmful, but the problem is leaving a client without services, not blurring roles.
- **C** **Correct.** Abandonment means ending services a client still needs without adequate notice, transition or help arranging continued care (NASW Code 1.17(b)). Here a client with active symptoms is cut off by email, with no referral and no closing session. An outburst may call for a safety review, but it does not excuse ending care this way. It beats A because an interruption of services is a disruption caused by circumstances such as illness or relocation (1.15); this ending was the worker's deliberate choice.

Takeaway: Cutting off a client who still needs help, with no notice, transition or referral, is abandonment, whatever prompted it.

Sources: NASW Code of Ethics (2021), 1.17(b); NASW Code of Ethics (2021), 1.15
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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60. INT-1542 (rev 1) · Best answer: B

PRETEST — unscored on this form; do not count it

On a medical unit, a patient being treated for pneumonia insists on leaving today, before the planned discharge, to attend a parent's funeral in another city. The physician has explained the risks, the patient has decision-making capacity, and the patient still plans to leave. What is the **MOST** appropriate action by the social worker?

- **A.** Explain that insurance may deny payment if the patient leaves early
- **B.** Arrange follow-up care and prescriptions before the patient leaves
- **C.** Explore whether the patient could delay the trip by one day

Why each option is right or wrong

- **A** **Incorrect** (trap: Misreads law or ethics). This is inaccurate: a study of discharges against medical advice found no insurer denied payment because the patient left early (Schaefer et al., 2012). Using a false financial threat to keep a capable patient is coercive and undermines trust.
- **B** **Correct.** A patient with decision-making capacity may refuse further hospital care (NASW Code 1.02). Leaving against advice does not end the duty to plan: the team should still offer the plan that does the least harm, including follow-up appointments, medications and instructions (Alfandre, 2009). The patient has heard the risks and decided, so the social worker's job is to make the departure as safe as possible. It beats C because the funeral is fixed and the decision is made; pressing for delay serves the original plan, not the patient, and uses time needed for arrangements.
- **C** **Incorrect** (trap: Good but not best). Problem solving around barriers to staying fits when the reason is flexible or the patient is unsure. Here the patient has heard the

risks and decided because of a parent's funeral, so planning a safe exit is the better use of time.

Takeaway: When a capable patient leaves against medical advice, respect the choice and still arrange follow-up care, medications and instructions.

Sources: Alfandre, D. J. (2009). 'I'm going home': discharges against medical advice. Mayo Clinic Proceedings, 84(3), 255-260, whole article; Schaefer, G. R., Matus, H., Schumann, J. H., et al. (2012). Financial responsibility of hospitalized patients who left against medical advice: medical urban legend? Journal of General Internal Medicine, 27(7), 825-830, whole article; NASW Code of Ethics (2021), 1.02

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.22 Discharge planning, aftercare, wraparound, follow-up · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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61. INT-1092 (rev 2) · Best answer: B

A client whose partner died in a car crash 16 months ago has seen a social worker for supportive counseling for 3 months. The worker's assessment finds intense daily yearning, a sense that part of the client has died, avoidance of the crash route, a feeling that life is meaningless, and missed work most weeks. The worker has no training in treatments designed for this condition, and a nearby program offers one. What should the worker do **NEXT**?

A. Continue weekly supportive counseling and track the symptoms

B. Refer the client for a therapy that targets this grief

C. Help the client accept that grief has no set timeline

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Supportive counseling and monitoring would be reasonable if the client were improving. After 3 months, symptoms remain intense and work is still disrupted, which signals a need for a treatment designed for this condition.

B **Correct.** Intense, impairing grief more than 12 months after a death fits prolonged grief disorder in adults (DSM-5-TR), and a treatment targeting it outperformed interpersonal psychotherapy in a randomized trial (Shear et al., 2005). The worker lacks training in such methods and an accessible program offers one; social workers should refer when specialized expertise is needed or progress is not being made (NASW Code 1.16(a)). It beats A because supportive counseling has run 3 months while the client still misses work most weeks; continuing it delays effective care.

C **Incorrect** (trap: Near-miss diagnosis). Grief has no fixed timeline in general, but this grief is intense, impairing and has lasted over a year. Treating it as ordinary mourning would leave a treatable condition unaddressed.

Takeaway: When grief stays intense and impairing beyond 12 months and current help isn't working, refer for treatment that targets prolonged grief.

Sources: DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Prolonged Grief Disorder; Shear, K., Frank, E., Houck, P. R., & Reynolds, C. F. (2005). Treatment of complicated grief: A randomized controlled trial. JAMA, 293(21), 2601-2608, whole article; NASW Code of Ethics (2021), 1.16(a)

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.10 Working with loss, separation, and grief · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

Section 2

62. INT-1687 (rev 1) · Best answer: B

A social worker with 8 years of hospice experience recently joined a new hospice team. In supervision, the social worker praises every suggestion, repeats the supervisor's phrasing word for word and recently said the supervisor is 'just like my first mentor, who always knew best.' The social worker has stopped offering their own case formulations. What is the **MOST** appropriate response by the supervisor?

A. Offer more direction while the social worker settles in
B. Name the pattern and explore it with the social worker
C. Suggest personal therapy to work through the mentor relationship

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Added structure suits a worker new to a practice area, as developmental models suggest (Stoltenberg & McNeill, 2010). This worker has years of hospice experience; more direction would reinforce the dependence that is crowding out their judgment.

B **Correct.** Supervisee transference, relating to the supervisor as to an important figure from the past, is handled within the supervisory relationship and kept distinct from therapy (Bernard & Goodyear, 2019). An experienced hospice worker who now copies the supervisor and compares them to a first mentor is showing idealization, and it is displacing the worker's own formulations. Naming the pattern and exploring it, with a focus on the work, helps the worker reclaim independent judgment. It beats A because more direction would feed the idealization in a worker who needs no extra structure.

C **Incorrect** (trap: Hands off too soon). Personal therapy may be suggested if personal issues impair practice and do not shift in supervision. Starting there skips a direct conversation about a pattern the supervisor can address within the supervisory role.

Takeaway: Address supervisee transference within supervision by naming and exploring it in relation to the work before suggesting outside help.

Sources: Bernard, J. M., & Goodyear, R. K. (2019). Fundamentals of Clinical Supervision (6th ed.). Pearson, Supervisory relationship: transference and countertransference; Stoltenberg, C. D., & McNeill, B. W. (2010). IDM Supervision: An Integrative Developmental Model for Supervising Counselors and Therapists (3rd ed.). Routledge, Domains of development; supervision environments by level
Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.3 Transference and countertransference in supervision · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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63. INT-1546 (rev 1) · Best answer: B

Which practice role involves pressing an agency, on a client's behalf, to provide a service the client is entitled to receive?

A. Broker
B. Advocate
C. Mediator
D. Enabler

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). A broker links clients with resources they need, such as connecting a family to a food bank. Brokering assumes a willing provider; pressing a reluctant agency on the client's behalf is advocacy.

B **Correct.** In the advocate role, the social worker champions a client's rights and entitlements and presses a resistant agency or system to provide services the client should receive (Hepworth et al.). The definition's key features are acting on the client's behalf and challenging a refusal. It beats A, broker, which connects clients to resources but does not involve pushing back against a provider that is withholding them.

C **Incorrect** (trap: Wrong model or concept). A mediator helps two parties in a dispute reach agreement and stays neutral between them. Pressing the agency on the client's side is not neutral.

D **Incorrect** (trap: Wrong model or concept). An enabler helps clients find and use their own strengths and coping resources to make changes. The definition describes acting toward an outside agency for the client, which is advocacy.

Takeaway: Advocates press systems for clients' entitlements; brokers link clients to resources; mediators stay neutral between disputing parties.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Social work roles: broker, mediator, client advocate

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.13 Advocating for needed services and resources · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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64. ASM-1512 (rev 1) · Best answer: A

A person chooses on purpose to set a painful worry aside until a better time. Which defense mechanism is this?

A. Suppression
B. Repression
C. Denial

Why each option is right or wrong

A **Correct.** Suppression is the conscious choice to postpone attention to a distressing thought or feeling, and Vaillant classes it among the mature defenses (Vaillant, 1992). Choosing on purpose decides it, because most defenses operate outside awareness. It beats B because repression pushes distressing material out of awareness unconsciously, so the person does not choose it and often cannot recall what was pushed away. Suppression is adaptive, for example setting worries aside to finish an exam.

B **Incorrect** (trap: Wrong model or concept). Repression excludes distressing thoughts or memories from awareness unconsciously. It is the classic contrast to suppression: the same goal, but without the deliberate choice the question describes.

C **Incorrect** (trap: Wrong model or concept). Denial refuses to acknowledge an external reality, such as a diagnosis or a loss, and operates largely outside awareness. It does not involve consciously setting a concern aside for later.

Takeaway: Suppression is conscious, mature postponement of a distressing concern; repression is unconscious exclusion from awareness.

Sources: Vaillant, G. E. (1992). Ego Mechanisms of Defense: A Guide for Clinicians and Researchers. American Psychiatric Press, suppression; repression
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.11 Defense mechanisms and how they shape behavior and relationships · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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65. INT-1691 (rev 2) · Best answer: B

At a county benefits office, intake staff spend most of each appointment checking that every form field is complete. Workers turn away applicants in urgent need over minor paperwork gaps, and staff meetings focus on audit error rates rather than whether families get help. Supervisors praise flawless files. Which organizational problem is **MOST** likely present?

A. Mission drift
B. Goal displacement
C. Groupthink
D. Creaming

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Mission drift is a gradual shift in what an organization does or whom it serves, often driven by funding. The office still exists to deliver benefits; the problem is that procedural compliance has replaced that aim in daily work.

B **Correct.** Goal displacement occurs when rules and procedures meant to serve an organization's purpose become ends in themselves (Merton, 1940). Here complete forms and low audit error rates are what staff pursue and supervisors reward, even as families in urgent need go unserved. It beats A because mission drift is a change in the organization's purpose or population, often following funding; this office's purpose is unchanged, but procedure has eclipsed it in practice.

C **Incorrect** (trap: Wrong model or concept). Groupthink is a cohesive group's drive for agreement overriding critical judgment in decisions (Janis, 1982). The issue here is rules becoming the aim of everyday work, not a team suppressing dissent.

D **Incorrect** (trap: Wrong model or concept). Creaming is selecting the clients most likely to succeed, often to meet performance targets. Applicants here are turned away for paperwork gaps regardless of their prospects, which points to rule fixation rather than selection for success.

Takeaway: When procedures meant to serve clients become the organization's real aim, the problem is goal displacement.

Sources: Merton, R. K. (1940). Bureaucratic structure and personality. Social Forces, 18(4), 560-568, whole article; Janis, I. L. (1982). Groupthink: Psychological Studies of Policy Decisions and Fiascoes (2nd ed.). Houghton Mifflin, groupthink defined
Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.5 How organizations are structured, function, and develop · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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66. ASM-1579 (rev 1) · Best answer: B

A 74-year-old hospital patient had a stroke 5 days ago and now speaks in short, halting phrases. The team plans discharge in 2 days and asks the social worker whether the patient can take part in choosing between home and a rehabilitation facility. The patient's yes and no answers to the social worker's questions have been inconsistent. Which action is **BEST**?

A. Ask the patient simple yes-or-no questions about the options
B. Request a speech-language evaluation of the patient's communication
C. Ask the patient's adult child to choose the discharge setting
D. Note in the record that the patient cannot state a preference

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Simple yes-or-no questions are a common support for people with aphasia, but this patient's answers have already been inconsistent. Without knowing how well the patient understands, the worker cannot tell whether answers reflect real preferences.

B **Correct.** Aphasia can hide a person's competence, so poor speech does not show that someone cannot understand or decide (Kagan, 1998). A speech-language pathologist can assess comprehension and expression and recommend supports, such as written key words or pictures, that let the patient take part. Referral fits when another professional's expertise is needed (NASW Code 1.16(a)). It beats A because the inconsistent answers are exactly the problem; more yes-no questions cannot settle what the patient understands.

C **Incorrect** (trap: Overrides the client's choice). Turning the decision over to family assumes the patient cannot participate before communication has been properly assessed. Family input matters, but a substitute decision maker is considered only after the patient's own ability is evaluated.

D **Incorrect** (trap: Skips assessment). Recording that the patient cannot state a preference treats a communication barrier as an inability to decide. It could shut the patient out of a major decision before anyone has tried supported communication.

Takeaway: When speech or language impairment clouds a client's responses, obtain a communication evaluation before concluding the client cannot participate.

Sources: Kagan, A. (1998). Supported conversation for adults with aphasia: Methods and resources for training conversation partners. Aphasiology, 12(9), 816-830, whole article; NASW Code of Ethics (2021), 1.16(a)
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.8 Evaluating a client's communication skills · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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67. ETH-1611 (rev 2) · Best answer: A

At a hospital discharge meeting, a 40-year-old patient with cerebral palsy communicates by typing on a speech-generating device. Each time the team asks about plans for home, members turn to the patient's caregiver for the answer. The patient starts typing an answer but stops. What should the social worker do **FIRST**?

A. Ask the patient the question and wait for the typed reply
B. Ask the caregiver to answer, then check with the patient
C. Switch to yes-or-no questions to speed up the meeting

Why each option is right or wrong

A **Correct.** Effective communication with a person with a disability means communicating with that person through their own method, with time to use it (28 CFR 36.303(c)(1)). The patient is a competent adult making discharge decisions, yet the team keeps redirecting questions to the caregiver. Turning back to the patient and waiting for the typed answer restores the patient's voice in decisions about their own life (NASW Code 1.02). It beats B because checking with the patient after the caregiver speaks still treats the caregiver's answer as the main one.

B **Incorrect** (trap: Right action, wrong time). Caregivers often add useful detail, and their input fits after the patient has answered. Letting the caregiver speak first and then asking the patient to agree makes the patient a reviewer of their own plan.

C **Incorrect** (trap: Misuses the technique). Yes-or-no questions can help in some situations, but they narrow what a capable patient can say about plans for home. Using them to save time trades the patient's full answer for the team's convenience.

Takeaway: With clients who use alternative communication, speak directly to them and allow time for their reply rather than turning to companions.

Sources: ADA Title III regulations, 28 CFR 36.303(c)(1); NASW Code of Ethics (2021), 1.02
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.4 Access: language, interpretation, ASL, physical, cultural, neurodiversity · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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68. ETH-1655 (rev 2) · Best answer: C

A social worker in a housing program volunteers for a ballot campaign to fund affordable housing, a measure that would help many clients. The campaign asks volunteers to share flyers urging a yes vote. The social worker plans to hand them out on home visits and tell clients that support is optional. Which action is **MOST** consistent with the NASW Code of Ethics?

A. Hand out the flyers, stressing that support is optional
B. Keep the election out of home-visit conversations
C. Offer nonpartisan voting information in place of the flyers
D. Give campaign flyers to clients who ask about the measure

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Saying support is optional sounds respectful, but the power gap remains: clients who rely on the worker for housing help may agree just to keep the relationship smooth. Stating that a request is voluntary does not remove the pressure of a partisan request from one's case manager.

B **Incorrect** (trap: Good but not best). Steering clear of the campaign avoids exploitation, but silence misses a chance to support clients' civic participation, which the Code encourages. Neutral voting information serves that aim without promoting the worker's side.

C **Correct.** Social workers should not use a professional relationship to advance their own political interests (NASW Code 1.06(b)), yet they should help the public take an informed part in shaping policy (6.02). Nonpartisan information on registering, voting and where to learn about ballot measures serves clients without pressing the worker's position. It beats A because clients who depend on the worker for housing help may not

feel free to refuse, so calling support optional does not remove the pressure. The worker remains free to campaign on personal time, away from clients.

D **Incorrect** (trap: Boundary slip or collusion). Waiting for clients to ask is less pushy, but handing over campaign material still enlists the professional relationship for the worker's cause. If a client asks, the worker can point to neutral sources that describe the measure.

Takeaway: Support clients' civic participation with nonpartisan information; do not use the professional relationship to advance the worker's political cause.

Sources: NASW Code of Ethics (2021), 1.06(b); NASW Code of Ethics (2021), 6.02
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** MOST · **Cognitive** reasoning · **Difficulty** 3 of 3

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69. ETH-0007 (rev 2) · Best answer: B

A social worker supervises a newly licensed social worker whose recent divorce is affecting their work. The supervisee has been late to sessions and tearful with clients. In supervision, the supervisee asks the supervisor to provide personal therapy for a few weeks, saying 'You already know my situation.' What is the supervisor's **BEST** response?

A. Agree to provide short-term therapy during supervision
B. Decline to provide therapy and address the impact on clients
C. Report the supervisee's impairment to the state licensing board

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). Becoming the supervisee's therapist creates a dual relationship. It undermines the supervisor's ability to evaluate the supervisee fairly and to protect clients. The supportive function of supervision does not extend to treatment.

B **Correct.** Supervisors should avoid providing psychotherapy to supervisees, because the dual role compromises fair evaluation and blurs accountability for client care (Supervision Standards, Standard 4; NASW Code 3.01(c)). The supervisor's job is to name the effect on clients, plan protective steps such as adjusting the caseload, and encourage the supervisee to seek outside help, which the Code expects of any social worker whose personal problems affect practice (NASW Code 4.05(b)). It beats C because the problem is recent and the supervisee is still reachable through supervision, so supportive and corrective steps come before a formal report.

C **Incorrect** (trap: Misreads law or ethics). Formal reporting fits impairment that continues after the colleague has been approached and has not taken adequate steps (NASW Code 2.08). The supervisor has a direct, earlier route: addressing the problem in supervision.

Takeaway: Supervision supports but never becomes therapy; address the impact on clients and refer the supervisee for personal help.

Sources: NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Standard 4 (Ethical Issues): Boundaries; NASW Code of Ethics (2021), 3.01(c); NASW Code of Ethics (2021), 4.05(b)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.7 Ethics of supervising and managing · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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70. ASM-1677 (rev 2) · Best answer: B

A client in outpatient treatment for gambling disorder had not placed a bet for 4 months. After a sports app sent a promotion, the client bet throughout a weekend and lost several hundred dollars. In session, the client says, 'Maybe I don't need to quit completely. I could just bet small amounts.' What should the social worker do **FIRST**?

A. Revise the relapse-prevention plan to block betting app promotions
B. Ask how the client now sees their recovery goals
C. Reassure the client that slips are common and progress remains

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Blocking the trigger that set off the lapse is a sound relapse-prevention step once the client recommits to a goal. Done first, it passes over the client's stated doubt about quitting and assumes an abstinence goal the client may no longer hold.

B **Correct.** Readiness is not fixed; after a lapse, people often cycle back to an earlier stage of change (Prochaska, DiClemente & Norcross, 1992). The client's new talk of betting small amounts signals that the abstinence goal itself is now in question. Asking how the client views their goals reassesses where they stand before the worker chooses a response. It beats A because revising the relapse-prevention plan assumes the client still holds the abstinence goal; if the goal has shifted, the revision pushes a plan the client no longer owns and invites argument (Miller & Rollnick, 2023).

C **Incorrect** (trap: Good but not best). Normalizing a lapse can ease shame and is often helpful. Here, though, the client is voicing a changed goal rather than shame, so reassurance alone leaves the shift in readiness unexamined.

Takeaway: After a lapse, reassess the client's current goals and stage before resuming relapse-prevention work; readiness can move backward.

Sources: Prochaska, J. O., DiClemente, C. C., & Norcross, J. C. (1992). In search of how people change: Applications to addictive behaviors. American Psychologist, 47(9), 1102-1114, spiral model of the stages of change; Miller & Rollnick (2023). Motivational Interviewing (4th ed.). Guilford Press, Sustain talk and discord
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.4 Assessing motivation, barriers, and readiness for change · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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71. ETH-1084 (rev 2) · Best answer: B

PRETEST — unscored on this form; do not count it

A client at a community agency receives disability benefits that limit how much the client may earn. In session, the client mentions doing cash yard work most weekends. The client then asks the social worker to sign a letter to the benefits office stating that the client has no income. What is the **MOST** appropriate response?

A. Sign a letter worded as the client's own report of no income
B. Decline to sign while keeping the disclosure private
C. Report the unreported earnings to the benefits office

Why each option is right or wrong

- **A** **Incorrect** (trap: Misreads law or ethics). Framing the statement as the client's report seems to keep the social worker honest. The worker knows about the cash earnings, though, so signing still lends the worker's name to a claim the worker believes is false, which is the deception 4.04 bars.
- **B** **Correct.** Two duties pull apart here. Without consent, the Code allows disclosure only for compelling professional reasons, such as preventing serious, foreseeable and imminent harm (NASW Code 1.07(c)), and unreported earnings are not such a reason. But signing a statement the social worker knows is false would mean taking part in fraud (4.04). Declining the letter while keeping the disclosure private honors both duties, and the worker can discuss options such as benefits counseling. B beats A because wording the letter as the client's own report still puts the worker's signature behind a statement the worker knows is false.
- **C** **Incorrect** (trap: Misreads law or ethics). Reporting would disclose confidential information without consent and without a compelling professional reason such as serious, imminent harm. Disagreeing with a client's conduct does not create grounds to breach confidentiality.

Takeaway: Keep client disclosures confidential unless serious imminent harm is at stake, but never sign or endorse a statement known to be false.

Sources: NASW Code of Ethics (2021), 1.07(c); NASW Code of Ethics (2021), 4.04
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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72. INT-1128 (rev 3) · Best answer: A

After 8 months of psychodynamic therapy for relationship problems, a client starts having panic attacks and now avoids driving on highways. The client asks to focus on the panic. When the social worker explores the request, the client says the attacks feel urgent and are hurting the client's job. The social worker is trained in CBT for panic disorder. What is the **BEST** response?

- **A.** Shift to treating the panic with interoceptive exposure
- **B.** Explore how the panic connects to the relationship themes
- **C.** Add a CBT panic workbook while continuing the current sessions

Why each option is right or wrong

- **A** **Correct.** Evidence-based practice joins research evidence, clinical expertise, and client preference. CBT with interoceptive exposure is a well-supported first-line treatment for panic disorder (Craske & Barlow, 2022). The client wants the panic addressed, avoidance is spreading to work, and the social worker has the training, so shifting the focus by agreement to this CBT method fits all three. It beats B because the worker has already explored the request; continuing to search for relational meaning delays an effective treatment while the avoidance grows.
- **B** **Incorrect** (trap: Good but not best). Linking symptoms to relationship themes is standard psychodynamic work, and it was right to explore what the request means. That step is done, and the client has said the attacks feel urgent. More interpretation now postpones a targeted, effective treatment.

C **Incorrect** (trap: Good but not best). CBT-based self-help can reduce panic and keeps the existing work going, so this is tempting. But the client wants the panic treated now, avoidance is spreading to work, and the worker is trained to deliver the treatment directly; a workbook beside sessions that stay on relationship themes addresses the urgent problem only in part.

Takeaway: When a new disorder emerges in therapy, offer an evidence-based treatment the client wants and the worker is trained to provide.

Sources: Craske, M. G., & Barlow, D. H. (2022). Mastery of Your Anxiety and Panic: Therapist Guide (5th ed.). Oxford University Press, treatment overview; interoceptive exposure

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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73. ASM-1577 (rev 2) · Best answer: A

PRETEST — unscored on this form; do not count it

A 15-year-old comes to a pediatric clinic with a parent for a sports physical. The social worker plans a brief psychosocial screen covering substance use, sexual activity and mood. The parent sits beside the teen and starts answering the social worker's first questions. What should the social worker do **FIRST**?

A. Explain confidentiality and its limits, then meet with the teen alone

B. Ask the teen the screening questions in a calm, matter-of-fact way

C. Assure the teen that everything said today will stay private

Why each option is right or wrong

A **Correct.** Adolescents disclose substance use, sexual activity and mood symptoms more honestly in private time with the provider after hearing what stays confidential and what does not, such as serious risk of harm (Hagan et al., Bright Futures). With the parent answering, the teen's own account is lost, so private time comes before the sensitive questions. Explaining the limits honors the duty to discuss confidentiality at the outset (NASW Code 1.07(e)). It beats B because a matter-of-fact tone helps, but asking with the parent present makes honest answers unlikely.

B **Incorrect** (trap: Right action, wrong time). A calm, normalizing tone is good technique for sensitive topics, but asking these questions with the parent beside the teen comes too early. Set up private time and explain confidentiality first, then use this approach.

C **Incorrect** (trap: Misreads law or ethics). Promising complete privacy is inaccurate. Disclosures of abuse or serious risk of harm can require action, so the teen must hear the limits before speaking. A false promise can damage trust later.

Takeaway: Before asking adolescents about sensitive topics, explain confidentiality and its limits and arrange time alone with the teen.

Sources: Hagan, J. F., Shaw, J. S., & Duncan, P. M. (Eds.) (2017). Bright Futures: Guidelines for Health Supervision of Infants, Children, and Adolescents (4th ed.). American Academy of Pediatrics, Adolescence visits: confidentiality and time alone with the adolescent; NASW Code of Ethics (2021), 1.07(e)

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.7 Eliciting sensitive or confidential information (substance use, sexual abuse) · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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74. INT-1657 (rev 1) · Best answer: C

In a home-visiting program, a parent asks the social worker how to use time-out for a 3-year-old who hits a younger sibling. The parent says most of their time together is spent giving commands or corrections, with little relaxed time. The hitting has caused no injuries. What should the social worker do **FIRST**?

A. Teach a consistent time-out procedure for each hitting incident
B. Explore how the parent was disciplined as a child
C. Coach brief daily child-led play with specific praise

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Time-out is an effective tool and the parent asked for it, so it will likely be taught. Taught first, in a relationship made up mostly of corrections, it has little positive time to contrast with and often turns into power struggles.

B **Incorrect** (trap: Good but not best). A parent's own history can shape discipline and may be worth discussing. As a first step it gives the parent no skill to use, and the clearest need described is too little positive interaction.

C **Correct.** Behavioral parent training builds a warm, positive relationship before adding discipline; in Parent-Child Interaction Therapy, the child-directed phase, with praise and attention for positive behavior, comes before the discipline phase (McNeil & Hembree-Kigin, 2010). This parent's time with the child is mostly commands and corrections, so time-out would land in a relationship with little positive attention to lose. Daily play builds that base while the parent simply separates the children if hitting occurs. It beats A because time-out works best once positive time is plentiful; taught first, it often fails and escalates conflict.

Takeaway: Build positive parent-child time with praise and child-led play before teaching discipline techniques such as time-out.

Sources: McNeil, C. B., & Hembree-Kigin, T. L. (2010). Parent-Child Interaction Therapy (2nd ed.). Springer, child-directed interaction before parent-directed interaction
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.6 Building parenting capacity and skills · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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75. ASM-1537 (rev 1) · Best answer: C

A parent remarried 4 months ago. The new spouse, who has no children, began enforcing strict curfews and chore rules with the parent's 13-year-old. The teen now slams doors and refuses to speak to the stepparent. The couple asks the social worker how to restore order at home. Which guidance is **BEST**?

A. Have the parent and stepparent agree on rules and enforce them equally
B. Have the stepparent stay out of the teen's daily life for now
C. Have the parent handle discipline while the stepparent builds a bond

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Agreeing on rules matters, and the couple should do so privately. Having the stepparent enforce them as an equal authority this early, before a relationship with the teen exists, tends to provoke the kind of rebellion the couple is seeing.

B **Incorrect** (trap: Misuses the technique). Stepping back from discipline is wise, but withdrawing from the teen's daily life altogether gives up the chance to build the relationship that later makes the stepparent's authority possible. Connection, not distance, is the stepparent's early task.

C **Correct.** Stepfamily guidance holds that a new stepparent has little authority at first and should build a relationship before taking on discipline, while the biological parent remains the main disciplinarian (Papernow, 2013). Four months in, a stepparent enforcing strict rules has set off the teen's resistance. It beats A because a united front on rules is important, but having the stepparent enforce them equally before a bond exists produces exactly this conflict; the adults can agree on rules privately while the parent delivers them.

Takeaway: In new stepfamilies, the biological parent leads discipline while the stepparent builds a relationship first; authority grows from connection.

Sources: Papernow, P. L. (2013). Surviving and Thriving in Stepfamily Relationships: What Works and What Doesn't. Routledge, parenting challenge: stepparent role and discipline
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.3 Relationship dynamics and development: families, couples, groups, polyamory · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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76. ETH-0005 (rev 1) · Best answer: C

A social worker is the only mental health provider in a small rural town. A new client's child plays on the same youth soccer team as the social worker's child, so they will run into each other at weekly games. The nearest other provider is 2 hours away. What is the **MOST** appropriate action?

A. Refer the client to the provider who is 2 hours away
B. Continue treatment without raising the overlap with the client
C. Agree with the client on how to handle outside contact

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). Referral fits when an overlapping relationship risks exploitation or impaired judgment. Here the contact is incidental, and a 2-hour trip could put care out of reach. Referring would shift the social worker's boundary task onto the client.

B **Incorrect** (trap: Misreads law or ethics). Saying nothing leaves the client unprepared for public contact and risks an accidental breach, such as a friendly greeting that reveals the relationship. Managing boundaries is the social worker's job, and it starts with a conversation.

C **Correct.** Dual relationships that cannot reasonably be avoided are not automatically unethical. The social worker is responsible for setting clear, appropriate boundaries that protect the client (NASW Code 1.06(c)). Talking with the client ahead of time about greetings, privacy and conversation at games, then documenting the plan and consulting as needed, manages the risk. It beats A because sending the client 2 hours away creates a real barrier to care when the overlap is incidental and manageable. Rural practice ethics accepts that some overlap cannot be avoided (Reamer, 2020).

Takeaway: When a dual relationship can't be avoided, the social worker sets clear boundaries with the client, documents the plan and consults.

Sources: NASW Code of Ethics (2021), 1.06(c); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, unavoidable dual relationships in small communities
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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77. INT-1508 (rev 1) · Best answer: A

A client with schizophrenia who lives in supportive housing tells the social worker, 'I want a real job, like stocking shelves.' The client's symptoms have been stable on medication, but the client has not worked in 8 years. Housing staff say a job is unrealistic for now. What is the social worker's **BEST** response?

A. Refer the client to a supported employment program now
B. Enroll the client in a prevocational work-readiness group first
C. Explore whether the client's job goal is realistic right now
D. Ask the treatment team to decide when the client is ready

Why each option is right or wrong

A **Correct.** The strengths perspective assumes no upper limit on growth and lets the client's own goals lead (Rapp & Goscha, 2012). Supported employment in the Individual Placement and Support model accepts anyone who wants to work and starts the job search quickly, without readiness tests (Drake, Bond & Becker, 2012). The client is stable and has named a specific goal, so linkage now honors it. It beats B because readiness groups delay competitive work, and placing people in jobs with ongoing support has stronger employment outcomes.

B **Incorrect** (trap: Wrong model or concept). Training before placement sounds cautious, but the evidence favors placing people in competitive jobs quickly and supporting them on the job. Readiness groups often delay real work or replace it.

C **Incorrect** (trap: Overrides the client's choice). Questioning whether the goal is realistic starts from the staff's doubts rather than the client's strengths. Exploring the client's hopes and job preferences would fit; testing the goal's realism signals low expectations.

D **Incorrect** (trap: Overrides the client's choice). Handing the timing to the team overrides a stable client's own choice. Supported employment eligibility rests on the client's wish to work, not on staff judgments of readiness.

Takeaway: Strengths-based practice follows the client's goal without assumed limits; for a work goal with serious mental illness, link to supported employment.

Sources: Rapp, C. A., & Goscha, R. J. (2012). The Strengths Model: A Recovery-Oriented Approach to Mental Health Services (3rd ed.). Oxford University Press, principles of the strengths model; Drake, R. E., Bond, G. R., & Becker, D. R. (2012). Individual Placement and Support: An Evidence-Based Approach to Supported Employment. Oxford University Press, IPS principles: zero exclusion, rapid job search
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.1 Strengths-based and empowerment interventions · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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78. ETH-1123 (rev 2) · Best answer: A

A social worker in solo private practice gave clients a personal cell number when the practice opened and now answers their texts most evenings and weekends. The worker sleeps poorly, feels irritable in sessions, and recently answered a client's late-night scheduling text sharply. What is the **MOST** effective way to address the problem?

A. Set after-hours limits and explain crisis options to clients
B. Begin personal counseling for the stress and poor sleep
C. Stop replying to evening texts without telling clients

Why each option is right or wrong

A **Correct.** Professional self-care is essential to competent, ethical practice (NASW Code, Purpose), and constant availability is draining the worker's sleep and patience with clients. Social workers discuss their technology policies with clients (1.03(e)), so a clear after-hours policy, with information on crisis lines and emergency services, protects both the worker's well-being and clients' safety. A beats B because counseling can ease the strain, but the cause, unlimited availability through a personal phone, would continue.

B **Incorrect** (trap: Good but not best). Personal counseling is a sound form of self-care, which the Code treats as essential, so this is tempting. It treats the effects while the arrangement causing them, constant after-hours texting, goes on unchanged.

C **Incorrect** (trap: Ignores risk). Going silent without notice could leave a client in crisis waiting for a reply that never comes. Any change in availability must be explained, with alternatives for urgent needs.

Takeaway: Sustainable self-care includes clear limits on availability, explained to clients along with crisis alternatives.

Sources: NASW Code of Ethics (2021), Purpose (self-care paragraph); NASW Code of Ethics (2021), 1.03(e)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.7 Principles of self-care for the social worker · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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79. ETH-1565 (rev 2) · Best answer: A

A client with major depressive disorder has employer-sponsored health insurance through a large company. The plan denied payment for therapy after 12 visits this year, citing a visit cap. The social worker reviewed the plan documents with the client and found no visit cap on outpatient medical care. What should the social worker do **NEXT**?

A. Help the client appeal the decision under federal parity rules
B. Switch the client to a reduced self-pay fee starting right away
C. Tell the client the plan broke the law and should be sued

Why each option is right or wrong

A **Correct.** Federal parity law bars covered group health plans from placing visit limits on mental health benefits that are stricter than those applied to most medical and surgical benefits (29 U.S.C. 1185a). A therapy-only cap in a large employer's plan raises exactly this concern. Helping the client file an appeal that cites the law uses the plan's own process and keeps treatment going, serving the client's interests first (NASW Code 1.01). It beats B because a reduced fee may be needed if the appeal fails, but starting there gives up coverage the client may be owed.

B **Incorrect** (trap: Good but not best). A reduced fee can keep treatment going if coverage is truly unavailable. Moving to self-pay first gives up benefits the client may be entitled to and shifts a cost the plan may owe onto the client.

C **Incorrect** (trap: Outside the social work role). Declaring that the plan broke the law and recommending a lawsuit is a legal judgment outside the social work role. The appeal process comes first, and seeking legal counsel is the client's choice if it fails.

Takeaway: When insurance limits mental health care more strictly than medical care, help the client use appeal rights under federal parity law.

Sources: Mental Health Parity and Addiction Equity Act of 2008, 29 U.S.C. 1185a(a)(3)(A); NASW Code of Ethics (2021), 1.01

Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.8 How local, state, federal policy affects service delivery · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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80. INT-1044 (rev 2) · Best answer: C

A 38-year-old client moved across the country 10 years ago and has had almost no contact with their family since. The client reports a series of intense, short romantic relationships and wants help with them. The social worker uses Bowen family systems therapy. Which intervention is **BEST**?

A. Invite the parents to a session to confront them about the past

B. Explore the pattern in the client's romantic relationships

C. Coach the client to plan brief visits and stay calm during them

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Confronting parents about the past invites the high emotion and blame that Bowen therapy works to lower. The model favors calm, one-to-one contact in which the client observes and manages their own reactions.

B **Incorrect** (trap: Wrong model or concept). Looking at the romantic pattern is a natural start, and it is what the client asked about. Bowen theory, though, places its source in unresolved attachment to the family of origin, so working only on current partners leaves the cutoff that drives the pattern untouched.

C **Correct.** Bowen theory treats emotional cutoff, managing unresolved family issues through distance, as a sign of low differentiation; the issues stay dormant and often resurface in other close relationships (Bowen, 1978). The client's intense, short relationships fit that pattern. Bowen-oriented work coaches the client to re-establish contact and practice staying calm and thoughtful, speaking from an I-position, when old reactions arise. It beats A because confrontation would heighten emotional reactivity, the opposite of the calm, differentiated contact the model seeks.

Takeaway: Bowen therapy views emotional cutoff as unresolved attachment; coach calm, differentiated reconnection rather than confrontation or continued distance.

Sources: Bowen, M. (1978). Family Therapy in Clinical Practice. Jason Aronson, emotional cutoff; differentiation of self

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.21 Family therapy models and interventions · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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81. ASM-1687 (rev 1) · Best answer: C

Staff at a nursing facility ask the social worker to begin weekly talk therapy for a resident with moderate Alzheimer's disease. The resident yells at and pushes away aides several times a week, usually during morning bathing. The resident cannot recall conversations from earlier in the day. What is the **MOST** appropriate approach?

A. Hold weekly individual sessions exploring the resident's frustrations

B. Ask the physician to consider medication to calm the resident

C. Help caregivers adjust the routine that sets off the distress

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Reflective sessions depend on remembering and building on earlier conversations. With moderate Alzheimer's disease, any gains would not carry over to the bathing routine where the distress actually happens.

B **Incorrect** (trap: Right action, wrong time). Medication may be considered for severe or dangerous symptoms, but antipsychotics carry a boxed warning of increased mortality in older adults with dementia-related psychosis (Kaplan & Sadock's Synopsis). Behavioral and environmental changes come first here.

C **Correct.** Modality should match the client's abilities. With moderate memory loss, the resident cannot carry insights from one session to the next, so a caregiver-delivered behavioral approach fits better. Outbursts cluster around morning bathing, and an expert panel recommends describing such patterns and having caregivers adjust routines and surroundings, generally before turning to medication (Kales, Gitlin & Lyketsos, 2014). It beats A because weekly talk sessions rely on recall and reflection the resident no longer has, while aides are present at the moment the distress happens.

Takeaway: Choose interventions that fit cognitive ability; for agitation in moderate dementia, caregiver-delivered changes to triggers come before talk therapy or medication.

Sources: Kales, H. C., Gitlin, L. N., & Lyketsos, C. G. (2014). Management of neuropsychiatric symptoms of dementia in clinical settings: Recommendations from a multidisciplinary expert panel. Journal of the American Geriatrics Society, 62(4), 762-769, DICE approach; Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Antipsychotics: boxed warning in older adults with dementia-related psychosis

Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.3 Choosing intervention modality by ability, development, culture, life stage · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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82. ASM-1508 (rev 1) · Best answer: C

A school social worker meets with the parent of a 14-year-old. Over the past 2 months, the teen has stopped seeing friends, quit the swim team, sleeps most afternoons and has dropped from B's to failing grades. The parent says, 'It's just a moody teenage phase.' What should the social worker do **FIRST**?

A. Agree that these changes are common in adolescence
B. Refer the teen to a psychiatrist for medication
C. Meet with the teen to screen for depression
D. Set up a daily homework check with teachers

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Ordinary adolescent moodiness is brief and does not erase friendships, activities and grades. Agreeing with the parent normalizes a cluster of changes that may signal depression and delays needed assessment.

B **Incorrect** (trap: Skips assessment). Medication decisions belong to a prescriber after a full evaluation. Sending the teen for medication before any screening skips the assessment that would show whether a psychiatric referral is needed.

C **Correct.** Some moodiness is typical in adolescence, but a marked, lasting change across several areas of functioning is atypical and calls for assessment (Zuckerbrot et al., 2018). Two months of withdrawal from friends, quitting a valued activity, excess sleep and a sharp grade drop suggest possible depression, so screening the teen directly, including asking about suicidal thoughts, comes first. It beats D because a homework plan targets the grades but not the likely cause, and could leave a depressed teen unassessed. Screening results then guide referral and school supports.

D **Incorrect** (trap: Right action, wrong time). School supports may help later, once the cause of the decline is known. A homework plan made first targets one symptom and leaves the possible mood disorder unassessed.

Takeaway: Distinguish typical adolescent moodiness from a lasting, multi-area change in functioning, which calls for direct screening before planning supports.

Sources: Zuckerbrot, R. A., Cheung, A., Jensen, P. S., Stein, R. E. K., Laraque, D., & GLAD-PC Steering Group (2018). Guidelines for Adolescent Depression in Primary Care (GLAD-PC): Part I. Pediatrics, 141(3), e20174081, identification and assessment

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.16 Typical vs. atypical development across the lifespan · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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83. INT-1511 (rev 1) · Best answer: D

During a home visit, a parent tells a family services social worker that the school denied the child's request for a classroom aide at an IEP meeting. The parent says, 'I feel like the only one fighting this school.' The parent also mentions that several other families in the program have had the same request denied. What is the **MOST** appropriate action?

A. Attend the next IEP meeting and present the parent's case to staff
B. Teach the parent relaxation skills to use in school meetings
C. File a complaint with the state education agency for the parent
D. Connect the parent with a parent-led disability advocacy network

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Presenting the case is direct advocacy and may help in the short term, but it does for the parent what the parent can learn to do and leaves the parent isolated. Attending as a support while the parent speaks would fit empowerment better.

B **Incorrect** (trap: Wrong model or concept). Coping skills may ease stress in meetings, but they treat a shared, structural problem as a personal one and leave the parent alone against the school.

C **Incorrect** (trap: Overrides the client's choice). A state complaint is a legitimate formal channel, but filing it for the parent bypasses the parent's own voice and decision. It may be a later step the parent chooses with support.

D **Correct.** Empowerment practice reduces powerlessness by linking people who share a problem so they can act together, building knowledge, skills, and collective voice (Gutierrez, 1990). The parent feels isolated, and other families face the same denials, so a parent-led advocacy network both ends the isolation and strengthens the parent's capacity to press the school. It beats A because speaking for the parent may win this request but leaves the parent dependent and alone. The worker can still attend meetings as support.

Takeaway: When clients share a problem rooted in a system, empowerment links them to act together rather than having the worker act for them.

Sources: Gutierrez, L. M. (1990). Working with women of color: An empowerment perspective. Social Work, 35, 149-153, empowerment through connection and collective action

Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.1 Strengths-based and empowerment interventions · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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84. INT-1106 (rev 1) · Best answer: B

PRETEST — unscored on this form; do not count it

On a stroke rehabilitation unit, a 68-year-old patient has expressive aphasia. The patient follows conversation and nods appropriately but struggles to find words. The social worker needs the patient's preferences for discharge. The patient's adult child offers to answer for the patient. What is the **MOST** appropriate way for the social worker to proceed?

A. Ask the adult child to describe the patient's preferences

B. Use short, simple questions and allow ample time to answer

C. Finish the patient's sentences when the patient pauses

D. Give the patient written questions to answer later

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Relatives often know a great deal, but letting the adult child speak for a patient who understands, and can communicate with support, takes the decision away from the patient. Family input fits as a supplement, with the patient's agreement.

B **Correct.** Aphasia affects language, not the right to decide. Because the patient understands speech, the social worker communicates directly: short, uncomplicated sentences, fewer distractions, acceptance of gestures or pointing, and plenty of time to respond (NIDCD). This keeps the patient at the center of a discharge decision that is

theirs to make. It beats A because the adult child can add information with the patient's agreement, but answering for a patient who can express preferences, given time, sidelines the patient (NASW Code 1.02).

C **Incorrect** (trap: Misuses the technique). Completing sentences seems helpful but puts words in the patient's mouth and cuts off the extra time people with aphasia need. Guidance for families advises allowing time and accepting any form of communication instead.

D **Incorrect** (trap: Good but not best). Written questions help some patients, but expressive aphasia often affects writing as well as speech, and deferring the conversation delays the patient's input on discharge. Writing key words during the talk is a better use of print.

Takeaway: With aphasia, communicate directly using short sentences, extra time and any form of expression; don't let others speak for a patient who understands.

Sources: NIDCD (National Institute on Deafness and Other Communication Disorders). Aphasia (NIH fact sheet), tips for communicating with a person with aphasia; NASW Code of Ethics (2021), 1.02
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.2 Verbal and nonverbal communication; congruence, body language · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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85. INT-1162 (rev 2) · Best answer: A

In week 4 of a 10-week hospice bereavement group, a member whose partner died 3 months ago stays after the session. In response to the social worker's questions, the member says they now drink most of a bottle of wine every night to sleep, have missed 6 workdays this month, and their adult child has asked them to cut back. What should the social worker do **NEXT**?

A. Help the member arrange a substance use evaluation and stay in group
B. Suggest a bedtime routine and check on the drinking next week
C. Explore further how the loss has affected the member's sleep
D. Ask the member to pause group until the drinking is under control

Why each option is right or wrong

A **Correct.** Social workers refer clients when another professional's expertise is needed to serve them fully (NASW Code 1.16(a)). Nightly heavy drinking, missed work, and family concern go beyond what a time-limited bereavement support group can address. Linking the member to a substance use evaluation while keeping them in the group adds needed care without removing a support. It beats B because a sleep routine and a check-in show care but leave a pattern that is already costing work and worrying family unassessed for another week.

B **Incorrect** (trap: Ignores risk). A bedtime routine is reasonable grief support, and checking back shows care. Nightly heavy drinking with missed work and family concern, though, calls for a professional substance use assessment now; waiting a week while the pattern continues under-reacts to the risk.

C **Incorrect** (trap: Ignores risk). Exploring grief and sleep fits a bereavement group, but the member has already described the drinking and its effects. More exploration postpones the step those facts call for while the health risk continues.

D **Incorrect** (trap: Hands off too soon). Making return depend on controlled drinking removes a support during a high-risk time and makes help conditional. Grief support and substance use care can run at the same time.

Takeaway: When a group member's needs exceed the group's scope, add a referral for specialized assessment while keeping the member's group support.

Sources: NASW Code of Ethics (2021), 1.16(a)
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.23 Group work: managing process and building cohesion · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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86. ASM-1663 (rev 1) · Best answer: B

PRETEST — unscored on this form; do not count it

In a second session, a client is describing a pregnancy loss that happened 2 months ago. Mid-sentence, the client stops, looks down and begins to cry softly. The client has spoken openly so far and has not asked for anything. What response would be **MOST** helpful at this moment?

A. Ask what thoughts are coming up for the client right now
B. Stay quietly present and let the client resume when ready
C. Reassure the client that this kind of grief reaction is normal

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Inviting the client to put thoughts into words is useful a little later. Asked right as the tears start, it interrupts the feeling and moves the client into explaining before they have had a moment with it.

B **Correct.** Purposeful silence gives a client room to feel and to collect thoughts, and attentive silence after strong emotion conveys acceptance (Hepworth et al.). This engaged client is in the middle of grief and has not asked for help, so quiet, attentive presence lets the feeling unfold and leaves the next step to the client. It beats A because an open question is a sound skill, but asked in the first moments of tears it pulls the client into explaining before they are ready. If the silence stretches or the client seems stuck, a gentle prompt then fits.

C **Incorrect** (trap: Good but not best). Normalizing grief can comfort, but offered at once it can feel like an effort to close down the emotion. It also puts the social worker's words in place of the client's experience at a tender moment.

Takeaway: After a client's strong emotion, attentive silence often helps more than questions or reassurance; prompt gently only if the client seems stuck.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Use of silence; attending and empathic communication
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.2 Interviewing skills: clarifying, validating, confronting, reflecting, interpreters · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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87. INT-1653 (rev 1) · Best answer: B

A parent of two arrives upset, saying the family's food assistance 'just got cut off out of nowhere.' The parent holds a letter from the benefits office but has not read it closely and asks the social worker to call the office director right away. What should the social worker do **FIRST**?

A. Call the office director to object to the cutoff
B. Review the notice together to find the stated reason
C. Help the parent submit a written fair hearing request today

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). Calling leadership may become useful later, but without the stated reason the social worker cannot make a specific case. Starting at the top also bypasses the frontline fix and the formal appeal route that may resolve it.

B **Correct.** Effective case advocacy starts with the facts: what action was taken, why, and what deadlines and appeal rights apply (Hepworth et al.). A notice of adverse action must state the reason and the right to a fair hearing (7 CFR 273.13(a)(2)), so reading it together takes minutes and shows whether the problem is a missing document, an error, or something only a hearing can fix. It beats C because a hearing may well be needed, but the notice may point to a quicker fix and will show the deadline and whether benefits can continue meanwhile.

C **Incorrect** (trap: Right action, wrong time). A fair hearing protects the family's rights and may be the right step. Filing before reading the notice risks missing a quicker fix, such as sending a missing pay stub, and leaves the request without the facts it needs.

Takeaway: Before advocating against a benefits decision, read the notice with the client to learn the reason, deadlines and appeal rights.

Sources: Supplemental Nutrition Assistance Program regulations, 7 CFR 273.13(a)(2); Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Advocacy and social action: case and cause advocacy
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.13 Advocating for needed services and resources · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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88. ETH-1118 (rev 2) · Best answer: A

What term names the good feelings helpers get from doing trauma work well?

A. Compassion satisfaction
B. Vicarious resilience
C. Posttraumatic growth

Why each option is right or wrong

A **Correct.** Compassion satisfaction is the positive feeling that comes from performing competently as a trauma professional (NCTSN, 2011). It is the positive counterpart to burnout and secondary traumatic stress in a helper's professional quality of life. A beats B because vicarious resilience refers to a helper's own growth in coping that comes from witnessing clients' resilience (Hernández et al., 2007), not the reward of competent work and collegial support.

B **Incorrect** (trap: Wrong model or concept). Vicarious resilience is a positive transformation in helpers who learn from the ways clients overcome adversity. It describes learning from clients' resilience, not the reward of doing the work well.

C **Incorrect** (trap: Wrong model or concept). Posttraumatic growth describes positive change in a person after their own struggle with trauma (Tedeschi & Calhoun, 2004). It is not specific to the rewards of helping others.

Takeaway: Compassion satisfaction, the reward of competent helping, is the positive counterpart to burnout and secondary traumatic stress.

Sources: National Child Traumatic Stress Network, Secondary Traumatic Stress Committee (2011). Secondary Traumatic Stress: A Fact Sheet for Child-Serving Professionals, Sidebar: Secondary Traumatic Stress and Related Conditions; Hernández, P., Gangsei, D., & Engstrom, D. (2007). Vicarious resilience: A new concept in work with those who survive trauma. Family Process, 46(2), 229-241, whole article; Tedeschi, R. G., & Calhoun, L. G. (2004). Posttraumatic growth: Conceptual foundations and empirical evidence. Psychological Inquiry, 15(1), 1-18, whole article

Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.6 Burnout, compassion fatigue, and secondary traumatic stress · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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89. INT-1036 (rev 1) · Best answer: D

In a middle school group on handling peer pressure, a 13-year-old accurately lists and acts out the refusal steps the social worker modeled. Later, the student says that when an older cousin turned down a vape at a party, friends mocked the cousin for weeks. Since then, the student laughs along when offered a vape. Which part of observational learning is **MOST** likely blocking use of the skill?

A. Attention
B. Retention
C. Reproduction
D. Motivation

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Attention problems would show up as missing key parts of the model. The student watched closely enough to list every refusal step, so attention is working.

B **Incorrect** (trap: Wrong model or concept). Retention means storing the behavior in memory. The student can still list the steps accurately, which shows the skill has been retained.

C **Incorrect** (trap: Wrong model or concept). Reproduction is the ability to perform what was observed. The student acted out the steps in the group, so the gap lies in whether they choose to use the skill with peers.

D **Correct.** Bandura described four processes in observational learning: attention, retention, reproduction, and motivation (Bandura, 1977). A person can learn a behavior fully and still not use it if they expect it to be punished. Watching the cousin get mocked was vicarious punishment, which lowers the incentive to refuse. The student's accurate recall and performance show the skill was attended to, stored, and can be produced. It beats C because the student has already acted out the steps in the group, so the problem is the expected cost, not the ability.

Takeaway: Learning a modeled behavior differs from performing it; seeing a model punished lowers motivation even when the skill is learned.

Sources: Bandura, A. (1977). Social Learning Theory. Prentice-Hall, observational learning: attention, retention, reproduction, motivation

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.26 Using role modeling to teach behavior · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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90. ASM-1046 (rev 2) · Best answer: C

During a home visit, a parent worries that their 5-year-old talks and plays with an invisible friend. The child sets a place for the friend at meals but says, 'My friend is just pretend.' The child attends preschool, plays well with peers and sleeps normally. What is the **MOST** helpful response to the parent?

A. Recommend an evaluation for possible hallucinations
B. Suggest the parent gently discourage the pretend friend
C. Explain that imaginary companions are common at this age

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). An evaluation would fit if the child believed the friend was real, seemed distressed or showed other changes. A friend the child knows is pretend, with good functioning, is typical development.

B **Incorrect** (trap: Good but not best). Imaginary play is normal and can support social skills and creativity. Asking the parent to discourage it treats ordinary development as a problem and adds no benefit.

C **Correct.** Imaginary companions are common in early childhood, and children who have them usually know the friend is pretend and function well socially (Taylor, 1999). This child calls the friend pretend, attends preschool, plays with peers and sleeps normally, so nothing suggests mental illness. Explaining this answers the parent's worry directly. It beats B because discouraging normal pretend play is unnecessary and may signal to the child that something is wrong.

Takeaway: An imaginary companion that a young child knows is pretend, with good functioning, is typical development, not a sign of mental illness.

Sources: Taylor, M. (1999). Imaginary Companions and the Children Who Create Them. Oxford University Press, characteristics and prevalence of imaginary companions

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.16 Typical vs. atypical development across the lifespan · **Qualifier** MOST · **Cognitive** application · **Difficulty** 1 of 3

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91. ETH-1617 (rev 1) · Best answer: A

A 15-year-old came out as gay to the parents a month ago and is now withdrawn at home; a screening last week found no suicidal thoughts. In a family session, the parents say, 'We love our child, but our faith teaches this is wrong. We won't let these friends visit, and we pray for a change.' The teen looks down and says nothing. Which approach is **BEST**?

A. Show how specific accepting behaviors protect the teen's health
B. Ask the parents to put their religious beliefs aside for the teen's sake
C. See the teen individually and pause the family sessions

Why each option is right or wrong

A **Correct.** Family responses to a youth's sexual orientation shape the youth's well-being: young adults who recalled high family rejection in adolescence reported far higher rates of suicide attempts and depression (Ryan et al., 2009). These parents express love alongside rejecting behaviors, such as banning friends and praying for change. Helping them see how specific behaviors affect their child, and that support can coexist with faith, works with their love rather than against their beliefs (NASW Code 1.05(c)). It beats B because demanding they drop their faith sets up a contest that is likely to end engagement.

B **Incorrect** (trap: Too directive). The parents' religious values are part of their identity, and asking them to abandon those values sets up a contest the social worker cannot win. Change is more likely when the focus is on specific behaviors.

C **Incorrect** (trap: Good but not best). Individual support for the teen may be needed alongside family work. Pausing family sessions gives up the chance to change the rejecting behaviors at home, which drive much of the risk.

Takeaway: When families reject a youth's sexual orientation, link specific accepting or rejecting behaviors to the youth's health without demanding they abandon beliefs.

Sources: Ryan, C., Huebner, D., Diaz, R. M., & Sanchez, J. (2009). Family rejection as a predictor of negative health outcomes in white and Latino lesbian, gay, and bisexual young adults. Pediatrics, 123(1), 346-352, whole article; NASW Code of Ethics (2021), 1.05(c)

Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.3 Culture, race, gender, sexuality, intersectionality shaping identity and behavior · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 1 of 3

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92. ETH-1620 (rev 1) · Best answer: C

In a pediatric clinic, a Black parent tells the social worker about recent talks with an 11-year-old. The parent explained that some people may treat the child unfairly because of race, then practiced with the child how to stay calm and respond if stopped by police or followed in a store. Which ethnic-racial socialization practice is the parent **MOST** likely using?

A. Promotion of mistrust
B. Cultural socialization
C. Preparation for bias
D. Egalitarianism

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Promotion of mistrust emphasizes caution and distrust toward other racial groups and lacks coping guidance. This parent teaches concrete responses to unfair treatment, which marks a different practice.

B **Incorrect** (trap: Wrong model or concept). Cultural socialization teaches children about their heritage, history and traditions to build pride. The parent's talks center on unfair treatment and how to respond, not on heritage or cultural practices.

C **Correct.** Preparation for bias refers to parents making children aware of discrimination and teaching ways to cope with it (Hughes et al., 2006). This parent names possible unfair treatment because of race and rehearses calm, practical responses to a police stop or store surveillance. It beats A because promotion of

mistrust stresses wariness of other groups without coping strategies, while this parent focuses on skills for handling unfair treatment safely. Recognizing the practice helps the social worker see protective parenting rather than fearfulness.

D **Incorrect** (trap: Wrong model or concept). Egalitarianism emphasizes individual qualities and equality across groups, often downplaying race. This parent's message centers race and how others may respond to it.

Takeaway: Teaching children that unfair treatment may happen because of race, and how to cope with it, is preparation for bias, a protective practice.

Sources: Hughes, D., Rodriguez, J., Smith, E. P., Johnson, D. J., Stevenson, H. C., & Spicer, P. (2006). Parents' ethnic-racial socialization practices: A review of research and directions for future study. Developmental Psychology, 42(5), 747-770, whole article
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.3 Culture, race, gender, sexuality, intersectionality shaping identity and behavior · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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93. ASM-1634 (rev 1) · Best answer: C

At a family intake, parents say their 15-year-old's silence and closed bedroom door force them to keep checking in and asking questions. The teen says the constant questions are the reason for staying in the room. Each side asks the social worker to say who is right. What should the social worker do **FIRST** to understand how the family functions?

A. Determine which family member's behavior started the conflict
B. Coach the parents to give the teen more privacy
C. Map how each person's response sets off the other's

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Searching for who started it reflects linear cause-and-effect thinking. In an ongoing pattern each behavior maintains the other, so naming a first cause adds blame without explaining why the cycle continues.

B **Incorrect** (trap: Right action, wrong time). Giving the teen more privacy might later be part of a plan. Recommending it at intake sides with the teen and skips the assessment of how both sides keep the pattern going.

C **Correct.** Systems thinking treats family problems as circular: each person's behavior is both a response to and a trigger for the others'. Here the parents' questions and the teen's withdrawal feed each other, so the first assessment task is to trace that repeating sequence rather than assign blame. It beats B because coaching the parents is an intervention that takes the teen's side before the pattern has been mapped, and the teen's part in it would go unexamined (Nichols & Davis, 2020).

Takeaway: Assess family problems as circular sequences in which each member's behavior both triggers and responds to others', rather than seeking a first cause.

Sources: Nichols, M. P., & Davis, S. D. (2020). Family Therapy: Concepts and Methods (12th ed.). Pearson, systems concepts: circular causality
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.2 Family functioning and dynamics and their effects · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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94. ETH-1187 (rev 2) · Best answer: A

At a community mental health agency, a psychiatrist asks the social worker on a 9-year-old's treatment team to give the child a standardized intelligence test to help clarify a diagnosis. The social worker has never been trained to administer or interpret such tests. A psychologist on staff provides cognitive testing. What is the **MOST** appropriate response by the social worker?

A. Refer the child to the psychologist for testing
B. Give the test after studying its administration manual closely
C. Do the testing and have the psychologist score it
D. Describe the child's abilities from interview observations

Why each option is right or wrong

A **Correct.** The NASW Code's Competence principle and 1.04(a) limit practice to areas of education, training and supervised experience, and 1.16(a) calls for referral when another professional's specialized knowledge is needed. The social worker is untrained in standardized cognitive testing, and a qualified psychologist is on staff, so referral serves the child and the diagnostic question. It beats C because scoring help does not fix the problem: valid scores depend on standardized administration, which the social worker has not learned (AERA, APA, & NCME, 2014).

B **Incorrect** (trap: Outside the social work role). Reading a manual is not the supervised training that standardized testing requires. Errors in giving the test can produce misleading scores that shape the child's diagnosis and services.

C **Incorrect** (trap: Outside the social work role). Having an expert score the test still leaves administration to someone untrained, and administration errors can make scores invalid. Shared work fits only after the social worker has been trained and supervised in the instrument.

D **Incorrect** (trap: Good but not best). Interview observations add useful context, but they cannot substitute for a standardized measure when the psychiatrist needs one. Offering them in place of testing leaves the diagnostic question unanswered.

Takeaway: When a task requires training you lack and a qualified colleague is available, refer rather than attempt it.

Sources: NASW Code of Ethics (2021), Ethical Principles: Competence; NASW Code of Ethics (2021), 1.04(a); NASW Code of Ethics (2021), 1.16(a); AERA, APA, & NCME (2014). Standards for Educational and Psychological Testing. American Educational Research Association, Test Administration, Scoring, Reporting, and Interpretation: standardized administration

Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.5 Core values: competence, social justice, integrity, dignity and worth · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

95. ETH-1037 (rev 2) · Best answer: A

At a community health center, a 34-year-old client says their partner shoved them into a wall during an argument last week. The client, who lives alone and works full time, asks the social worker not to involve the police. What should the social worker do?

A. Work out a plan with the client for staying safe
B. Report the assault to adult protective services
C. Urge the client to seek a protective order this week

Why each option is right or wrong

A **Correct.** Violence between competent adults is generally not a mandated report to protective services, and competent clients decide how to respond to it (NASW Code 1.02). HIPAA allows a report about an adult victim only when law requires it, the client agrees, or narrow exceptions apply (45 CFR 164.512(c)). The right response is a safety plan built with the client. A beats B because adult protective services generally serve older or dependent adults, not a working adult who lives alone.

B **Incorrect** (trap: Misreads law or ethics). Adult protective services generally serve older adults and adults with disabilities who depend on others. This client lives alone and works, so a report would breach confidentiality without a legal basis. A report could apply to an older or dependent victim.

C **Incorrect** (trap: Too directive). A protective order is one option to describe, and some clients choose it. Urging it before planning with the client puts the worker's solution ahead of the client's own judgment about what will keep them safe (NASW Code 1.02), and filing can raise danger if the partner retaliates.

Takeaway: Partner violence between capable adults generally falls outside mandated reporting; follow the client's wishes and plan for safety.

Sources: NASW Code of Ethics (2021), 1.02; HIPAA Privacy Rule, 45 CFR 164.512(c)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** none · **Cognitive** application · **Difficulty** 2 of 3

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96. ASM-1542 (rev 1) · Best answer: C

A 2-year-old has been in a children's hospital for 3 weeks, and the parents can visit only on weekends. For the first few days, the child screamed and searched the doorway for the parents. Now the child lies quietly, plays little, eats poorly and rarely cries. Nurses describe the child as 'settled.' Which phase of separation response is the child **MOST** likely in?

A. Protest
B. Detachment
C. Despair
D. Rapprochement

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Protest is the first phase: crying, screaming and searching for the absent parent. The child showed this in the first days but has since become quiet and withdrawn, a sign of the next phase.

B **Incorrect** (trap: Wrong model or concept). Detachment is the later phase, when the child seems to recover, engages with staff and may treat the parents with indifference when they return. This child is withdrawn and apathetic rather than outwardly sociable.

C **Correct.** Robertson and Bowlby described young children's responses to prolonged separation from caregivers, as in hospitals, in three phases: protest, despair and detachment (Bowlby, 1973). In despair, the child becomes withdrawn, inactive and apathetic, a quiet state that staff often mistake for settling in. This child has moved from screaming and searching to lying quietly and eating poorly. It beats B because detachment comes later, when the child grows more sociable with staff and may greet returning parents with indifference.

D **Incorrect** (trap: Wrong model or concept). Rapprochement is a Mahler subphase of typical development, around 16 to 24 months, in which toddlers alternate between seeking and pushing away the caregiver. It describes normal separation-individuation, not a reaction to prolonged separation.

Takeaway: Young children's responses to prolonged separation run from protest to despair to detachment; quiet despair is easily mistaken for adjustment.

Sources: Bowlby, J. (1973). Attachment and Loss, Vol. 2: Separation: Anxiety and Anger. Basic Books, Ch. 1: protest, despair and detachment

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.13 Effects of out-of-home placement (hospital, foster, residential, justice) · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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97. ASM-1029 (rev 1) · Best answer: B

At a community mental health intake, a client completes the DSM-5-TR Level 1 Cross-Cutting Symptom Measure. Most domains are rated low, but the client rates the items about sleeping less than usual and starting many more projects than usual as moderate. The social worker reviews these answers with the client. What should the social worker do **NEXT**?

A. Record a provisional bipolar disorder diagnosis

B. Give the matching Level 2 measure for follow-up

C. Repeat the Level 1 measure at the next visit

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). One screen with moderate ratings cannot establish a mood episode. Diagnosing now skips the focused assessment of duration, severity and impairment that any bipolar diagnosis requires.

B **Correct.** In DSM-5-TR, the Level 1 Cross-Cutting Symptom Measure is a broad screen. A domain rated at or above its threshold is followed by a Level 2 measure that examines that area in more depth, along with clinical follow-up. Moderate ratings on sleeping less and starting many projects call for that closer look at possible manic symptoms. It beats C because repeating the broad screen later delays the focused assessment the positive domain already calls for. Screens point to further assessment; they do not diagnose (DSM-5-TR).

C **Incorrect** (trap: Good but not best). Repeating the broad screen later wastes the signal already present. A domain at or above threshold calls for focused follow-up now, not another general screen.

Takeaway: A positive domain on the Level 1 cross-cutting screen leads to a Level 2 measure and clinical follow-up, not straight to a diagnosis.

Sources: DSM-5-TR (APA, 2022), Section III, Assessment Measures: Level 1 and Level 2 Cross-Cutting Symptom Measures
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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98. INT-1679 (rev 1) · Best answer: D

A youth shelter invites line staff to help design an evaluation of its case management. While drafting measures, months before data collection begins, staff start defining what a 'stable exit' means, discussing outcomes in team meetings and changing how they set goals with youth. Which type of evaluation use does this **MOST** likely show?

A. Instrumental use
B. Symbolic use
C. Conceptual use
D. Process use

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Instrumental use is acting directly on findings, such as changing a program because results showed a problem. Data collection has not started, so the changes cannot be a response to findings.

B **Incorrect** (trap: Wrong model or concept). Symbolic use is invoking an evaluation to legitimize decisions already made or to signal accountability. Staff here are changing their own practice in substance, not using the evaluation for appearances.

C **Incorrect** (trap: Wrong model or concept). Conceptual use is a change in how people understand a program that comes from evaluation findings, even without a specific decision. It fits the shift in thinking but not the timing: the shift came from designing measures, before any data.

D **Correct.** Process use refers to changes in thinking and behavior that come from taking part in an evaluation, as distinct from use of its findings (Patton, 2008). Staff are clarifying outcomes and changing goal setting while drafting measures, months before any data exist. It beats C because conceptual use is a shift in understanding produced by evaluation results; here the change comes from participation itself. Patton treats process use as a benefit evaluators can plan for by involving staff.

Takeaway: Changes that come from taking part in an evaluation, before findings exist, are process use; acting on findings is instrumental use.

Sources: Patton, M. Q. (2008). Utilization-Focused Evaluation (4th ed.). SAGE, Ch. 5: Intended Process Uses: Impacts of Evaluative Thinking and Experiences
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.3 Program evaluation and its effect on services · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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99. INT-0003 (rev 1) · Best answer: A

A client who injects opioids says they are not ready to stop using but are worried after a friend overdosed. The client wants to keep meeting with the social worker. What is the **MOST** appropriate intervention?

A. Provide naloxone and teach overdose prevention steps
B. Make continued services contingent on abstinence
C. Refer the client to a residential treatment program this week

Why each option is right or wrong

A **Correct.** Harm reduction meets clients where they are and reduces the risks of ongoing use without requiring abstinence first (SAMHSA, 2023). The client's worry after a friend's overdose is a real opening. Naloxone and overdose-prevention teaching, such as not using alone, protect the client's life now and keep the relationship going. It beats C because a client who says they are not ready to stop is unlikely to accept residential treatment, and pushing it risks losing the engagement the client wants.

B **Incorrect** (trap: Overrides the client's choice). Requiring abstinence for continued services pushes out the client most at risk. Withdrawing support from someone not ready to stop increases danger and conflicts with meeting clients where they are.

C **Incorrect** (trap: Hands off too soon). Residential treatment may become appropriate if the client's goals change. Referring now ignores the client's stated readiness and may end engagement with a client at high overdose risk.

Takeaway: With a client not ready to stop using, harm reduction such as naloxone and safer-use teaching protects life and keeps engagement.

Sources: SAMHSA (2023) Harm Reduction Framework, pillars and principles; SAMHSA (2018) Opioid Overdose Prevention Toolkit (SMA18-4742), naloxone and overdose prevention
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.12 Harm-reduction approaches · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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100. ETH-1512 (rev 1) · Best answer: B

A social worker in a state prison provides counseling to an incarcerated client. Near the end of a session, the client hands the social worker a sealed letter and asks the worker to mail it to the client's adult child from outside the facility. The client says the letter holds family news. What is the **MOST** appropriate response?

A. Mail the letter as a one-time favor to the client
B. Explain how to send it through the facility's mail
C. Read the letter first, then mail it if it is harmless
D. Report the request to facility security as misconduct

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). A small favor can feel like kindness, but carrying mail outside official channels gives the client a special arrangement, may break facility rules and can be used to press for more favors later.

B **Correct.** The power imbalance in a prison makes favors outside official channels a boundary risk: they create a special relationship, can become leverage and may break facility rules (NASW Code 1.06(c)). Explaining kindly how to send the letter through

the facility's mail meets the real need, contact with family, while keeping the professional role clear. It beats D because the request may be innocent; declining and redirecting handles it without punishing the client, and the worker can follow facility reporting rules if pressure continues (Reamer, 2020).

C **Incorrect** (trap: Misreads law or ethics). Screening the letter does not fix the problem. The worker would still be bypassing the facility's mail process, and reading private mail adds a privacy intrusion that falls outside the counseling role.

D **Incorrect** (trap: Skips assessment). Reporting may be needed if the client keeps pressing or the request suggests contraband, but treating a first, possibly innocent request as misconduct jumps ahead of the facts and can damage trust.

Takeaway: In settings with large power imbalances, decline personal favors outside official channels and redirect clients to the proper process.

Sources: NASW Code of Ethics (2021), 1.06(c); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, altruistic gestures
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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101. INT-1069 (rev 1) · Best answer: A

In the working stage of a process group at a veterans' center, one member describes feeling like a stranger at home. Another member turns to the social worker and says, 'I think what they mean is they feel invisible to their family.' What is the **MOST** appropriate response?

A. Ask the member to say that directly to the other member
B. Agree and add the social worker's own reading of it
C. Thank the member and move on to the next person
D. Restate the speaker's feelings in the leader's own words

Why each option is right or wrong

A **Correct.** In the working stage the leader becomes less central and fosters direct member-to-member exchange, the main channel of here-and-now work and cohesion (Yalom & Leszcz, 2020). Asking the member to speak to the other member directly turns a comment about someone into a conversation with them. It beats D because reflecting feelings is a sound skill, but done by the leader here it pulls the exchange back through the leader at a stage when members should be relating to each other.

B **Incorrect** (trap: Good but not best). Adding the leader's own reading makes the leader the authority on the member's experience and keeps communication flowing through the leader. It also risks putting words in the speaker's mouth.

C **Incorrect** (trap: Good but not best). Moving on drops a moment of connection between two members. The working stage is when such exchanges carry the most therapeutic value, so the leader builds on them rather than passing over them.

D **Incorrect** (trap: Misuses the technique). Restating feelings is a core skill that fits early sessions, when the leader carries more of the work. In the working stage it routes the exchange through the leader instead of between members.

Takeaway: In the working stage, the leader steps back and redirects comments so members speak to each other directly.

Sources: Yalom, I. D., & Leszcz, M. (2020). The Theory and Practice of Group Psychotherapy (6th ed.). Basic Books, the here-and-now; the therapist's role in the working group
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.23 Group work: managing process and building cohesion · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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102. ASM-1667 (rev 2) · Best answer: A

A parent whose two children entered foster care meets with a child welfare social worker to build a case plan. The court has ordered substance use treatment and parenting classes. The parent says the top priority is finding a job, because without income they will lose their apartment, and the court also requires stable housing for reunification. What is the **MOST** appropriate way to proceed?

A. Include a job goal in the plan alongside the court-ordered services
B. Focus the plan on the court orders and add employment later
C. Make employment the first goal and begin services once the parent works

Why each option is right or wrong

A **Correct.** Social workers help clients identify and clarify their own goals (NASW Code 1.02), and with involuntary clients, plans pair nonnegotiable requirements with goals the client chooses. This parent's job goal also serves a court requirement, stable housing, so it belongs in the plan. Placing it beside the ordered treatment and classes, and helping the parent schedule all three, respects the parent's priority without dropping required conditions. It beats B because putting off employment ignores the parent's own reasoning and the housing requirement that depends on income (Hepworth et al.).

B **Incorrect** (trap: Good but not best). Court-ordered services must be in the plan, but leaving out the parent's main goal weakens engagement and neglects the housing requirement that rests on income. Required and client-defined goals can work together.

C **Incorrect** (trap: Misreads law or ethics). Honoring the parent's priority matters, but court-ordered services are not optional, and delaying them could stall reunification. The parent's goal should join the plan, not postpone required steps.

Takeaway: In mandated planning, include the client's own priority goals alongside required services rather than choosing between them.

Sources: NASW Code of Ethics (2021), 1.02; Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Developing goals and contracts with involuntary clients
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.3 Engaging clients in problem identification and intervention planning · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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103. ASM-1061 (rev 1) · Best answer: C

What does the mental status exam contribute to a clinical assessment?

A. A diagnosis that meets DSM-5-TR criteria
B. A chronological history of the presenting problem
C. A snapshot of present thought, mood and behavior
D. A norm-referenced score of symptom severity

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Diagnosis integrates the mental status exam with history, collateral information and course over time. Mental status findings alone, such as flat affect, fit several diagnoses.

B **Incorrect** (trap: Wrong model or concept). A chronological account comes from the history of the present illness and the psychosocial history. The mental status exam records present-moment observations rather than past events.

C **Correct.** The mental status exam (MSE) is a structured, cross-sectional description of how the client appears, behaves, thinks, feels and reasons at the time of the interview. It covers domains such as appearance, mood and affect, thought process and content, perception, cognition, insight and judgment. It beats A because the MSE supplies observations that, combined with history and other data, inform a diagnosis; it does not establish one by itself (Kaplan & Sadock).

D **Incorrect** (trap: Wrong model or concept). Norm-referenced severity scores come from standardized instruments such as the PHQ-9. The mental status exam is a clinician's structured observation, not a scored and normed test.

Takeaway: The MSE is a cross-sectional snapshot of present mental functioning that informs, but does not by itself establish, a diagnosis.

Sources: Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Psychiatric interview: mental status examination
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.10 Mental status exam: components and purpose · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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104. ETH-1188 (rev 1) · Best answer: B

At a training, a social worker at an opioid treatment program learns that the federal confidentiality rule for substance use disorder records was revised in 2024. The program's consent forms date from 2019, and the social worker will ask new clients to sign them this week. The program has a compliance officer who oversees confidentiality policies. What should the social worker do **FIRST**?

A. Continue using the forms until the program issues new ones
B. Ask the compliance officer whether the forms meet the current rule
C. Revise the consent form using a summary of the new rule
D. Pause new consents until the compliance officer approves new forms

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Waiting for the program to act could mean new clients sign forms that do not match the rule they are told protects them. Knowing about a possible gap creates a duty to check, not to wait.

B **Correct.** Social workers should strive to remain proficient (4.01(b)) and seek consultation from colleagues with relevant expertise (2.05(b)). The 2024 revision of 42 CFR Part 2 changed consent requirements (2.31), and the social worker does not know whether the 2019 forms still comply. The compliance officer oversees these policies, so checking there is the first step before more clients sign. It beats A because waiting passively leaves a known question unanswered while clients keep signing forms that may be outdated.

C **Incorrect** (trap: Outside the social work role). Rewriting a legal consent form from a summary risks errors and bypasses the person responsible for compliance. Helping with revisions is fine once the compliance officer confirms what must change.

D **Incorrect** (trap: Skips assessment). Stopping consents assumes the forms are invalid before anyone has checked, and it can delay care coordination for new clients. If the compliance officer finds a problem, a short pause might then be warranted.

Takeaway: When a rule governing your practice changes, verify current requirements with the responsible expert before relying on old procedures.

Sources: NASW Code of Ethics (2021), 4.01(b); NASW Code of Ethics (2021), 2.05(b); 42 CFR Part 2, 2.31
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.8 Keeping knowledge current: training, licensing CE, supervision, consultation · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 3 of 3

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105. ETH-1506 (rev 1) · Best answer: C

An employee assistance program social worker reviews the next day's schedule and sees that a newly assigned client is someone the social worker had a brief sexual relationship with 10 years ago. The two have had no contact since. The program has 4 other staff social workers. What should the social worker do?

A. Begin services, since the relationship ended long ago
B. Meet once to learn whether the history concerns the client
C. Ask that the case be reassigned to a colleague
D. Disclose the history and let the client decide

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Time does not change the rule. The Code bars clinical services to a former sexual partner with no time limit, because the prior intimacy can distort judgment and the client's trust no matter how long ago it ended.

B **Incorrect** (trap: Boundary slip or collusion). Even a single session would be clinical service to a former sexual partner. Checking how the client feels about the history misses the point when the past relationship itself is the barrier to serving this client.

C **Correct.** Social workers do not provide clinical services to people with whom they have had a prior sexual relationship, because the history makes clear boundaries hard to keep and risks harm (NASW Code 1.09(d)). With 4 colleagues available, reassigning the case before any contact protects the client and spares them an awkward first meeting. It beats D because the client's agreement does not cure the conflict; the Code's bar applies regardless of time passed or the client's preference (Reamer, 2020).

D **Incorrect** (trap: Misreads law or ethics). Openness is usually a good instinct, but here it shifts the decision onto the client and may pressure them. The Code's bar does not

depend on the client's consent, and the disclosure itself could be uncomfortable for the client.

Takeaway: Do not provide clinical services to a former sexual partner; arrange for a colleague to serve the client before services begin.

Sources: NASW Code of Ethics (2021), 1.09(d); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, intimate relationships: former partners as clients
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** none · **Cognitive** application · **Difficulty** 2 of 3

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106. ASM-1131 (rev 2) · Best answer: B

A client in private practice was laid off 2 months ago and since then has had poor sleep, tearfulness and trouble concentrating. The client is suing the former employer and asks the social worker to record PTSD because 'that will help the case.' The social worker's assessment supports adjustment disorder with mixed anxiety and depressed mood. What is the **MOST** appropriate response?

A. Record PTSD as a provisional diagnosis while gathering more history
B. Explain why the findings point to adjustment disorder instead
C. Record adjustment disorder and direct legal questions to the client's attorney

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Calling PTSD provisional still puts an unsupported diagnosis in the record to serve a lawsuit. The provisional label applies when criteria are expected to be met, not when the required traumatic exposure is missing.

B **Correct.** Diagnoses must be accurate, and clients deserve a clear account of what their records say. PTSD requires exposure to actual or threatened death, serious injury or sexual violence; a layoff, however painful, does not meet that requirement, and the symptoms fit adjustment disorder. Explaining this openly, including why PTSD does not apply, respects the client while declining a false record. It beats C because handing the question to the attorney treats a clinical question as a legal one and leaves the client to be surprised later, possibly in court (DSM-5-TR; NASW Code 3.04(a), 4.04).

C **Incorrect** (trap: Hands off too soon). The diagnosis is correct, and the social worker should not give legal advice. But the client asked what the record will say, which is a clinical question. Sending it to the attorney sidesteps explaining the findings and leaves the client to learn the diagnosis later.

Takeaway: Record only diagnoses the assessment supports, and explain the findings openly when a client requests a different label.

Sources: DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Posttraumatic Stress Disorder; NASW Code of Ethics (2021), 3.04(a); NASW Code of Ethics (2021), 4.04
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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107. ETH-1177 (rev 2) · Best answer: C

A hospital patient with cancer and full decision-making capacity is visited by an adult child, who does most of the talking. When the social worker mentions the upcoming scan, the patient says, 'Just give the results to my child. I don't want to hear them.' The oncologist's usual practice is to give results directly to patients. What should the social worker do **FIRST**?

A. Encourage the patient to hear the results from the oncologist
B. Request an ethics consultation on keeping results from a patient
C. Meet privately with the patient to confirm the wish
D. Help the team record the wish and give results to the adult child

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Hearing results directly is the usual practice, but a capable patient may decline information. Urging the patient to receive results they have refused puts the team's view of truthfulness ahead of the patient's autonomy.

B **Incorrect** (trap: Hands off too soon). Ethics consultation helps when duties remain in conflict after analysis. Here a short private conversation can settle whether the wish is the patient's own, so a consultation is premature.

C **Correct.** Respect for self-determination includes a capable patient's choice not to receive information and to involve family in decisions (NASW Code 1.02), but the social worker must first be sure the wish is the patient's own. The request came with the adult child present and doing most of the talking, so a private conversation about what the patient wants to know, and who should hear it, comes first. It beats D because recording the wish and routing results is the right next step once the private talk confirms the preference.

D **Incorrect** (trap: Right action, wrong time). Documenting the wish and sharing results with the adult child, which HIPAA permits when the patient agrees (45 CFR 164.510(b)), is right once the preference is confirmed as the patient's own. Acting on a request made in front of a relative who dominates the conversation skips that check.

Takeaway: When a capable patient waives information in front of family, confirm the wish privately; then the patient may direct results to family.

Sources: NASW Code of Ethics (2021), 1.02; HIPAA Privacy Rule, 45 CFR 164.510(b)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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108. INT-0112 (rev 1) · Best answer: A

A community clinic is redesigning its intake. Currently, staff seat each new client, then ask a fixed list of trauma history questions in the same order for everyone. Which change is **MOST** consistent with a trauma-informed approach?

A. Offer clients choices about pacing and what to share
B. Collect a detailed trauma history at the first visit
C. Refer clients who report trauma to a trauma specialist

Why each option is right or wrong

A **Correct.** A trauma-informed approach applies to everyone and builds in safety, collaboration, and empowerment, voice and choice, while resisting re-traumatization (SAMHSA, 2014). A fixed, staff-controlled trauma interview removes control from clients, which can echo the powerlessness of trauma. Offering choices about pacing and what to share returns some control and builds trust. It beats B because requiring a detailed trauma history at the first visit, before trust exists, risks re-traumatization; screening can be brief, with fuller assessment later when the client is ready.

B **Incorrect** (trap: Right action, wrong time). Detailed trauma questioning at the first contact, before any trust or choice, can overwhelm clients and risk re-traumatization. A brief screen now and fuller assessment later fits trauma-informed practice.

C **Incorrect** (trap: Hands off too soon). Trauma-informed care is an agency-wide approach, not a specialty service. Sending clients elsewhere leaves in place the intake practices that may harm them, though trauma-specific treatment can be offered when indicated.

Takeaway: Trauma-informed care gives clients choice and control in routine procedures and applies to everyone, whether or not trauma is disclosed.

Sources: SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), six key principles: empowerment, voice and choice
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.1 Trauma-informed care across trauma types (complex, vicarious) · **Qualifier** MOST · **Cognitive** application · **Difficulty** 1 of 3

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109. ASM-1120 (rev 1) · Best answer: C

At a pediatric clinic, a parent reports that their 11-year-old, whose weight is in the healthy range, recently began calling themselves 'fat' and comparing their stomach to a sibling's. The parent recently put the family on a strict diet and weighs the child each morning 'to keep everyone on track.' What is the social worker's **MOST** appropriate response?

A. Teach the child to challenge negative thoughts about their body
B. Have the parent arrange an eating disorder evaluation
C. Help the parent swap dieting for regular shared meals

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Cognitive skills can help a child with body dissatisfaction. Used alone, they leave the dieting and weigh-ins in place, so the child is asked to challenge messages the household repeats every morning.

B **Incorrect** (trap: Skips assessment). An eating disorder evaluation fits when there are signs such as restriction, weight loss, purging or compulsive exercise. Body talk that follows a family diet calls first for changing the home environment while watching for those signs.

C **Correct.** Family dieting and weight talk are linked to body dissatisfaction and disordered eating in youth, and pediatric guidance advises families to avoid dieting and weight talk and to focus on healthy habits and family meals (Golden et al., 2016). The child's new 'fat' talk began after the strict diet and morning weigh-ins, and the child's weight is healthy. Helping the parent shift to health-focused habits targets the main

influence. It beats A because coping skills for the child leave in place the daily routine that keeps feeding the dissatisfaction.

Takeaway: When a child's body dissatisfaction follows family dieting or weight talk, guide parents toward health-focused habits rather than weight.

Sources: Golden, N. H., Schneider, M., Wood, C., & AAP Committee on Nutrition, Committee on Adolescence, Section on Obesity (2016). Preventing obesity and eating disorders in adolescents. Pediatrics, 138(3), e20161649, Recommendations for pediatricians

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.10 Influences on self-image and body image · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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110. ASM-1562 (rev 3) · Best answer: C

PRETEST — unscored on this form; do not count it

During a biopsychosocial assessment in a hospital dialysis unit, a patient mentions that prayer has helped them cope, but adds that they have not attended services since starting treatment 3 months ago. The intake form already lists the patient's religious affiliation. What should the social worker do **NEXT**?

A. Refer the patient to the hospital chaplain for support

B. Move on to the social history section of the assessment

C. Ask what faith means to the patient and how it affects care

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). Chaplain involvement may well help and fits the care-planning part of a spiritual history. Referring before asking what the patient wants hands off a domain the social worker should first assess.

B **Incorrect** (trap: Good but not best). Recording affiliation alone misses the spiritual domain. The patient has signaled both a coping strength and a lost community connection, and both affect support during dialysis.

C **Correct.** A biopsychosocial-spiritual assessment covers beliefs, their importance, community and how they should be addressed in care, the areas of the FICA spiritual history (Puchalski & Romer, 2000). This patient named prayer as a coping resource and has lost contact with a faith community since treatment began, so learning what faith means to the patient and how it shapes care is the next step. It beats A because a chaplain referral may be useful, but it should follow the patient's own account of needs and wishes.

Takeaway: A biopsychosocial assessment includes a spiritual history: beliefs, their importance, community and how the client wants them addressed in care.

Sources: Puchalski, C., & Romer, A. L. (2000). Taking a spiritual history allows clinicians to understand patients more fully. Journal of Palliative Medicine, 3(1), 129-137, FICA spiritual history

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.7 Biopsychosocial assessment framework and its components · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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111. INT-1187 (rev 1) · Best answer: A

A client in early recovery from alcohol use disorder starts taking a new route home from work 'to beat traffic.' The route passes a liquor store the client used to visit daily. The next week, the client stops there 'just to buy a soda' and drinks again that night. Which relapse prevention concept **MOST** likely describes the route change?

A. An apparently irrelevant decision
B. The abstinence violation effect
C. Positive outcome expectancies
D. A lapse

Why each option is right or wrong

A **Correct.** In Marlatt's relapse prevention model, apparently irrelevant decisions are small, harmless-looking choices that move a person step by step toward a high-risk situation while hiding the link to using (Marlatt & Gordon, 1985). Choosing a route past the old liquor store 'to beat traffic' fits that pattern and set up the later drink. It beats D because a lapse is the first return to use itself, the drinking that night, not the earlier choice that led there.

B **Incorrect** (trap: Wrong model or concept). The abstinence violation effect is the guilt and 'I've blown it' thinking that can follow a lapse and turn it into a full relapse. It describes a reaction after drinking, not the earlier route choice.

C **Incorrect** (trap: Wrong model or concept). Positive outcome expectancies are beliefs that drinking will bring good effects, such as relaxation or confidence. The vignette shows a chain of small choices, not a stated belief about what alcohol will do.

D **Incorrect** (trap: Wrong model or concept). A lapse is a single slip back to use, here the drinking that night. The question asks about the route change, the earlier choice that set up the high-risk situation.

Takeaway: Apparently irrelevant decisions are small choices that quietly lead toward high-risk situations; relapse prevention teaches clients to spot them early.

Sources: Marlatt, G. A., & Gordon, J. R. (Eds.). (1985). Relapse Prevention: Maintenance Strategies in the Treatment of Addictive Behaviors. Guilford Press, Apparently irrelevant decisions; lapse versus relapse
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.17 Intervening with addiction · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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112. ASM-1529 (rev 2) · Best answer: A

A school social worker meets with a 14-year-old who often falls asleep in class and has fallen behind on assignments. The student explains that a parent with multiple sclerosis needs help getting out of bed, bathing and eating, and the other parent works nights. The student says, 'It's my job. I don't mind.' Which action is **BEST**?

A. Arrange in-home personal care with the parents
B. Arrange a school plan with the parents for extended deadlines
C. Refer the student to counseling for caregiver stress

Why each option is right or wrong

A **Correct.** Young caregivers are common, as many as 1.4 million U.S. children ages 8 to 18 in one national survey, and are often overlooked (National Alliance for Caregiving & United Hospital Fund, 2005). This student's sleep loss and missed work come from a care load the family cannot meet alone, so the most effective step is helping the parents arrange in-home care for the parent with MS. It beats B because school accommodations ease the symptoms but leave a 14-year-old providing bathing and personal care that adult services should cover.

B **Incorrect** (trap: Good but not best). Extended deadlines and similar school supports may help and can be part of the plan. Alone, they adjust school around an excessive care load instead of reducing it, so the student's sleep and schoolwork keep suffering.

C **Incorrect** (trap: Hands off too soon). Counseling may help the student cope, but it treats the strain as the student's problem to manage. The source is an unmet care need at home that in-home services can address; counseling can be added if stress persists.

Takeaway: When a child carries heavy caregiving duties, reduce the load by arranging adult care services for the ill family member, not only supporting the child.

Sources: National Alliance for Caregiving & United Hospital Fund (2005). Young Caregivers in the U.S.: Findings from a National Survey, prevalence of caregivers ages 8 to 18
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.14 How caregiving affects families · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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113. ETH-1588 (rev 1) · Best answer: C

A white social worker goes with a Black client to a benefits office to appeal a denial. The caseworker there speaks to the social worker alone and brushes off the client's answers. The client prepared the appeal and wants to present it. What is the **BEST** action by the social worker?

A. Present the appeal so the caseworker will listen and decide
B. File a complaint about the caseworker's conduct after the visit
C. Turn each question back to the client and stay present

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Presenting the case might speed a decision, but it overrides the client's wish to speak and confirms the caseworker's pattern of treating the white professional as the real authority.

B **Incorrect** (trap: Right action, wrong time). A complaint may be appropriate later if the client wants one. It does not change what happens in the room, where the client is being sidelined now.

C **Correct.** Racial and professional privilege can lead institutions to defer to a white worker over a Black client. Empowerment-focused advocacy uses that access to make room for clients to speak for themselves rather than taking over (NASW Cultural Competence Standards, Standard 6; NASW Code 1.02). The client prepared the appeal and wants to present it, so turning questions back to the client while staying present honors that choice and counters the caseworker's deference. It beats A because speaking for the client may win the appeal but reinforces the dynamic that sidelined the client.

Takeaway: Use racial and professional privilege to amplify the client's voice, not to replace it.

Sources: NASW Standards and Indicators for Cultural Competence in Social Work Practice (2015), Standard 6: Empowerment and Advocacy; NASW Code of Ethics (2021), 1.02
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.5 Power differentials, racial privilege, internalized inferiority/superiority · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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114. ASM-1520 (rev 1) · Best answer: C

Staff at a senior center ask the social worker about a 76-year-old member who has stopped joining group activities. In the busy dining room, the member often answers questions that were not asked and asks others to repeat themselves. A relative wonders if dementia is starting. The social worker meets with the member in a quiet office, where they talk easily about recent events. What should the social worker do **NEXT**?

A. Move the member to smaller, quieter activity groups
B. Refer the member for a neurocognitive evaluation
C. Recommend that the member get a hearing evaluation

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Quieter groups may help the member participate and could become part of the plan. Doing this before checking hearing manages the symptom without identifying or treating the likely cause of the withdrawal.

B **Incorrect** (trap: Skips assessment). A neurocognitive evaluation is appropriate when memory or thinking problems appear across settings. Here the member does well in a quiet room, so referring for possible dementia first overlooks a likely sensory explanation.

C **Correct.** Hearing loss is common in older adults, and people who cannot hear well are often mistaken for confused, uncooperative or withdrawn (National Institute on Aging). This member struggles and answers off-target in a noisy room but converses easily and recalls recent events in a quiet office, a pattern that points to hearing rather than cognition. It beats B because jumping to a neurocognitive evaluation skips the simpler, likely explanation; if problems persist after hearing is addressed, cognitive evaluation becomes appropriate.

Takeaway: Rule out sensory loss, especially hearing, before attributing an older adult's withdrawal or off-target answers to cognitive decline.

Sources: National Institute on Aging (NIH). Hearing Loss: A Common Problem for Older Adults, signs and effects of hearing loss
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.17 Effects of physical and mental ability across the lifespan · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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115. ASM-1067 (rev 2) · Best answer: A

A client who has taken sertraline for a year begins tramadol after dental surgery. The next day, at a scheduled clinic visit, the client is agitated and sweating heavily, with diarrhea, a racing heart and jerking muscles in both legs. The client thinks it is a stomach bug. What should the social worker do **FIRST**?

A. Have the client seen in an emergency department now
B. Call the prescriber's office to report the new medicine
C. Advise the client to stop the tramadol and rest at home

Why each option is right or wrong

A **Correct.** Serotonin syndrome can follow when serotonergic drugs are combined, as when tramadol is added to an SSRI. It typically starts within hours to a day and brings agitation, sweating, diarrhea, rapid heart rate and muscle jerking (clonus), especially in the legs. It can become life threatening, so it needs emergency care now. It beats B because a call to the prescriber's office delays treatment of a possible medical emergency; the prescriber can be informed once the client is safe (Boyer & Shannon, 2005).

B **Incorrect** (trap: Ignores risk). The prescriber should learn of the new medication, and calling the office is right for routine side effects. With agitation, heavy sweating, a racing heart and muscle jerking a day after adding tramadol, waiting for a callback is unsafe.

C **Incorrect** (trap: Outside the social work role). Stopping the added drug is likely part of treatment, which makes this tempting. But telling a client to stop a prescribed medicine is a medical decision outside the social work role, and resting at home leaves agitation, a racing heart and muscle jerking unmonitored when the syndrome can worsen quickly.

Takeaway: Agitation, sweating, diarrhea and muscle jerking soon after adding a serotonergic drug suggest serotonin syndrome and need emergency care.

Sources: Boyer, E. W., & Shannon, M. (2005). The serotonin syndrome. New England Journal of Medicine, 352(11), 1112-1120, whole article

Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.5 Prescription, OTC, alternative medications: uses and side effects · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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116. ETH-1661 (rev 1) · Best answer: B

PRETEST — unscored on this form; do not count it

A community mental health agency hires a former client as a peer support specialist. The new hire finished treatment with a staff social worker 2 years ago and did well. The program director asks that social worker to be the new hire's direct supervisor, citing their good rapport. What is the **MOST** appropriate response from the social worker?

A. Accept, setting clear rules that keep supervision work-focused
B. Ask that another staff member supervise the new hire
C. Object to the agency hiring its former clients as staff

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). Clear rules are the right tool when an overlapping relationship cannot be avoided, as in a small town. Here another supervisor is available, and evaluative power over a former client carries real risk, so avoiding the overlap is better.

B **Correct.** A dual relationship with a former client should be avoided when it carries a risk of exploitation or harm, and that applies to relationships that follow one another (NASW Code 1.06(c)). As supervisor, the worker would judge the job performance of someone who once shared private struggles; the employee may find it hard to disagree, and the worker's view may be colored by the treatment history (Reamer, 2020). It beats A because boundary rules manage an overlap that cannot be avoided, and this one is easy to avoid.

C **Incorrect** (trap: Good but not best). Hiring people with lived experience is a recognized strength of recovery-oriented services. The ethical problem lies in the supervisory pairing, not in employing a former client, so opposing the hire overcorrects.

Takeaway: Avoid taking a role with power over a former client when someone else can readily fill it.

Sources: NASW Code of Ethics (2021), 1.06(c); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, dual relationships with former clients
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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117. INT-1592 (rev 2) · Best answer: A

An agency director announces that an outside firm will evaluate the family support program this spring. Several case managers tell the program's coordinator, a social worker, that they fear the results will be used to cut positions, and some say they may not cooperate. Planning has not begun. What should the coordinator do **FIRST**?

A. Involve staff in shaping the evaluation questions and uses
B. Assure staff that the evaluation results will stay confidential
C. Start data collection and address concerns as they come up

Why each option is right or wrong

A **Correct.** Utilization-focused evaluation holds that findings are used when the people who will use them help frame the questions and plan how results will be used from the start (Patton, 2008). Engaging case managers early lowers fear, improves cooperation and data quality, and makes it more likely the evaluation improves services. This beats C because collecting data over staff objections invites poor cooperation and distorted data, and concerns are easier to settle before methods are fixed.

B **Incorrect** (trap: Good but not best). The coordinator does not control how an outside firm or the director will use the results, so this promise may not hold. It also leaves the real worry, how findings will be used, unaddressed.

C **Incorrect** (trap: Right action, wrong time). Data collection is the right step later. Starting it before staff concerns are addressed invites poor cooperation and skewed data, which weakens the evaluation the program needs.

Takeaway: Engage the people who deliver and use a program in planning its evaluation; early involvement improves cooperation and use of findings.

Sources: Patton, M. Q. (2008). Utilization-Focused Evaluation (4th ed.). SAGE, Ch. 3: Fostering Intended Use by Intended Users: The Personal Factor

Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.3 Program evaluation and its effect on services · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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118. ETH-1039 (rev 2) · Best answer: D

What threshold of belief triggers a mandated reporter's legal duty to report possible child abuse or neglect?

A. Probable cause
B. Clear and convincing evidence
C. Preponderance of the evidence
D. Reasonable suspicion

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Probable cause is the standard police and courts use for arrests and search warrants. Requiring it before reporting would push social workers into investigating, which reporting laws leave to child protective services.

B **Incorrect** (trap: Wrong model or concept). Clear and convincing evidence is a high civil standard used in decisions such as terminating parental rights. A reporter needs far less, and waiting for that level of certainty would delay protection.

C **Incorrect** (trap: Wrong model or concept). Preponderance of the evidence, meaning more likely than not, is the usual civil trial standard and is used in many states to substantiate a finding after investigation. A report requires only reasonable suspicion beforehand.

D **Correct.** Federal law requires states to mandate reports of known and suspected abuse (CAPTA, 42 U.S.C. 5106a(b)(2)(B)(i)), and state laws set the trigger as reasonable suspicion, or reason to believe, not proof. Deciding whether abuse occurred is the job of child protective services and the courts. D beats A because probable cause is a law enforcement standard for arrests and searches, a higher bar than the suspicion that triggers a report.

Takeaway: Mandated reporters report on reasonable suspicion; proof and investigation belong to child protective services and the courts.

Sources: Child Abuse Prevention and Treatment Act, 42 U.S.C. 5106a(b)(2)(B)(i)

Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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119. ETH-1192 (rev 1) · Best answer: C

A hospital discharge planner whose caseload has doubled has started calling patients by bed number and diagnosis in team meetings. When a family calls in tears about a delayed discharge, the planner answers curtly and feels little concern. Which component of burnout is **MOST** likely present?

A. Emotional exhaustion
B. Reduced personal accomplishment
C. Depersonalization

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Emotional exhaustion means feeling drained and depleted by the work. A doubled caseload makes it likely, but the behaviors described, treating patients as cases and responding without concern, point to depersonalization.

B **Incorrect** (trap: Wrong model or concept). Reduced personal accomplishment is a sense of ineffectiveness and lack of achievement at work. The stem describes how the worker treats patients and families, not how the worker judges their own effectiveness.

C **Correct.** Maslach's model describes burnout through three components: emotional exhaustion, depersonalization (a callous, detached response to the people served) and reduced personal accomplishment. Calling patients by bed number and answering a distressed family coldly show the detached stance that defines depersonalization. It beats A because exhaustion, feeling drained and depleted, is often the first and most common component, but the signs in this stem concern how the worker treats people (Maslach et al., 2001).

Takeaway: Depersonalization, the interpersonal component of burnout, shows as a detached, callous response to the people served.

Sources: Maslach, C., Schaufeli, W. B., & Leiter, M. P. (2001). Job burnout. Annual Review of Psychology, 52, 397-422, Three dimensions of burnout
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.6 Burnout, compassion fatigue, and secondary traumatic stress · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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120. ETH-1630 (rev 1) · Best answer: A

Which question is central to a restorative justice response to wrongdoing?

A. Who was harmed, and what do they need?
B. Which rule was broken, and what penalty fits?
C. How can both sides compromise and move on?

Why each option is right or wrong

A **Correct.** Zehr frames restorative justice around harms, needs and obligations: who has been hurt, what they need, and whose obligation it is to repair the harm (Zehr, 2015). The process engages those with a stake in the harm, starting with the person harmed. It beats C because restorative justice is not mediation between equal parties; the person who caused harm must accept responsibility, so a mutual compromise misstates its moral footing. B is the question of the retributive model that restorative justice was built to contrast with.

B **Incorrect** (trap: Wrong model or concept). This is the core question of a retributive, criminal-justice frame: identify the violation and assign a proportionate punishment. Restorative approaches may run alongside the legal system, but they ask about harm and repair instead.

C **Incorrect** (trap: Wrong model or concept). Compromise suits disputes in which both parties share fault, such as a neighbor conflict in mediation. In restorative justice one party has caused harm and must own it, so splitting the difference would minimize the wrong.

Takeaway: Restorative justice asks who was harmed, what they need and who must repair it, rather than which rule was broken and what punishment fits.

Sources: Zehr, H. (2015). The Little Book of Restorative Justice (revised and updated ed.). Good Books, Restorative justice defined; guiding questions
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.8 Restorative practice, truth and reconciliation with historically oppressed groups · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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121. INT-1567 (rev 1) · Best answer: B

An outpatient social worker gives a client with depression the PHQ-9 at each session of cognitive behavioral therapy. After 8 sessions, the totals remain in the moderate range with no downward trend. The client says the sessions are helpful and would like to keep going the same way. What should the social worker do **FIRST**?

A. Continue the current approach because the client finds it helpful
B. Go over the results with the client and revise the plan
C. Ask a psychiatric prescriber to evaluate the client for medication

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). The client's satisfaction and the alliance matter, but flat scores after 8 sessions show the approach is not reducing symptoms. Continuing without change risks prolonging the depression.

B **Correct.** Routine outcome monitoring is meant to change care when a client is not on track; feedback on stalled progress reduces deterioration (Lambert & Shimokawa, 2011). Eight sessions without change is a signal to act. Sharing the graph with the client and revising the plan together respects the client's view while taking the data seriously (Bloom, Fischer & Orme, 2009). It beats A because liking the sessions is not the same as improving, and continuing unchanged ignores what the measure shows.

C **Incorrect** (trap: Right action, wrong time). A medication evaluation may well become part of a revised plan. Choosing it before reviewing the data with the client skips the joint review that should shape what changes.

Takeaway: When progress data stay flat, review them with the client and revise the plan together, even if the client likes the sessions.

Sources: Lambert, M. J., & Shimokawa, K. (2011). Collecting client feedback. Psychotherapy, 48(1), 72-79, whole article; Bloom, M., Fischer, J., & Orme, J. G. (2009). Evaluating Practice: Guidelines for the Accountable Professional (6th ed.). Pearson, single-system designs: baselines, AB, withdrawal and multiple-baseline designs
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.1 Evaluating progress and effectiveness of intervention and service plans · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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122. ETH-1589 (rev 1) · Best answer: B

In a residential treatment group for adolescents, one member says to an Asian American member, 'Your English is really good for someone like you.' The Asian American member laughs and looks down. Other members glance at the social worker leading the group. What is the social worker's **BEST** response?

A. Ask the Asian American member to share how the comment felt
B. Pause the group and name the comment as a stereotype
C. Talk with the member who made the comment after group
D. Continue the discussion, since the member laughed it off

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Inviting feelings is often useful, but asking the targeted member to explain the hurt in front of peers puts the burden of response on that member. Support for the member can follow privately once the leader has addressed the comment.

B **Correct.** Microinterventions by a leader or bystander include making the hidden message visible and interrupting it, which protects the target and sets group norms (Sue et al., 2019). The member's laugh and lowered gaze, and the group's glances, show the comment landed. Pausing to name it as a stereotype that the member is foreign addresses the harm where it happened and shows the group how such comments are handled. It beats C because a private talk can educate the speaker but leaves the target and the group without a response.

C **Incorrect** (trap: Right action, wrong time). A private conversation with the speaker can help and may follow. On its own it leaves the targeted member and the group with silence, which can read as approval.

D **Incorrect** (trap: Good but not best). Laughing it off is a common way targets cope with a microaggression, not a sign it caused no harm. Moving on leaves the stereotype unchallenged in front of the group.

Takeaway: When a microaggression occurs in a group, the leader interrupts it in the moment rather than leaving the target to respond.

Sources: Sue, D. W., Alsaidi, S., Awad, M. N., Glaeser, E., Calle, C. Z., & Mendez, N. (2019). Disarming racial microaggressions: Microintervention strategies for targets, White allies, and bystanders. American Psychologist, 74(1), 128-142, microintervention strategies: make the invisible visible

Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.6 Privilege, implicit bias, microaggressions and their client impact · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3